

OPENING
Q1 — OPEN
Q2
Q3
Q4 — INFO GAP
Q5
Q6
Q7 —
Q8
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Q11 — CLINIC
Q12
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Q14 — TOOLS
Q15
Q16 — CLOSE
Q17
SIGNAL CHECK

CAROUSEL Interview Guide — Patient discovery
Thank you for making time. I'm doing research to understand what the experience of going through egg freezing or IVF is actually like — not the clinical side, but everything around it. There are no right or wrong answers. The most valuable thing you can tell me is the truth.
"Tell me about your egg freezing / IVF experience. Walk me through it — from the moment you decided to do it to where you are now." ▶ Just listen. Note where energy shifts.
"What was harder than you expected?"
"What was easier than you expected?"
"When you got lab results during your cycle — how did you find out about them, and what did you do with that information?"
"Were there moments where you felt like you didn't fully understand what was happening in your body or what your results meant?" ▶ If yes: "Tell me about one of those moments. What did you do?"
"Where did you go when you had questions between appointments?" ▶ Note exactly: Google, Reddit, Facebook, called clinic, partner, nothing.
"How would you describe the emotional experience — not the clinical side, just what it felt like?"
"Were there moments where you felt alone in this process?" ▶ If yes: "What did that feel like? What would have helped?"
"What support did you have — from your clinic, from people in your life, from apps or communities?"
"Was there anything you needed emotionally or informationally that you just couldn't find?"
"How would you describe your relationship with your clinic during the process?"
"How did you communicate with your clinic between appointments? How did that feel?"
"After your retrieval or transfer — what happened to your relationship with the clinic?"
"Did you use any apps, platforms, or tools to help you manage the process? What did you like or not like about them?"
"If someone had built a platform specifically designed for women going through this — what would you want it to do?" ▶ Do NOT suggest features. Let them answer freely.
"Is there anything about your experience I haven't asked about that you think is important?"
"If you could change one thing about the experience — just one — what would it be?"
GREEN: Spontaneous Google/Reddit mention, emotional isolation, portal frustration, wishing for more support. YELLOW: "It was fine" — probe deeper. "My clinic was great" — ask what was still hard. RED: Genuinely good experience with no gaps across multiple women.

ID	Link to Interview	Name	Interview Date	Current Age	 Status	Date of Procedure(s) and Age at time of procedure	Procedure(s)	Clinic	Doctor Name
P01	Jordana Reim and Sarah Lewen - 2026/03/23 14:13 EDT - Notes by Gemini	Jordana Reim	3/23/2026	42	Het F; Single	2/1/2024 (39 at time)	EGG F, IVF	Advanced Fertility Clinic Cancun Mexico	Didn't share
P02	n/a	Lissy Alden	3/23/2026	38	Het F; Married	34; embryo freezing @ 37	EGG F, EMB F	NYU Langone	Dr. Blakemore
P03	Ilana Peña and Sarah Lewen - 2026/03/26 14:15 EDT - Notes by Gemini	Ilana Pena	3/26/2026	34	Het F; Single	07/2024	EGG F	<u>Reproductive Partners Beverly Hills</u>	Dr. Carrie Wambach

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P04	Monya De and Sarah Lewen - 2026/03/26 16:30 EDT - Notes by Gemini	ABC	3/27/2026	43	Het F; Single		10/2025	EGG F	Didn't share	Didn't share
P05	Ana Cordera and Sarah Lewen - 2026/03/27 11:55 EDT - Notes by Gemini	Ana Cordera	3/27/2026	29	Het F; Married		12/1/2022 - age 26	EGG F	Spring Fertility	Catha Fischer
P06	Betina Grinspun and Sarah Lewen - 2026/03/27 14:15 EDT - Notes by Gemini	Betina Grinspun	3/27/2026	33	Het F; Single		2024	EGG F	Kindbody	

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P07	Percia Safar and Sarah Lewen - 2026/03/27 16:27 EDT - Notes by Gemini	Percia Safar	3/27/2026	35	Het F; Dating		2024	EGG F	<u>Reproductive Partners Beverly Hills</u>	Dr. Wambach
P08	Alexandra Perrotti and Sarah Lewen - 2026/03/30 11:00 EDT - Notes by Gemini	Alexandra Perrotti	3/30/2026	37	Het F; Single		12/2025, 02/2026, 03/2026	EGG F	Shady Grove Fertility	John Csokmay

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P09	Rory / Sarah (IVF UX Research) - 2026/03/30 11:58 EDT - Notes by Gemini	Rory Mabin	3/30/2026	37	Het F; Dating	2 - canceled 2 - retrievals 1 - one more upcoming	EMB F	RMA of NYC	<u>Dr. Chelsea Canon RMA</u>
P10	Aviva Copaken and Sarah Lewen - 2026/03/30 14:00 EDT - Notes by Gemini	Aviva Copaken	3/30/2026	39	Het F; Single	02/2026	EGG F	Tree of Life	Dr. Johanitch

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P11	Shira Berg and Sarah Lewen - 2026/04/02 13:30 EDT - Notes by Gemini	Shira Berg	4/2/2026	35	Het F; Single	Jan 2025	EGG F	Cornell Weill NYC	Dr. Alexis Melnick
P12	Savannah Allan and Sarah Lewen - 2026/04/02 14:00 EDT - Notes by Gemini	Savannah Allen	4/2/2026	35	Het F; Dating	2025	EGG F	Spring Fertility SF	Dr. Julia Kim

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P13	justine and Sarah Lewen - 2026/04/06 12:59 EDT - Notes by Gemini	Justine Chausson	4/6/2026	38		Het F; Single	2024	EGG F	RMA New York	Dr. Rachel Danis RMA

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P14	Garvey Chui and Sarah Lewen - 2026/04/07 14:28 EDT - Notes by Gemini	Garvey Chui	4/7/2026	43		Het F; Married	2017; 2019; 2024 (IVF)	EGG F, IVF, EMB F	Kindbody	Dr. Kristin Bendickson
P15	Clair Simpson and Sarah Lewen - 2026/04/08 16:30 EDT - Notes by Gemini	Clair Simpson	4/8/2026	35		LGBTQ	13 rounds over 2020-2023	EGG F, EMB F, IVF	Create Fertility in Toronto and CNY in New York	Dr Prati Sharma in Create, Dr Falcone in CNY
P16	Katie Kilroy and Sarah Lewen - 2026/04/09 14:30 EDT - Notes by Gemini	Katie Kilroy	4/9/2026	35		Het F; Married	02/2024 04/2024 06/2024 07/2024 10/2024 03/2025	EGG F, EMB F, IVF, Other	Shady Grove Fertility Colorado	<u>Shona Murray, M.D. Colorado Springs</u>

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P17		Maggie Connors	4/9/2026	38		Het F; Single	2022; 2024	EGG F	Kindbody LA/NY	
P18	Jess Thevenoz and Sarah Lewen - 2026/04/09 15:29 EDT - Notes by Gemini	Jess Thevenoz	4/9/2026	30		Het F; Married	2025 (age 29)	EGG F		https://www.pinnaclefertility.com/physicians/dr-lynn-b-davis/
P19	Jenny Dwork and Sarah Lewen - 2026/04/10 14:30 EDT - Notes by Gemini	Jennifer Dwork	4/10/2026	40		Het F; Married	Egg Fr @ 33 (16 eggs); Egg Fr @ 38 (4 eggs); IVF @ 39 (12 embryos!)	EGG F, IVF	Extend Fertility (Egg Freeze) / RMA (IVF)	

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P20	Pauline McGlone and Sarah Lewen - 2026/04/14 14:43 EDT - Notes by Gemini	Pauline McGlone	4/14/2026	30		Het F; Single	Egg Fr @ 29	EGG F	NYU Langone	Dr. Blakemore
P21	Lori and Sarah Lewen - 2026/04/14 16:29 EDT - Notes by Gemini	Lori Ying	4/14/2026	34		Het F; Dating	Egg Fre - 2 rounds - @ 31 Split cycle - @ 35	EGG F, EMB F	RMA (Lucky); NYU Langone (Egg + Embryo Split Cycle)	Dr. Lucky

ID	Link to Interview	Name	Interview Date	Current Age		Status	Date of Procedure(s) and Age at time of procedure	Procedure(s)	Clinic	Doctor Name
P22	Deniz Melen and Sarah Lewen - 2026/04/14 17:25 EDT - Notes by Gemini	Deniz Melen	4/14/2026	40		Het F; Married	7 IUIs Ages 34-37 (2020 - 2022) Egg freeze - 37 Egg freeze - 40	EGG F, IUI	Anova Fertility (Toronto)	Dr. Marjorie Dixon
P23	Maia Keerie and Sarah Lewen - 2026/04/15 15:58 EDT - Notes by Gemini	Maia Keerie	4/15/2026	39		Het F; Single	Aug 2023	EGG F	Newlife IVF (Australia)	Dr Hugo Fernandes
P24	Joan Zhang and Sarah Lewen - 2026/04/15 17:00 EDT - Notes by Gemini	Joan Zhang	4/15/2026	unknown		Het F; Married	2022 - 2026	IUI, IVF, EGG F, EMB F	Various	Various

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P25	Lydia Hirt and Sarah Lewen - 2026/04/16 16:29 EDT - Notes by Gemini	Lydia Hirt	4/16/2026	41		Het F; Married	2024 - 2025	EGG F, EMB F	NYU Langone RMA Cornell	https://weillcornell.org/mohamad-irani-md
P26	Nour Sharara and Sarah Lewen - 2026/04/17 14:01 EDT - Notes by Gemini	Nour Sharara	4/17/2026	39		Het F; Single	2023 (35) 2023 (36)	EGG F	Cornell Weill	Dr. Alexis Melnick

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# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	Single woman, mindfulness teacher and retreat leader, independent contractor. Froze eggs at 39 in Cancun, Mexico for cost reasons after initial consultation at Kindbody in the US priced at ~\$15,000. All-in cost in Mexico including travel and accommodation: ~\$6,000. Retrieved 10-12 mature eggs plus 2-3 immature eggs frozen as well. Currently pays \$650/year storage in Mexico. Now 42, has decided she wants to have a child as a solo mother and is actively researching IVF with the same Mexican clinic. Participated voluntarily — found the interview through a WhatsApp group. Offered to continue the conversation and asked for a follow-up 15-minute call. Genuine, emotionally intelligent, highly self-aware.	Described her decision as happening "in a fear-based state in order to empower herself." Never planned to freeze eggs — believed she would meet a partner during her fertile years. Was scared into action by an oversharing acquaintance in her 50s who had a child at 45. Was in a relationship with a younger man who told her he would break up with her if she wanted kids — she was doing this entirely without his support or participation. Ambivalent about having children at the time — primary motivation was buying time and preserving optionality, not because she wanted children immediately. AMH and follicle count results at Kindbody (follicle count ~9, just below average for age) amplified her urgency and anxiety.	After Kindbody consultation revealed numbers on the low side of normal and quoted ~\$15,000, she immediately began researching international options. Googled extensively — found women's blogs, fertility coaches, checklists of questions to ask clinics. Did a paid coaching call with a fertility-adjacent expert. Narrowed to Mexico for proximity and cost. Chose a Cancun clinic based on their willingness to do multiple video calls with her before committing — did 2-3 pre-procedure video calls with the doctor. Verified the clinic's answers against her research checklist. This was pre-AI — all research was manual, Google-driven, and community-sourced.
3	Lissy is a personal friend of Sarah's — knew the context, was enthusiastic about the company from the start. Two separate procedures: egg freezing at 33-34 (13 eggs retrieved), embryo freezing at 37 (20 eggs, 7 embryos including one mosaic). Currently lives in Columbia, South Carolina. Has a family connection to NYU — her uncle is a physician and mentor to Dr. Blakemore, which is how she chose the clinic. Two strong clinical outcomes. Transcript captures approximately 70% of interview — final portion cut off mid-sentence. Note: this is a personal relationship, not a cold research subject — treat data accordingly.	Deeply conflicted. Felt pressure from all directions — mom, friends, society — to freeze her eggs. First appointment at 33 was triggered by external pressure, not internal readiness. The initial consultation at NYU felt like a sales pitch — "the best sales pitch of all time" — which she found deeply confronting. Left the first appointment saying "I'm not f***ing doing this" because the doctor, however medically excellent, overrode her emotional state rather than acknowledging it. Waited a full year before returning. When she came back at 34, she went on her own terms with her own timeline — and the experience felt completely different. Described the emotional entry as	Did not do extensive independent research — chose NYU based on family connection and reputation as the best outcomes program in the country. Her uncle is part of the practice and Dr. Blakemore's mentor. Did not shop around. First appointment included a detailed presentation she couldn't photograph — furiously took handwritten notes. Found the presentation confronting and data-heavy without emotional acknowledgment. Did not use apps, communities, or platforms for research. Her network was her primary information source.
1	Single woman in Los Angeles (Silver Lake). Froze eggs at 33 after years of thinking about it — finally pulled the trigger when she had the time, could afford it, and friends started doing it. Got insurance coverage by finding a doctor willing to document a medical justification (heavy periods potentially leading to endometriosis). Retrieved 21 eggs. Pays approximately \$200/quarter for storage. Highly social, highly networked, very community-oriented. Articulate, anxious by her own description, rule-follower. Offered to stay involved as Carousel develops — genuine enthusiasm. Signed up for waitlist at end of call and offered to share with her network.	"I wanted it to have been done." Classic anticipatory anxiety — not fear of the procedure itself but the logistical weight of managing it around her life. High stakes mentality: "It felt so high stakes and I only want to do this once." Driven by wanting to protect her life and do it on her own terms — very deliberate about timing around travel, friends' weddings, Mexico trips. Anxiety was specific and practical, not existential. Her dad died suddenly when she was five which she connects to heightened health anxiety generally — "I'm going to mix the vial wrong and then I'm going to die."	Entirely community and recommendation-driven. Asked everyone in her network — who did you go to, did you like them, how did it affect your life. Specifically sought out single women doing it as insurance. Valued social proof heavily — friend's cousin had two living children from frozen eggs, which she described as "really good data." Tried Kindbody first based on a friend's recommendation but didn't love the doctor or the vibe. Switched to Reproductive Partners Beverly Hills on a second recommendation. Insurance navigation was significant — found a doctor willing to document a medical justification. Described this as something people whisper about but nobody openly discusses. Did not use any apps or formal platforms.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	Single woman, came out of a long-term relationship, froze eggs at 42 for practical reasons — to preserve optionality and avoid rushing into a partnership or making decisions for the wrong reasons. Paid out of pocket (employer under 100 employees, not covered). Located in California based on area code. Found the interview via a WhatsApp group but couldn't identify which one. Requested anonymization — no AI processing of her name, agreed to proceed with pseudonym.	Described her entry as "partly empowering but partly annoying — like being assigned another full-time job when you're already doing a full-time job." Not overwhelmed in a fearful way — more burdened by the logistical weight of it on top of an already full life. Was doing it largely alone — described an "unusual lack of family support." Her primary emotional support was one close male friend. Did not tell many people.	Research was limited and informal. Primary sources: women's Facebook groups of educated women, some primary research studies, and a summary of It Starts with the Egg (not the full book). Did not do extensive research — described deferring to the clinic once she chose one. Clinic choice was primarily recommendation-based from her network. Did not use any apps. Passive online behavior only — would search something and land on Reddit or an acupuncturist's site. Never posted publicly or as herself in any
1	26-year-old when she froze, now 29. Northwestern alum, based in Prospect Heights Brooklyn, works at Meta. Did one cycle in December 2022 — fully covered by Meta's fertility benefits including three years of storage. Engaged, getting married next year, turning 30. Did it proactively as an insurance policy before her current relationship, not because of any clinical urgency. Strong outcome — double-digit eggs, no second cycle needed. Currently facing the storage payment decision as Meta's coverage expires this December. Enthusiastic participant, offered to share the landing page with 10 women. Also interested in exploring paths outside corporate — worth noting for future relationship.	First half of 2022 she was sad about not being in a relationship — that loneliness directly influenced the decision. "Who knows if I'm going to be in a relationship at 30 or 32 when I want kids." Combined with wanting to maximize Meta benefits before leaving. Emotional state was pragmatic and forward-planning rather than fearful — she described it as wanting to squeeze as much juice as possible from her benefits before leaving. Didn't feel she needed emotional support going in — framed it as a process to execute rather than an experience to have. That realization only came after retrieval.	Entirely marketing and influencer-driven. Saw Dr. Fischer speak at an event. Followed influencers who partnered with Spring Fertility — specifically Serena Carrian. Did not research Cornell, NYU, or other clinic systems. Validated the decision through a Meta internal Slack group where women posted about egg freezing experiences. A coworker who had used Spring previously gave informal endorsement. Did minimal independent clinical research — Mayo Clinic website, some Google searches. Her honest self-assessment: "I was completely caught in the marketing of it all." Would have weighted a personal friend recommendation far more heavily but had no friends who had done it yet — she was among the earliest in her circle at 26.
2	33-year-old woman, recently divorced before the freeze, single now. Did two egg freezing cycles at Kindbody — covered by employer insurance. Has approximately 40 eggs frozen. Has not done IVF and is not currently considering it. Her experience was notably more positive than other women in the dataset — she came in explicitly because she felt her experience was different from the negative narratives she kept hearing. Fitness-oriented, highly self-directed, proactive researcher. Told her boss and direct reports she was going through it. Remarkable self-awareness throughout. Sent Sarah the Kindbody portal screenshots after the call — follow up to receive those.	Got divorced one year before freezing. Having children was always her dream — the divorce forced the timeline question. Waited a period after the divorce to emotionally stabilize before acting. Did not freeze from fear or external pressure — froze from a clear-eyed, proactive decision to preserve optionality while young. Framed it explicitly as a privilege: "It's a privilege that my body is able to cope with that." Postponed initially due to negative narratives she had absorbed from others about hormonal side effects, moodiness, inability to work out. When she finally went, she entered with a rational, prepared mindset — "I told myself I'm doing this, let's go for it."	Chose Kindbody because it was her insurance partner and she had maxed out her out-of-pocket for the year — purely pragmatic timing. Worked with a nutritionist beforehand to prepare her body — supplements, diet optimization, hormone preparation. Also coordinated timing around finishing Accutane with her dermatologist. Did independent research on exercise restrictions — clinic said no working out, she researched why and determined she could maintain light exercise through day 10 before ovaries became too enlarged. Used her own research to make informed decisions rather than following blanket instructions. This is a critical product insight — she wanted the why, not just the what.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	35-year-old woman, San Francisco-based, currently in a new relationship and moving in together. Froze at 33, paid approximately \$16K out of pocket — no employer coverage at the time. One cycle, high egg yield (~40+ eggs retrieved despite ~30 follicles). Has not done IVF. Connected through Maddie and Michael — extended mutual network. Persia is the woman who recommended Dr. Womboch to Ilana Peña, who is also in the dataset. Warm personal relationship with Sarah — conversation evolved well past the interview into a genuine friendship conversation. Also on Percia's list to potentially set Sarah up. Note: significant portions of the second half of the call were personal conversation rather than research interview — research data is captured from the first half.	Went into the process relatively positive and pragmatic. Had friends who had done it earlier in their 20s so it had been on her radar for a while. Deliberately waited until closer to 35 because she had heard the earlier you freeze the less likely you are to use the eggs — wanted to time it for when having kids felt genuinely uncertain. Framing: insurance policy, not a desperate measure. Paid out of pocket which was "annoying but didn't stop me." Most significant emotional moment was post-retrieval — described feeling "so resentful" that she had to go through that level of pain because "dating sucks." Also felt resentful of friends who described it as empowering: "I don't feel empowered. I feel like I'm being gaslit into being excited about something that objectively sucks." Now, with distance, she feels less emotional about it.	Did minimal independent research. Cousin had done it. A good friend in LA had done it. Felt the process was "pretty vetted" through her network. Chose Dr. Womboch based on her friend's recommendation — same doctor she later recommended to Ilana. Did not research other clinics or doctors comparatively. Read a little online but peer network was the dominant information source by far.
3	37-year-old woman, Washington DC-based, single. Did three egg freezing cycles at Shady Grove Fertility between December 2025 and February 2026 — fully covered by employer benefits after she personally petitioned her company to expand their fertility benefit from active infertility only to include elective egg freezing. First cycle cancelled on day 11 due to poor follicle cohort. Second cycle successful — 7 eggs frozen. Third cycle — approximately 6 eggs frozen. Total: approximately 13 eggs. Has decided not to do another cycle despite not reaching her stated goal of 20 eggs. Connected through mutual Reality network. Goal-oriented, self-described results-focused personality. Went through a significant relationship breakup simultaneously with her first cycle — the relationship partly imploded over the partner being on the fence about children.	Had egg freezing on her radar but the employer benefit finally removing the financial barrier was the deciding factor. Was not deeply educated going in — didn't know what AMH was, didn't know about follicle counts or ovarian reserve before her first appointment. Was simultaneously going through a painful breakup, which caused her to emotionally dissociate from the data and the process — she just wanted to get through it. When the first cycle was cancelled she was unexpectedly and disproportionately distraught. Her primary emotional state throughout was wanting it to be over. The cancelled cycle hit harder than she anticipated precisely because she had not prepared for that possibility and had been operating on the assumption that if she did the steps correctly, it would work.	Went to her OB/GYN first — proactive and rare per Sarah's observation in the interview. OB verbally recommended Shady Grove. Alexandra didn't do additional comparative research — just confirmed insurance acceptance and location convenience. Had a best friend who is an OB/GYN who asked her medical questions and provided support. Did not do deep pre-research. Wished she had been given a one-pager or a link from her OB with what to expect — she would have read it if it had been handed to her but the verbal referral alone left a gap.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
5	38-year-old woman, New York-based, in a serious relationship of almost four years. Works at NASDAQ — fertility covered by Progyny. Doing embryo freezing with her partner, not solo egg freezing. Has been through four cycles — two completed retrievals, two cancelled mid-cycle due to poor follicular response to stimulation. Has one healthy female embryo frozen. Planning a fifth cycle in April with the intention of freezing one or two more embryos before trying naturally, with hopes of being pregnant later this year. This is the most medically complex case in the dataset — multiple cancellations, insurance intervention, hormonal resistance, and the deepest clinical journey by far. Connected through mutual network — knows Sarah personally. Was a product person herself — works in technology at NASDAQ.	Spent most of her adult life certain she didn't want children. Had multiple OB/GYNs refuse to perform tubal ligation telling her she would change her mind — a decision she resented at the time and now acknowledges they were right about. Met her current partner shortly after a 10-year marriage ended. Within six months he raised the possibility of having a family. She came around to the idea. Now almost four years together, actively planning. Did not try to conceive naturally first — went straight to embryo freezing for optionality and time. Entered the process cautiously but ignorantly optimistic — was not prepared for the actual weight of it.	Chose RMA through two simultaneous recommendations — a friend who had a positive experience there, and a strong recommendation from Progyny, her employer's fertility insurance provider. Progyny has a close relationship with RMA. This is the first case in the dataset where the insurance provider itself drove the clinic recommendation — a meaningful GTM insight for the employer benefits channel.
2	38-39 year old woman, Brentwood LA-based, single, no partner, no children. Dog walker — gig economy, financially precarious. Did two egg freezing cycles at a discount fertility clinic found via a Black Friday deal — total cost \$6,000, borrowed interest-free from a Jewish loan service and repaying monthly. Retrieved two eggs in each cycle — four eggs total. Deeply disappointed by outcome. Has not done additional cycles due to cost. Her mother died unexpectedly two years ago — her primary emotional anchor. Is estranged from all three siblings following a post-death family rupture. Her father had a stroke and cannot provide meaningful emotional or financial support. Her best friend is her only remaining close support person. Is on Prozac, sees a therapist weekly, describes herself as profoundly alone and in the most depressed state of her life heading into her 40th birthday in May. This is the most emotionally vulnerable	Had egg freezing on her radar casually for years. When she turned 38 she felt acute anxiety and urgency — no partner, no husband, deep desire to be a mother. Her mother's death two years prior had intensified everything — the grief of her mother not meeting her future children, her father's age (86), the family rupture, the isolation. She entered the process in a depressed and desperate state — her words. She was not in a stable emotional baseline when she started. The fertility journey compounded an already acute grief and loneliness crisis.	Pre-Procedure / Research Process Found the clinic via Google search for "affordable egg freezing near me." Found a Black Friday deal — \$6,000 total including first year storage. Financed through a Jewish interest-free loan service. Did not do comparative clinical research. Did not know the clinic's outcomes data or success rates. Made the decision based primarily on cost. Talked to her best friend, her father, and her sister. Did not use Facebook groups beforehand.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	Shira Berg, 35, New York City. Healthcare PR professional. Froze her eggs in January–February 2025 at Cornell with Dr. Alexis Melnick, age 34 at time of procedure. Paid \$14,000 out of pocket — no insurance coverage. One cycle. Retrieved 19 viable eggs. Objectively one of the most positive clinical experiences in the dataset — assigned nurse, email-based communication, seamless logistics, excellent outcome. And yet: the post-retrieval emotional crash was significant, she never felt "empowered," and she spontaneously designed the community/buddy system feature without any prompting. Important counter-data and important validation living in the same interview.	Egg freezing was on her radar since her late 20s — several banking friends froze before business school when it was employer-covered. Filed it away. COVID hit, she turned 30, was newly single, started thinking about it more seriously. Her OB at Cornell (Dr. Nolan) said her levels were fine and told her to come back if needed. She did — four or five months later she met her boyfriend. Tabled it again for three years. When they broke up in summer 2024, her first call — before she told her friends — was to Dr. Melnick. "Before I told my friends that my ex and I broke up, I called her. That's who I called." The decision was made in grief, not empowerment. She was in a dark place. Dr. Melnick's response to that call — warm, reassuring, "you are never going to use these eggs" — was the moment that locked her loyalty to both the physician and the clinic.	Chose Cornell entirely through trusted referral — her OB recommended Dr. Melnick, multiple friends had positive experiences with her, and a friend's aunt (Janice Fox, Head of Endocrinology at Harvard Brigham Women's) personally endorsed her. Did not shop other clinics. Had zero insurance coverage and paid entirely out of pocket — but this did not drive clinic selection. The decision was relational, not comparative. "There was no way I wasn't working with her." Notable: she had a prior OB who told her she had PCOS — she didn't believe it, switched doctors, and has been fine. That history of one bad clinical relationship sharpened her loyalty to physicians who actually listened.
2	Savannah Allan, 35, San Francisco. Product marketing professional. Two rounds of egg freezing at Spring Fertility SF (Vanes location) under Dr. Julia Kim, completed 2025. Long-term partner, relationship status not a driver — decision was entirely self-directed around optionality, not urgency. Had employer benefits through Carrot twice — first time couldn't use them due to timing and job change, second time exhausted them on round one and is now on a payment plan for round two. Pre-journey was characterized by repeated delays, misinformation, and a lack of coordinating guidance. The actual clinical experience at Spring was largely positive. The medication and financial navigation was a disaster she solved herself. Post-retrieval recovery was harder than expected with minimal follow-up. Validates the thesis strongly on the informational and navigation layer. Counter-data point on emotional isolation — she had friends and didn't feel alone. Salve app named explicitly and critiqued directly.	First thought about egg freezing around age 32 — prompted by friends doing it and an employer benefit through Carrot. Her male gynecologist at the time told her she was too young to worry about it and discouraged her from testing her AMH. She deferred. Looks back on that moment with frustration — she believes she lost time because of that dismissal. The second entry point came when she started her current job at 34 and immediately identified the benefit as something to use. By that point the emotional register was urgency mixed with frustration — she had already been delayed once and felt time pressure acutely."I feel like I kind of jumped on the wagon a little late due to just like my gynecologist kind of being like, 'Eh, you don't need to worry about that right now.'" Notable: she is ambivalent about whether she wants children at all. She froze her eggs to preserve optionality, not because she is certain she wants them. This is an underrepresented emotional register in the dataset — the woman who wants the door open but isn't sure she'll walk through it.	Chose Spring Fertility because a trusted friend invited her to a Spring-hosted Q&A event, had already done her freeze there, and spoke highly of it. Also met Dr. Julia Kim at that event — the in-person connection sealed the decision. Read some reviews. Looked briefly at one other clinic but didn't do deep comparative research. "I was very reliant on like this one friend's information because I just trust her a lot." Peer referral is the dominant selection mechanism — consistent with every other participant. The Spring info session is worth noting as a clinic acquisition channel: it converted a pre-clinical woman into a patient through a low-pressure in-person event. Savannah would not have found Spring otherwise. She had a prior attempt to engage with fertility services through Progyny at a previous employer — got to the initial consult stage and then left the company before completing the cycle. That experience introduced her to some of the vocabulary and process but also left her with a false sense of how fast things could move.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	French-American woman, 38, based in New York. Froze eggs at 36 at RMA Brooklyn (Dr. Rachel Danis) during one of the most stressful concurrent periods of her life: she was processing a separation/divorce after a 10-year relationship (met Sarah through the Hoffman Institute), and her direct boss went on unannounced sabbatical the day before her injections started, forcing Justine to absorb her boss's full role and workload for the entire month. Worked 3 weeks straight, no weekends, often until 10pm — including the night before her Memorial Day retrieval. Retrieved 9 eggs; only 1 mature. RMA called this "very unusual — we've never seen this before." The post-result call from Dr. Danis lasted approximately 2 minutes. The compounding weight of the divorce, work crisis, devastating result, and clinically cold follow-up led to a breakdown and Justine quitting her job. Currently 38, not working, cannot afford a second cycle (~\$20,000). No eggs in storage.	Decision was catalyzed post-Hoffman Institute, where she was processing the end of a 10-year relationship with someone she had discussed having children with. "Post-Hoffman, I was like — let's pull the trigger, do everything in one year." The decision was proactive and self-directed, not fear-based. However, the initial consultation included visual materials showing success rates by age with a visible drop-off at 36–37, and she recognized the for-profit framing: "They kind of have to scare you into it in a way." She acknowledged this intellectually but still felt the weight of it. She entered the cycle in a deeply vulnerable emotional context — active divorce, relationship grief — that no one at RMA ever assessed or acknowledged.	Chose RMA based entirely on word-of-mouth from multiple friends. Did not consult any other clinics. Read some online reviews but trusted personal referrals far more: "If you don't know the person writing the review, you're just not really going to trust it." Onboarding at RMA was thorough: laminated visual materials with statistics by age, a financial advisor walkthrough, and assignment of a dedicated non-clinical coordinator (not a nurse, not a doctor) who was accessible by email, responded quickly, and walked her through medication logistics. Portal included educational videos about the process and injections — clear, concise, not outdated, helpful for a visual learner. RMA closes annually for a regulatory period; she adjusted from her planned June timing to May as a result.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
3	Garvey is the first participant in the dataset who completed the full arc — egg freezing through successful IVF through live birth. She is also the first participant with significant international complexity: froze in New York, transferred eggs to California, did IVF monitoring in Canada while eggs remained at Kindbody US. Her journey with Kindbody spans approximately 6-7 years and is a case study in clinic deterioration at scale — her early experience was positive, her later experience was operationally catastrophic. She escalated to Kindbody co-founders directly and was managed only because she complained loudly and persistently. She is analytically sharp, strategically minded, and went out of her way to connect with other affected Kindbody patients through a Bloomberg journalist. Her daughter was 10 weeks old on the call — born from the one viable embryo.	Notably low stakes at first. She was ambivalent about having children — framed it as something she understood as "her responsibility in a heterosexual relationship" rather than a deep personal desire. Froze for optionality and insurance, not urgency. First round was almost casual. Emotional weight increased significantly with the second round at 39 — just out of a traumatic relationship, leaving the US, under pressure. IVF phase was where the experience became genuinely traumatic — not clinically but operationally and psychologically, compounded by horror stories from other patients.	Entirely community and peer-driven. Friends in Asia were circulating a shared spreadsheet. Egg freezing was illegal for unmarried women in many Asian countries at the time — she was aware of the legal and geographic complexity early. Chose Kindbody for the first round because they were new, tech-forward, and offered a discount at an info session. No comparative clinical research. Word of mouth and proximity.
13	Scottish-born, lives in Toronto. Same-sex couple — did reciprocal IVF where partner provided eggs and Clair carried. Partner did 13 egg retrievals. Clair did 6 transfers. Two children now ages 4 and 2. Total cost approximately \$200,000 — in debt. Both nearly died in childbirth. Success came when a visiting US embryologist introduced a new petri dish protocol that stopped embryo arrest — the decisive clinical intervention. Five years consumed by IVF.	Did not start wanting children — her partner did. Felt pressure of partner's age (met at 35). Massive learning curve from the start — described IVF as "basically a full-time job" for five years. Facebook groups and peer community were primary information sources.	No structured research resources — entirely community and peer-driven. Facebook groups for protocol research, supplement guidance, drug comparison. This was before AI tools were widely available. Described it as "my whole life for 5 years." Did not use any formal platform or app.
6	Military family, lives in Richmond VA. Discovered rare genetic kidney disease after first pregnancy — could not safely carry again. Did 5 complete IVF rounds at Shady Grove Colorado Springs Feb 2023 - summer 2024. Husband deployed for all cycles except the last. Now pregnant with boy via surrogate due May 2026. Daughter age 4. Connected via mutual friend Liz Walton (CMO at Katie's company). Warm, articulate, highly open. Described IVF as "by far the hardest thing I've ever done."	Not anticipatory — thrust into IVF by medical necessity after kidney disease diagnosis post-first pregnancy. Nephrologist said "I wouldn't have another kid yourself." Made the decision in one appointment. No prior fertility concerns. All-in mentality from the start.	Google-driven clinic research — no referrals from friends or family. Watched testimony videos on clinic websites. Read doctor reviews obsessively. Rejected CCRM and Kindbody for feeling "too machine-like." Chose Shady Grove on vibes and Dr. Murray's human, approachable video. Husband deployed — navigated entire selection process alone.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
2	Personal friend of Sarah's — reconnected through Sarah's LinkedIn fertility posts. 39, single, lives in NYC (near Prospect Park). Divorced at 34. Did first cycle at Kindbody LA (Feb 2024, age 37), second cycle Kindbody NYC (age 38, motivated by 25% off National Infertility Month deal). Got divorced and then started seriously considering egg freezing. Was initially turned off by the commercialism of the fertility industry (Bogo offers, "bring a friend" discounts). Now actively researching solo IVF / sperm donors / IUI as a potential single mother. Mom lives nearby in Boerum Hill, supportive.	Divorce at 34 was the catalyst for taking fertility seriously. Initial objections: cost (\$12-15K felt enormous), skepticism about commercialism of fertility industry. Two years of resistance before finally acting at 37. Also noted: the "if you bring a friend" marketing made her feel IVF was "all a business" — had to get over cynicism before she could engage. Key internal reframe: "I can either cynical my way through this and not have the option, or participate and have eggs on ice."	Word-of-mouth dominant — friend's friend recommendation of Kindbody LA plus a specific doctor. Read Yelp and reviews. Initially skeptical of Kindbody's brand ("too cute, too yellow"). Asked in her first consult: "What corners are you cutting? What happens to my eggs if you go out of business?" Appreciated when they answered the storage/bankruptcy question credibly. Chose LA Kindbody partly on price vs. established Beverly Hills clinics.
1	Born in Montreal, lives in Seattle. 30 years old. Froze 37 eggs at age 29 at SRM / Pinnacle in Seattle (June 2025). Had early consultation conversations at age 26-27 but procrastinated until months before turning 30. Married. Career-driven founder. Sister had fertility challenges at 37 — key catalyst. Had almost no peer community doing it at the time she went through it. Explicitly chose a female doctor. Texts with clinic via dedicated nurse on-call line. Experienced OHSS scare during process. Recovery harder than expected. Eggs are stored — no plans to use yet.	Biological clock pressure combined with career-first mentality — saw successful founder-women have kids late and wanted to replicate that. Sister's fertility challenges at 37 added urgency. Procrastinated despite knowing she should do it — "it's preventative, it's never urgent enough." Landmark of turning 30 forced action. Primary emotion: uncertainty and lack of trust — "everyone has their own perspective but no one has the macro perspective."	OB/GYN referral to fertility clinic — didn't know that was how it worked ("felt really dumb"). Chose between two insurance-covered options — chose SRM based on website, reputation, and that the doctor was female and head of the clinic. Near panic attack when she discovered "really bad reviews" on Google Maps en route to first injection. Husband helped filter reviews and contextualize. Very little peer guidance — knew almost no one who had done it before.
3	Personal friend/neighbor of Sarah's in Williamsburg. 40 years old. Two egg freezing rounds at Extend Fertility (age 33: 16 eggs; age 38: 4 eggs) followed by IVF at RMA NY with Dr. Sutton. Used frozen eggs from age 33 — converted 16 eggs into 11 healthy embryos. IVF baby Addie born December 2025. Plans to implant again in October for a boy. Partner gave shots during IVF. Entered fertility journey via CBS Business School peer group with Dr. Pak Chong informational session. Chose Extend over Weill Cornell for cost (\$20K vs \$10K). Currently has embryos stored at RMA NY (\$400-500/yr more than warehouse storage in Western MA).	Post-relationship anxiety in early 30s. Entry point was social/peer-driven: CBS Business School friend hosted an informational session with Dr. Pak Chong. Saw the 35-year cliff chart for the first time there. Cost was a significant initial barrier at \$10-15K. Did not have employer fertility benefits at the time.	CBS peer group + Dr. Pak Chong informational session was the catalyst. Chose Extend Fertility over Weill Cornell on cost (half the price). Location (Midtown, near Chelsea) was a factor. Liked the low-friction informational session entry point. Wanted a clinic ranking system like NYMag Strategist: best for affordability, best for outcomes, best for scientific backing. Did not focus on individual doctors — focused on the overall apparatus and clinic reputation.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
1	Overview Pauline is 31, works in finance at NYU, and went through egg freezing two weeks ago at NYU Fertility Center with Dr. Jennifer Blakemore. Her entire process was expedited under unusual time pressure — she had just gotten a new job and needed to complete the cycle before her NYU benefits expired. She came in with zero prior knowledge of fertility treatment, was told her AMH was very low and she had only 2 follicles visible, made the gutsy decision to proceed anyway, and retrieved 14 mature eggs — far exceeding any expectation. Overall a highly positive clinical experience with specific friction points around the injection process, real-time support, and post-retrieval isolation.	Post-breakup in December 2025 catalyzed the decision — "I've always thought about this, but now it feels like the right time." Also motivated by watching a colleague go through it openly and enthusiastically. Strong family desire but no urgency — "definitely not right now, but with the right person eventually." Entry point was practical and agentic rather than fear-based, though the AMH news introduced significant emotional disruption mid-process.	Zero pre-research — described knowing "nothing" before starting. Clinic selection was entirely benefit-driven (NYU employee, fully covered through NYU) plus a PCP referral to Dr. Blakemore specifically. No independent research, no reviews, no influencer input. PCP said "she is that girl" and Pauline called the same day. Used personal connections to bypass a May waitlist and get in that Friday. Progyny activation was required as a third-party benefit step — described as seamless with a helpful Progyny coordinator who walked her through benefits and next steps.
3	Overview Lori is 34, New York-based, works in venture capital with a specific focus on women's fertility as an investment thesis. Single at 31 when she first considered freezing. Advocated internally at her employer to get egg freezing covered as a benefit — succeeded, coverage came through Progyny. Did two back-to-back retrievals at RMA with Dr. Lucky Sekhon at 33. First cycle: 7 mature eggs. Second cycle (protocol changed to include estrogen priming): 14 mature eggs. Now preparing for a split cycle (half eggs, half embryos) at NYU with Dr. Sarah Cascante. Delayed one month due to a cyst. Highly informed, data-driven consumer.	Emotional Entry Point Not emotional — transactional and strategic. "I view it kind of like my 401k. I need to get this done because it's the right thing to do." Felt significantly better after completing the retrieval cycles. No therapist, no emotional support structure needed. Important counterpoint in the dataset — validates that not all women experience this as emotionally fraught. Carousel must serve the full spectrum	Used SART and FertilityIQ as primary research tools. Only cared about success rates — did not weight friend recommendations. "On my one shot worth \$30K, I wasn't going to go on vibes." Cross-referenced top clinics (NYU, RMA, Cornell, CCRM) against best doctors on FertilityIQ. Got a cancellation slot with Dr. Lucky Sekhon. Key insight: SART data is valuable but nearly impossible to interpret without help. "If I were able to put in my situation and output which of the 10 stats I should actually care about — that would have been helpful

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
9	Deniz is 40, married, based in Toronto. She entered the fertility journey with no anticipation of issues — discovered a severe vitamin D deficiency, suspected PCOS, and a short luteal phase across separate appointments over years. Did 7 IUIs across two clinics, two IVF retrievals (first government-funded, second self-funded), five transfers, one pregnancy ending in termination at 26 weeks after anatomy scan revealed the baby was incompatible with life. Just completed her second retrieval this week. Waiting on biopsy results. Does not feel she has a third retrieval in her.	Emotional Entry Point Wanted children. Married at 31. Expected it to be straightforward. Fear and confusion accumulated slowly as each new discovery revealed a more complex picture than the last. "I had zero intentions of ever doing IVF, much less doing it twice."	Pre-Procedure / Clinic Selection Referred by GP with no real orientation to what the fertility clinic world was. First clinic was not good in hindsight. Switched to Anova only because a friend recommended it. No independent research or comparison — she didn't know what to compare.
2	Maia is Australian, based in San Francisco for 7 years. Froze eggs in Australia in August 2023 at age 36–37 — two back-to-back cycles. Chose Australia over the US due to cost and socialized healthcare. Began injections in the US, flew back to Australia for monitoring and retrievals, returned to San Francisco one week post-second retrieval. Experience was largely positive due to an exceptionally responsive nursing team. No companion app used — all communication via email and phone.	Emotional Entry Point Agnostic about having children for a long time. A friend's egg freezing journey at 36 prompted curiosity. Long-term relationship ended shortly after. Confronted with the possibility that the choice could be taken from her. "I don't feel good about that. I still want the choice." Decided to freeze to preserve optionality, not from a place of certainty	Pre-Procedure / Clinic Selection Chose Dr. Hugo Fernandez entirely through a trusted friend's personal referral — friend had a great experience with him. Did not compare clinics. Word of mouth was the entire selection process. Chose Australia over the US for cost reasons — total cost approximately \$3,000–\$5,000 AUD versus tens of thousands in the US.
6	Joan is a founder (Arise) who went through IVF simultaneously while building and fundraising her company over the past 2.5–3 years. PCOS diagnosis. First pregnancy ended in miscarriage at 13 weeks in a hospital ER waiting room alone during COVID. Four egg retrievals, four transfers — first ended in miscarriage at 6 weeks, second chemical, third and fourth flat failures. Currently paused. Considering gestational carrier. Has embryos banked.	Emotional Entry Point Did not anticipate complexity. PCOS was known but low-concern at first. Got pregnant quickly on timed intercourse — felt reassured. The ER miscarriage at 13 weeks alone during COVID was the rupture point. "It was just a terrible, terrible experience." Shifted into type-A control mode immediately after.	Pre-Procedure / Clinic Selection Saw 7–8 doctors after first miscarriage trying to understand what went wrong. Eventually found an OB proactive enough to run a recurrent pregnancy loss panel. Found a reproductive immunologist independently. Each specialist owned their domain and none coordinated.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
5	Lydia is a 41-year-old book publishing executive (14 years at Penguin Random House) living in the West Village, now 25 weeks pregnant from her first embryo transfer. Her journey spanned 5+ years and 4 clinics. Started egg freezing at NYU at 35 during COVID ("I was a little bored, my insurance covered it") but abandoned after five appointments when numbers came back too low post-birth-control. Restarted in 2024 with new husband, at 38. RMA was first due to faster response time but the experience was so poor they switched to Cornell. Cornell + Dr. Irani was significantly better but still had serious infrastructure problems. Describes IVF as "the hardest thing I've ever done." Naturally conceived and miscarried in March 2025 between retrievals — unprocessed trauma that compounded the emotional weight of subsequent cycles. Found meaningful therapeutic support only after switching to Kala Collective (reproductive health specialty) late in the process.	First attempt was casual — age 35, pandemic boredom, insurance coverage. Second attempt was urgent — OB told her at 38 "you're not young, you need to get your IUD out tomorrow and call a fertility specialist." That framing of urgency shaped the entire subsequent experience. Pursued IVF primarily because her husband was "passionate about having kids" and "the statistics are obviously just against us" at advanced maternal age. Carried shame about AMA throughout — "I was kind of insecure and a little ashamed of the advanced maternal age." Did not disclose treatment to most friends or family in real time, including close ones, due to that shame.	Pre-Procedure / Clinic Selection: First clinic (NYU, 2020) selected because it was covered by insurance — no deeper research. Second clinic (RMA, 2024) selected because they responded faster than Cornell ("RMA got back to me more quickly") — not due to quality assessment. Third clinic (Cornell, post-first-retrieval) selected because Dr. Irani — originally her OB's referral — had finally called her back. Lydia's own description: "RMA is a private equity owned company. I think their doctors are okay, but their support staff is miserable." Cornell felt "more on top of it" even though processes were imperfect. Had no framework for evaluating clinics pre-selection — speed of response dominated.
2	Nour is a 39-year-old Canadian public health professional now based in Dakar, Senegal, working on a Gates Foundation medication supply chain project. She was living in NYC when she did two egg freezing cycles in 2023 at Weill Cornell with Dr. Alex Melnick. Her first cycle was triggered by two things hitting at once: approaching 35 ("geriatric pregnancy") and a significant heartbreak in early January. She did a second cycle 8 months later, partly because she was disappointed with her 13-egg first outcome and partly because she'd been laid off and wanted to use her severance-period health insurance before it expired. Total cost: ~\$7,000 per cycle after insurance. Now has 33 eggs stored. Single, still wants to meet a partner and have children, sees egg freezing as "an insurance policy" that gave her a sense of agency in an otherwise out-of-control moment.	Dual trigger. (1) Turning 35 — the "geriatric pregnancy" cutoff was operating in her head as a hard deadline, and the decision was primarily age-driven and evidence-based. (2) A major heartbreak in early January. The heartbreak was the catalyst that turned abstract consideration into action. Her frame: "I'm not going to meet anyone in this state that I'm so sad... I had to do something where I felt like I was in control and I was just not the recipient of someone who was rejected." Egg freezing was the thing she could DO that took her out of the passive position of being rejected. Classic agency-seeking behavior in the aftermath of loss. Had previously made one attempt in July 2021 (just an ultrasound with Dr. Irani at Cornell to check ovarian reserve) but abandoned it for insurance and medication reasons. Switched to Weill Cornell for the second attempt.	Highly rigorous, research-driven selection process. Her frame: "I went very nerd on that." Her public health training informed a clear conviction: "The best health care is always in teaching hospitals. Private clinics — the incentives are completely not the right ones and definitely not in favor of the patient." Automatically ruled out RMA and other non-academic centers. Short list was Mount Sinai vs. Weill Cornell. Chose Cornell based on reputation + a friend's tip. Chose Dr. Melnick specifically by reading her academic publications ("are they board certified? What did they study? Did they have a specialty in it?"). Additional validation came from learning Dr. Melnick herself was a single mother by choice — Nour read that as "huge endorsement" that the doctor personally understood what she was about to do. No waitlist barrier. Scheduled around her birthday (before 36) and a planned trip to South Africa.

# Cycles	Overview	Emotional entry point	Pre-Procedure / Clinic Selection
3	Julie is a 44-year-old mother of two in Northbrook (suburban Chicago), Northwestern alum, with a background in medicine and law. Her journey is one of the most complete in the research dataset — it spans cancer-triggered fertility preservation consultation, IUIs at two clinics, two successful IVF retrievals, two successful FETs, and a post-completion phase that no one in the clinical system is equipped to support: what to do with the remaining embryos. She came to the interview not just to share her experience but to surface the "end of the process" question, which she correctly identified as a category-wide blind spot. Her journey began in 2016 at age 35 with a thyroid cancer diagnosis, newly married, with no time to plan. She lost her job the same year. Her professional background — medical and legal — gave her unusual literacy and leverage across the system, and she still found the experience brutal. Both her children were conceived through IVF at Northwestern	Fall 2016 — thyroid cancer diagnosis shortly after getting married. Her first contact with a fertility clinic was a preservation consult at Northwestern before her thyroidectomy to evaluate whether radiation treatment would necessitate egg freezing. Told preservation wasn't required. Did not need radiation. That year she also lost her job. Trying to conceive was delayed by the cancer. When she and her husband started trying in 2017, they had six months of no success — by that point she was already classified as advanced maternal age. The entry to fertility treatment was heavy with context: cancer recovery, job loss, delayed start, AMA pressure. She had to grieve the version of her life where children came easily and without the medical system in the middle of it.	Pre-Procedure / Clinic Selection: Two distinct decisions. First: her OB/GYN willingly started IUIs "right at 6 months" — at the time this felt like the right entry point ("less injections, less money, less drama"). But the OB/GYN practice infrastructure was incompatible with fertility's timing-driven reality. "They didn't work that way." She needed medications and clinical contact on fertility time, not business-hours time. She left the practice entirely over that experience. Second: selected Northwestern and Dr. Bernardi on a direct referral from her (trusted) OB/GYN. Did not shop around. "I just didn't because I trusted the doctor enough to go where she told me." Important signal: Julie chose her REI by trust transfer, not by independent evaluation. A common pattern worth noting — many women do not comparison-shop clinics even when they have the sophistication to do so, because the emotional load is too high by the time they're making that choice.
0	Overview: Ali is a 27-year-old product manager in San Francisco in a queer relationship (~2 years). She is explicitly pre-procedure — representing a critical category that Carousel's research dataset has been light on: women in the consideration and education phase, before any clinical contact has been made. Colgate undergrad (biology + women's studies double major), former healthcare SDOH company (Unite Us), now a PM at a health tech company whose CPO has a women's neonatal MRI background. Active in queer community organizing in SF. Referred by Maya Keerie via their shared women's-health-baddies network; Maya's friend Evelyn Cox at Johns Hopkins was the original connection path. This interview is significant because it captures the earliest phase of the journey — the phase where Carousel's D2C strategy needs to acquire users	Emotional entry point: Triggered by multiple converging inputs: (1) her age cohort reaching fertility-consideration years (multiple friends going through egg freezing, queer friends thinking about family planning without partners), (2) a close college friend going through "pretty severe endometriosis surgery" and a difficult fertility journey ("it just boils my blood"), (3) being in a ~2-year queer relationship that is becoming more serious with longer-term financial planning conversations, (4) personal signal that she got her period at 17 and wonders whether she's "probably infertile" — though no clinician has ever proactively assessed this. Her emotional state is anxious discovery, not active crisis. The word she used unprompted: "I'm scared to start "	She has not started. She describes her current approach as "taking a backseat" — learning from friends' experiences (good, bad, ugly), exploring financial options, and doing word-of-mouth research in her SF circles ("where did you receive treatment, what was it like for you"). Blocker #1 is financial: no fertility benefits through her current employer insurance. She does not have a clinical shortlist. She has not been engaging with social media content on fertility because "I'm not quite there in my journey" — meaning the algorithm hasn't started feeding her this content, so she's operating in an information vacuum. Important: she gets no proactive fertility conversation from her own annual gynecology visits. "I've brought this up in multiple annual gynecology visits. I joke to my friends, I'm probably infertile, but who knows? because no one's ever asking, there's no proactive approach to that." This is a structural failure in preventive women's health that happens before a woman ever touches a fertility clinic —

[illegible]

Procedure / process p1		Procedure / process p2	Post-procedure & outcome
Required one month of birth control pre-procedure to prevent cysts during travel. The birth control prescribed was severely problematic — women's reviews rated it 2.2/5, described as causing rage and terrible side effects. She felt agitated, on edge, and emotionally terrible for the entire month before the procedure even began. Nobody had advised her to research the specific birth control — it was just prescribed without discussion. Traveled to Cancun alone, navigated airport rental car scam at midnight as a single woman, drove to an Airbnb by herself. Had a friend coincidentally in Mexico who she met up with briefly. Clinic gave her a WhatsApp number for a nurse — used it throughout and that contact is still active three years later.		Injection instructions delivered in person by a nurse at the clinic — she took a video on her phone to reference. Instructions were in Spanish with English clarification available. Injections themselves were easier than expected — this was not her anxiety point. During the two weeks in Mexico, she simultaneously tried to process the deeper question of whether she even wanted children — was reading a new book on the subject. Found that trying to resolve that question while alone in a foreign country going through the procedure was too much for her nervous system — had to set the existential question aside and just focus on taking care of herself. Responded well to medication — retrieval happened on day 11-12, earlier than expected.	Post-Procedure & Outcome 10-12 mature eggs retrieved. Felt genuinely happy with the result. Post-procedure weight gain for 3-4 months despite being highly athletic (5-7 workouts per week) — described it as "stubborn weight that wouldn't come off." This was the hardest part — not the procedure itself but the aftermath nobody warned her about. The emotional weight of the physical weight gain was compounded by the ambivalence about children and her age. "It felt almost shameful." Relationship with the clinic post-retrieval was minimal for two years — just basic storage. In the past year has reengaged with the clinic extensively as she researches solo IVF — multiple video calls, cost breakdowns, resources shared. Currently pays \$650/year storage.
Once she committed, felt like "a cannon ready to be shot out" — but immediately hit logistical uncertainty. No clear calendar or visual guide — instructions came through phone calls and a portal with PDFs and links. Had to wait for a nurse to call her and was always anxious about missing the call. Kept her phone on loud for two weeks. Communication was primarily by phone and voicemail — she found this frustrating and old-fashioned. Wanted written instructions, not verbal ones. Scribbled down what she was supposed to do from phone calls. Portal was split across two systems — one for clinical, one for billing — which felt like managing two separate inboxes. No app — just a mobile website. Medication logistics were genuinely burdensome: had to travel from Williamsburg to the Upper West Side to Apothorp Pharmacy at 6am, had to do real-time math on how many CCs to pick up to avoid		Monitoring appointments every 3-5 days in the first week, more frequent in the second week. During one visit was seen by a PA rather than her doctor — left the appointment not knowing if things were going well. The PA either wasn't at liberty to say or wasn't communicating clearly. "I left being like, is it going okay? I don't know." That uncertainty was distressing. Results were visible in the portal but the meaning wasn't always clear. Doctor was warm and direct — Lissy genuinely loved Dr. Blakemore and felt held by her as a physician. "My doctor made it feel great — but the process and the operations of this thing felt confusing." The clinical relationship was excellent. The operational experience was not.	First cycle: 13 eggs retrieved. Doctor looked worried coming out of anesthesia — said she expected more. Lissy comforted the doctor. Her uncle later told her 13 eggs at NYU represents a 41% chance of one child — a more conservative estimate than other clinics would give for the same number. Appreciated NYU's honesty but acknowledged many women elsewhere are given unrealistic expectations. Second cycle at 37: different medication protocol, 20 eggs retrieved, 7 embryos including one mosaic ("Slowly"). Felt empowered and accomplished after both cycles. Noted the strange emotional reality: "You go through this physical thing to do a thing without any present at the end — except a promise of a future potential. There's nothing really like that in the
First cycle was cancelled due to a large follicle — had to wait an entire additional cycle, which she chose to delay further to protect her schedule and life. When she did start, her primary anxiety was the injections — not the needles themselves but her own ability to mix and administer them correctly. "I'm not a chemist. I don't know what I'm doing." Had a Sunday night meltdown about it and texted the doctor directly. Doctor was on a plane but responded warmly and invited her to come into the office the next day. Nurses took over her injections entirely — she drove from Silver Lake to Beverly Hills every day at 8:15am. This completely resolved her injection anxiety. Nurses waived the \$50 injection fee. Became close with the nursing staff because of daily contact.		Portal described as "hellish" — anxiety-inducing, slow response times. Had a concurrent bacterial vaginosis treatment and was anxious about drug interactions — waited a very long time for a portal response and found it extremely stressful. Missed a friend's wedding because she couldn't get a timely answer on whether she could drink — just didn't drink to be safe. Rule-follower nature amplified every gap in information. Also coordinated an IUD insertion during retrieval — had to find a new gynecologist, get a prescription, and track an IUD delivery to the Beverly Hills office the day before surgery. "God, this was kind of a hellish process now that I'm remembering all of it." A nurse she bonded with asked to sit in on the retrieval — had never done it before — just to be there for her. This was a deeply meaningful moment.	21 eggs retrieved. Felt good about the number initially — then talked to friends who got 40+ and experienced significant shame spiral. "I felt a lot of shame that I hadn't done it correctly." Later realized high numbers often reflect PCOS, which recontextualized the comparison. Now pays ~\$200/quarter for storage. Concerned about storage logistics — credit card linking, what happens if payment fails, demystifying the storage process. Has become the go-to resource in her own network for friends now going through it — actively helps friends navigate the process.

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
Described the experience as "very business-like and clinical." Key friction points: lifestyle guidance (diet, supplements) was mentioned verbally once but never provided in written form — she had to rely on memory. No written materials provided. No mental health support offered or suggested. Phone trees were difficult when trying to reach a human. Portal was rudimentary and ugly — her exact words: "How are you charging people this much money and you can't pay for a fancy website designer?"	Had to do an injection in the middle of a concert. Had to figure out travel protocols on her own — only found a pharmacy flyer about traveling with medications by accident, after the fact. "This would have been nice — where was this?" Felt the clinic was not proactively thinking about the full picture of her life outside the clinic walls. Described the ideal as "somebody thinking for me and following up — are you taking your supplements, do you have your medication for this week — like a concierge."	Has eggs in storage. Has periodically texted her doctor directly with relevant updates — unusually has the doctor's personal cell number. Doctor responds directly without redirecting to the portal. Clinic has not reached out proactively since retrieval. She acknowledged some responsibility is hers but would have appreciated the clinic initiating a next steps conversation. No formal plan in place for what happens next.
Physical experience was fine — bloating, needle discomfort, nothing severe. No history of birth control or hormone therapy made the process feel particularly foreign. Communication was via email and an online portal — no app. Instructions came via video links sent by the clinic and phone calls with nurses. Had one moment of uncertainty about whether she had correctly loaded a medication into the needle — called the clinic and was reassured. Found instructional videos helpful. Had a coworker who had been through it who gave her spare medication when she ran short. Described the overall process as having "an overwhelming amount of information thrown at me" that she had to manually track and remember.	The in-clinic experience with Dr. Fischer felt efficient but not personal. "She knows the script by heart. She has another patient to go to." Nurses were warmer than the physician. Felt a little rushed at times. Her boyfriend at the time — now her fiancé — was so new in the relationship that she didn't feel comfortable asking him for help with injections. Managed entirely independently by choice but in retrospect wishes she had been more emotionally vulnerable and asked for more help. Key realization: "It felt more like a process versus an experience." This is the most important quote from this interview.	Significant post-retrieval emotional crash — described as an extreme version of PMDD, potentially triggered by the trigger shot creating hormonal chaos in a body that had never been on birth control or hormone therapy. Lasted approximately 24 hours. Researched it on Reddit and online after the fact — clinic had not warned her this might happen. Friends she shared this with had not experienced it. Happy with her outcome — double digit eggs, no second cycle needed. Currently storing at Spring, Meta coverage expires December 2026. Facing a storage decision: pay for a 10-year plan or not, given she's getting married next year and turning 30. Recently attended a talk by a functional medicine hormonal doctor who argued against elective egg freezing for younger women without clinical urgency — "you're wreaking havoc on your hormones." This made her genuinely question whether she would have made the same decision with that information. "I
Overall experience was surprisingly positive. Clinic made her feel welcome from the start. Described a genuine community feel among nurses and staff. Physical symptoms: extreme fatigue, significant bloating, breast enlargement. Did not experience the moodiness she feared. Maintained a modified workout routine throughout by doing her own research. Went to appointments every other day — found this manageable. Notably comfortable with the process — maintained her professional life, told her team at work, kept going. One significant operational frustration: different nurses at every appointment. She wanted continuity of care — the same person who knew her case, not whoever was available. "It would help women feel safer if you got the same people all the time."	Communication with the clinic via a web messaging portal — responsive, including an emergency contact line. Had one 10pm panic when she administered the trigger shot incorrectly — called the emergency line, got through within minutes, felt genuinely supported. Also asked nurses at morning appointments to administer her injections for her rather than self-injecting — they did it without question. Kindbody's portal showed charts and visualizations of follicle counts, hormone levels, egg retrieval projections, and probability calculators for number of children — she found this extremely valuable. These visualizations influenced her decision to do a second cycle. Complained that the messaging system required logging into a website rather than operating like iMessage or WhatsApp — unnecessary friction.	First retrieval: high egg count — exact number not stated but described as unexpectedly high. Was in significant pain and couldn't walk the day after. Was not prepared for the severity of post-retrieval pain. This was her most negative moment — not from a crisis standpoint but from a lack of preparation and lack of real-time support in that specific window. Decided to do a second cycle independently — not on medical advice. Now has approximately 40 eggs frozen. Describes herself as "at peace" because of the high count. Does not feel urgency to engage with the clinic until she's ready for IVF, which is not currently on her horizon. Prefers to initiate that relationship herself when the time comes rather than receive outreach from the clinic.

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
Had to go in every few days — found the frequency genuinely annoying. Described the total time commitment including prep and recovery as "easily six weeks" even though the active cycle was only about 10 days. This surprised her — had not fully accounted for the logistical footprint. Was on birth control for a month beforehand to sync her cycle. Described the clinic environment as "not the prettiest space in Beverly Hills" but "fine — straight down the fairway." No strong positive feelings, no strong complaints. Interacting mostly with nurses rather than Dr. Womboch. Instructions were not the clearest — had moments of panicking about whether she'd mixed medication correctly. Had a portal but "it was so s*****, I don't think I really used it." Used Alto Pharmacy's videos more than the clinic's own materials. Described Alto Pharmacy as the unexpected highlight of the entire experience — clean, educational, sent a caramel with every order. "That was probably the highlight of the entire experience was my little Alto everything else sucked."	Had friends give her practical tips that the clinic never provided: wear a dress to appointments so you don't have to take off pants and shoes for every ultrasound. Buy a little tray to corral all medications in one place. These small peer-sourced tips meaningfully improved her experience. Was lucky to have a socially supportive network — friends sent flowers and gifts around retrieval. Mom came with her to retrieval. Had no 2am panic moments of needing answers — mostly assumed she had done things correctly when uncertain rather than seeking confirmation.	Retrieval was fast and smooth — described as the easiest part. Retrieved approximately 40+ eggs despite only ~30 follicles — body responded exceptionally well to the drugs. Was confused by the discrepancy between follicle count and egg count coming out of surgery. Days 2 and 3 post-retrieval were extremely painful — described as nearly crying from discomfort, severe bloating, had not been prepared for this level of pain. Took about a month to feel fully back to normal though the acute pain subsided after a few days. Felt resentful in that window. Clinic was later involved in a lawsuit claiming they lost someone's eggs — saw it in TMZ, caused anxiety about her own eggs. Now has ongoing low-grade uncertainty about whether to check on her eggs periodically. Paying storage fees with "little clear insight into that process."
Cycle one — December 2025. Cancelled on day 11. Dominant follicle blocking cohort development on one side, asynchronous follicle growth on the other. Was not prepared for cancellation as a possibility. Learned the news from a doctor she happened to know socially — who told her things weren't looking good. Had to wait two more days on altered medication before final cancellation decision. Emotionally very hard, compounded by year-end timing, the breakup, and the hormonal state. Cycle two — January/February 2026. Different protocol, different lead-in. Medically unremarkable. Monitored daily during a major ice storm. Seven eggs frozen. Big relief. This was what she had imagined the first cycle would feel like. Cycle three — Shortly after. Approximately six eggs frozen. Mostly saw ultrasound techs rather than PAs and doctors — less information volunteered. Had medication refill complications related to the ice storm and pharmacy delivery logistics.	<u>Portal and Communication</u> Shady Grove used a web-based portal — not an app. Described as glitchy and annoying — she compared it unfavorably to One Medical's messaging platform, which she used as her benchmark for good healthcare communication UX. Daily monitoring results were called in — if she answered the phone they wouldn't add it to the portal in writing, only if she missed the call. Blood work numbers were posted daily with reference ranges. She clicked through but didn't find the reference ranges particularly meaningful without context. By cycles two and three she used past cycle data as her own reference point — a workaround the clinic didn't design for her. Messaging was available 7am-4pm with roughly one-hour response time. After-hours had a callback service through a separate secure portal messaging system — not phone — which she found less reassuring and wondered if after-hours messages fully connected back to her main care team. Financial and medical communication were mixed in the same portal,	Approximately 13 eggs total across two successful cycles. Has decided not to do another cycle — for her body, her sanity, and other factors. Her doctor set an expectation early that she would need approximately 20 eggs for an 80% chance of one child. She internalized this as a goal. Not reaching it is hard for her as a goal-oriented person even though her doctor has not pushed her to do another round and has used language like "when or if you want to use these." She knows rationally that any number is better than none but still struggles with the feeling of not meeting the target. Wants to know where her eggs are, what the process looks like when she's ready to use them, and how much lead time is needed. Doesn't know yet what she wants to do and wants information available on her terms when she's ready — not pushed at her.

Procedure / process p1		Procedure / process p2	Post-procedure & outcome
The Process — Four Cycles Her body has been consistently resistant to stimulation — she has many follicles visible on her ovaries but the artificial hormones do not cause most of them to grow to retrievable size. This has been the central clinical puzzle she and Dr. Cannon have been working to solve across multiple protocols.Cycle one — successful. Retrieved eggs, created one healthy female embryo. Now frozen.Cycles two and three — cancelled mid-cycle due to insufficient follicular growth. Not enough mature eggs to justify retrieval.Cycle four — January 2026. Most egregious experience in the dataset by a significant margin. Progyny changed their policy at the start of 2026: if a patient has had multiple cancelled cycles, Progyny will not approve future cycles unless the current cycle goes all the way through to retrieval — regardless of whether the retrieval is medically warranted. Rory and Dr. Cannon		The Portal and Technology RMA's portal is described as "a hellscape" and "almost unusable." She has had no access to her own cycle data — no follicle size charts, no hormone trend graphs, no historical comparison across cycles. For a product person who is technically sophisticated, this is a particularly stark gap. She has been going through one of the most medically complex journeys in the dataset with essentially no data visibility into what her body is doing. All data stays with the clinic.	
Cycle one — two eggs retrieved. Left the clinic hysterically crying. The clinic offered a complimentary second round. Cycle two — two eggs retrieved again. Total: four eggs frozen. Was placed in post-retrieval recovery next to a young woman who had retrieved an extremely high number of eggs and was audibly celebrating. Aviva could hear her through the curtain. This is the most viscerally painful clinic experience in the dataset — an entirely preventable harm caused by careless patient placement in recovery.Instructions were unclear — she accidentally administered medication incorrectly during the first cycle, which may have affected outcomes. Could not self-administer injections — her friends came to her home every night to administer them after watching YouTube videos. The clinic did not offer in-clinic injection support.Physical experience was actually mild — no bloating, no significant fatigue, no side effects. The physical ease made the emotional difficulty of the low yield even more disorienting. She had expected more eggs partly because her mother had conceived naturally at 42 and her sister was reportedly fertile into her 50s.		Communication and Portal Portal was functional — could send messages and receive responses promptly. Phone number was accessible. This was not a major pain point for her. Communication was adequate. The deeper failures were clinical, human, and structural — not technological. Financial Reality This is the first participant in the dataset for whom cost was the primary barrier and the primary driver of every decision. She chose her clinic based on a Black Friday deal. She borrowed money to pay for it. She cannot afford additional cycles. She described cost as the single thing she would change about the	Post-Procedure and Current Status Four eggs frozen. Cannot afford more cycles. Deeply uncertain whether she will be able to have children. Approaching her 40th birthday in May — describes this as the most depressed she has ever been for any birthday. Feels scared she is "doomed." Actively in grief for her mother, her family rupture, her isolation, and the possibility that motherhood may not happen for her.

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
Eight days of medication. Clinic called in one day's worth of medication at a time to minimize out-of-pocket cost — she flagged this as a conscious, thoughtful effort by Cornell on her behalf. Went in every other day at 7am for blood draws and sonograms. Two locations — Tribeca (close, easy) and uptown for scans with Dr. Melnick directly (slightly inconvenient but appreciated the access). Trigger shot on day eight. The morning monitoring was factory-like by design — every woman cycling at the same time came in between 7 and 9am. She didn't mind the factory flow — she actually found it oddly comforting to see other women in the same situation. But she noted: the clinic knew exactly who was cycling together, and nobody ever connected those women. "They knew there were four other women doing the same procedure on the same three-day span as me. Like — why isn't someone doing something with that?"	Communication was handled almost entirely by a single assigned nurse named Molly — via the Cornell portal app, but Shira experienced it as direct email because Molly was the only person ever responding. This felt seamless to her. No confusion about who to contact, no rotating strangers, no dropped messages. "It wasn't like I heard Molly at Cornell. She was just the one responding to me." She described the communication as dramatically better than what she had heard from others — and she works in healthcare PR, so she has a reference point. The single point of contact model is what made it feel human rather than institutional. Medication questions were answered over email, not through a portal queue. She had no midnight panic moments — but she attributes this partly to having a healthcare-adjacent professional network, not to the clinic's design.	Retrieval day was the hardest logistically — long wait, couldn't eat, didn't go in until 3pm. Physically fine afterward. Retrieved 19 eggs, all viable. Objectively exceptional result. And then the emotional crash hit. Four to five days of what she described as out-of-body emotional fragility — crying at commercials, not recognizing herself, unable to regulate. She was aware of it happening, watching herself from a distance, unable to stop it. "Part of me was sitting over my shoulder watching me be like — pull it together, this is a commercial." Nobody warned her this was coming. The hormonal withdrawal was harder than the hormonal build-up. The clinic did not reach out after retrieval.
The pre-journey delays were the defining frustration of her experience — not the procedure itself. From the moment she decided to move forward to actually beginning her first cycle took approximately nine months. The reasons: a month-plus wait for a first appointment, discovering she needed to be off birth control for several months (she had been on it for 20+ years), three preliminary ultrasound appointments costing nearly \$1,000, and a concurrent bladder/vaginal surgery that pushed the timeline further. Nobody coordinated across these moving parts. She was left to manage competing medical opinions on her own. "I didn't feel like I had someone to advise me in that moment." She had a urologist saying he didn't know enough about egg freezing to advise. She had a fertility doctor saying she could probably proceed but might want more healing time. No one took a coordinating view. She made the decision herself and delayed — which she believes was right but experienced as stressful and lonely from a navigation standpoint.	Once in the actual cycle, Spring performed well. Good appointment availability, efficient in-and-out morning monitoring, reasonable communication through the app. The Salve app was functional but basic — she compared it unfavorably to the One Medical app, specifically calling out poor navigation, unclear labels, and a document system that required downloading PDFs rather than surfacing information in a digestible format. She used the word "gross." The medication coordination was the single worst part of the active cycle experience. The initial Alto Pharmacy quote for her medications was \$6,000–\$7,000. She independently discovered a self-pay rate at MDR Pharmacy, spent weeks going back and forth between the two pharmacies and the clinic, mixing and matching Carrot rates and self-pay rates to optimize costs, and ultimately got her medication cost down to approximately \$3,400 — nearly half the original quote. Spring provided essentially no guidance on this. The nurse sent the prescription. The finance coordinator knew about clinic costs only. Nobody told her the arbitrage existed. "They do not make it easy. And spring does not help — like, at all."	Recovery after both retrievals was harder than anticipated. She described the physical recovery as tougher than expected. Post-procedure follow-up consisted of a voicemail check-in from the clinic — and that was essentially it. No structured recovery support, no proactive information about what the next few days might feel like, no follow-up pathway. She has leftover medication from her cycles and mentioned wanting to be able to sell or donate it — Spring allows donation but does not facilitate peer-to-peer resale or matching. She flagged this as an obvious gap. On round two she exhausted her Carrot benefit and is now on Spring's payment plan — \$10,500 over 2.5 years in monthly installments, at a rate approximately \$1,500 higher than the Carrot plan rate. Medication costs also increased between cycles for no explained reason. The financial experience of round two was meaningfully worse than round one.

Procedure / process p1		Procedure / process p2	Post-procedure & outcome
KINDBODY — ROUND 1 (age 36-37) Positive. App was clean. UI/UX was good. She was among the first 500 users. Price was competitive. She was in a dating relationship and emotionally unattached to the outcome. Experience was fast, easy, unremarkable. Retrieved approximately 9-10 eggs. No significant complaints.		KINDBODY — ROUND 2 (age 39) Significant deterioration. Kindbody had scaled aggressively — picked up Walmart and other large corporate clients. Doctors leaving. Retention issues all over the internet. Her records weren't kept up to date. Passed from nurse to nurse. Scheduling was difficult. Double billing began. She was leaving the US mid-cycle — traveled back specifically for retrieval. Retrieved approximately 9-10 more eggs. Tolerated the experience because she got through it, but the cracks were visible. IVF PHASE — THE WORST EXPERIENCE IN THE DATASET FOR OPERATIONAL FAILURE This is the most operationally catastrophic account in the entire dataset. Key failures: Undisclosed nurse charges: \$250 per 30-minute nurse call, never disclosed upfront. She discovered it on the bill. Every question, every clarification — charged. She escalated repeatedly. Doctor left mid-journey: Same doctor she'd been with left Kindbody. Handed off without meaningful transition. Cross-border complexity with zero support: Doing IVF	One viable embryo from the IVF cycle. One successful pregnancy. Daughter born on her wedding anniversary, 10 weeks old at time of interview. The moment pregnancy was confirmed: "See you, clap my hands, see you later. No one followed up." The only contact from Kindbody was an email asking "Did you give birth?" Her response: "At this moment, there's no way in hell I'm going to make a referral."
Partner did not respond well to IVF drugs — low egg count and poor quality due to age. Clinic communication was poor: calls went unanswered, nurses were the most helpful touchpoint. Felt like "a piece of meat in a meat factory." Spent approx \$145/minute of doctor face time based on total cost vs. contact hours.		After 7+ failed cycles, a visiting American embryologist introduced a new petri dish media protocol — embryo arrest stopped and they got 6 viable embryos from the final cycles. This was never proactively identified or explained by the clinic. It was a random visiting clinician. The information gap almost cost them their family.	Two children. Eight embryos still in storage — have not been tested. Considering a third child but exhausted. Nearly died in both childbirths. \$200K in debt.
First cycle cancelled — follicles didn't respond. Was completely unprepared for this outcome — "it didn't even occur to me that the cycle wouldn't work." Devastating. Described not knowing in advance what would trigger a cancelled cycle as the core failure. "They're worried about hurting your feelings so they don't prepare you for the worst case." Wanted all scenarios in advance — even if scary. Second cycle produced the embryo for her son.		Medication management was the logistical nightmare — out-of-pocket costs meant she couldn't buy excess meds, ran short, had to drive to Denver (1.5 hrs away) mid-cycle. Alto pharmacy used but sometimes required in-person pickup. Husband deployed for every round — was at every monitoring appointment alone. Formed deep bonds with nursing staff and ultrasound techs. Saved the voicemail of the nurse telling her the embryo was chromosomally normal — "you can hear the joy in her voice."	Embryo for son — chromosomally normal, transferred via surrogate. Boy due May 2026. Daughter age 4 from natural pregnancy. 5 complete IVF rounds documented.

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
People were great. Tech was not: had to access portal via desktop only, 2FA login constantly, icons with no labels. No app version. Daily monitoring visits were convenient (clinic near Trader Joe's — always stacked them). Had a 24-hour call line she used. Had a pharmacy partner (priced competitively). Video instructions for medication were okay. Wished she could FaceTime a nurse one-on-one for injection night. Doctor drew pictures to explain the funnel — appreciated the visual but wanted a video version.	Anesthesiologist called night before and asked about her job (cannabis industry), then lectured her about smoking. She was already on edge — heart rate went up. This affected her experience going in. Gave feedback to Dr. Bower immediately post-retrieval and it was received well. NYC cycle: sipped water when arriving and nurse threatened to cancel surgery — another avoidable pre-op anxiety spike caused by poor communication.	Two cycles completed. Has eggs frozen from both. Now 39, reassessing what comes next. Considering: solo IVF with donor sperm, IUI, embryo freezing. Mom is supportive ("let's go to the sperm bank"). Actively researching options — reading Reddit, talking to women who have done solo IVF.
Text-based nurse communication was highly valued — could message any time, nurse responded. Was a comfort anchor throughout. OHSS scare: heard horror story from coworker (two-week hospitalization) while already in stimulation. Went into spiral. Nurse's response was "doctor coded" — legally cautious, not friend-coded. "They're protecting themselves so they can't say you don't have it." Wanted to hear: "these are the chances, here are the early signs, it's normal to be scared."	Recovery harder than expected — "people said I went back to work the next day." She and a friend both had difficult recoveries and were neither warned nor prepared. The variation in recovery experience is not explained to patients — no one tells you that some people need 2-3 days. Information existed about symptoms but not about emotional experience or the range of physical outcomes.	37 eggs frozen. One cycle. No plans to use yet. Still has unanswered questions: how long are eggs viable? What are the drop-off rates? What does she need to monitor about her own fertility going forward? Describes eggs as "an insurance policy — but I have no idea what's covered."
Round 1 at 33: straightforward, great outcome (16 eggs). Round 2 at 38: bad nurse experience — nurse could not find vein, kept sticking her, she passed out. Medication logistics overwhelming — told casually she might need more meds mid-cycle with no clear timeline. Calendar-driven personality found uncertainty deeply frustrating. Wanted a sample timeline: when going in daily, when to order extra meds, when she would feel bloated, when she could not exercise.	IVF at RMA NY: communications were very poor. Scheduling chaos — conflicting appointment times (7:40am vs 10am in same message thread), husband rearranged work meetings for wrong time. Multiple different staff communicating with no single point of contact. Mix of portal, nurse calls, and emails with no central source of truth. Financial information presented as confusing printout — this is probably insurance, this probably is not — overwhelming and not digestible. No clear real-time cost tracking or insurance breakdown.	Round 1 (age 33): 16 eggs retrieved. Round 2 (age 38): 4 eggs. IVF: 16 eggs from age 33 converted to 11 healthy embryos. Used highest quality embryo for baby Addie, born December 2025. Plans second transfer (boy) in October 2026. Eggs from Extend stored in warehouse in western Massachusetts. Embryos at RMA NY — \$400-500/yr more expensive but on-site. Paying ongoing storage fees — plans to stop once done having children.

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
First appointment: baseline blood work same day, then a follow-up appointment with Dr. Blakemore the next week featuring a 45-page PowerPoint covering all scenarios. Hour-long consultation — not rushed, calming, described as "almost like a yoga studio." Dr. Blakemore presented a custom case study of a patient with identical profile (long-term BC use, low AMH) who waited and saw exponential improvement. Ultrasound on the same visit showed 2 follicles. Pauline was frustrated and wanted to start immediately. Dr. Blakemore gave her the option to proceed while being transparent about likely poor results. Pauline ultimately decided to wait but then had to expedite due to new job.	Expedited into the cycle before coming off birth control for a full month — a deliberate gamble. Called when period arrived, came in the next morning at 8am, blood work and ultrasound confirmed starting dose. Injections: not pen-based as expected — required mixing diluent, which was surprising and stressful. First night, mid-injection, didn't know the right dose for the Ready Ject pen, called the clinic, was told they'd call back — 10-minute wait while mid-injection. Alto Pharmacy experience was exceptional — lived a block away, called ahead mid-process, meds were ready on arrival. Cycle ran 15-16 days, longer than expected because eggs were slow to grow. Monitored via serial ultrasounds — techs would not share results ("Dr. Blakemore will call you"). On third ultrasound the tech mentioned 7 follicles per ovary. Final result: 14 mature eggs retrieved. Dr. Blakemore called personally to deliver the result.	14 mature eggs — dramatically exceeded the 2-3 she was expecting. Described feeling "like she could cry" when she got the call. Post-retrieval: had to arrange her own ride home. Friend came to pick her up but didn't understand the recovery — suggested walking, didn't grasp the physical limitations. Had a bachelorette the same weekend — friends didn't understand she shouldn't be dancing. Felt isolated and unsupported in the post-retrieval window, not by the clinic but by her social support system's lack of education. Now in the "limbo" storage phase — starting a new job, eggs frozen, no clear next chapter articulated by the clinic.
RMA described as a factory — 15-20 women in the waiting room every morning. Waited an hour minimum every visit. Never saw her doctor once during the entire cycle — doctor reviewed labs remotely. All communication via random phone calls from nurses at unpredictable times. If she missed the call, she missed the update. No portal — instructions came via email. "I was just glued to my phone for that two-week period."	First cycle: 7 mature eggs. Emailed Lucky directly to express disappointment. Second cycle: estrogen priming added. 14 mature eggs from 16 follicles. Outcome delivered over email both times. Did not know to ask about priming on the first cycle — and did not question the doctor because she didn't want to jeopardize the relationship. "I'm not going to piss off my doctor because I still need her for this thing."	Eggs in storage — no contact from RMA post-retrieval. Later discovered storage provider had been acquired by PE and prices raised 70%. Was notified without warning that her eggs were being moved to a facility in Massachusetts. "I didn't sign up for any of this." Currently navigating split cycle logistics at NYU — three different portals, legal documentation for embryo creation, still unclear on storage cost structure for split cycle. Eventually got answers but with great difficulty

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
Procedure / Process P1 Did four IUIs back to back at first clinic with zero protocol changes. Only learned from a community message board — not her doctor — that repeating the same protocol was inappropriate. "I just had no idea what options existed." This is when she switched clinics.	Procedure / Process P2 First IVF retrieval got six embryos. Did not do PGT-A testing — was scared about what testing meant and didn't understand it. First transfer resulted in pregnancy then miscarriage at 6 weeks. Three more transfers failed. Fifth transfer — pregnancy progressed to 20 weeks. Anatomy scan at 20 weeks revealed baby was incompatible with life. Termination at 26 weeks, approximately one year ago. Sixth transfer failed. Second retrieval just completed	Post-Procedure & Outcome Waiting on biopsy results from second retrieval. Emotionally and financially depleted. Husband still job-searching. Self-funded this round. Endometriosis recently diagnosed via ultrasound in uterosacral ligaments — still awaiting specialist guidance on next steps.
Procedure / Process P1 Initial consult with Dr. Hugo: "He was really pragmatic, really warm — basically let me know in a very short session all the things I really needed to know." Got all testing and medication in Australia. Returned to the US with medication. Began injections in San Francisco. Flew back to Australia for scans and retrieval.	Procedure / Process P2 Two back-to-back cycles in Australia. Medication came as pens — no mixing required — which she recognized as less intimidating than what some friends experienced. No companion app — all communication via email and phone. Nursing team called back within 30 minutes consistently. "I just felt really supported." Communications were consistent with no major contradictions	Post-Procedure & Outcome Eggs frozen and stored in Australia. Returned to San Francisco one week after second retrieval. Currently in the storage years phase — no active plans to use eggs. Has not had proactive outreach from the clinic since.
Procedure / Process P1 Nobody at the hospital told her to save fetal tissue for genetic testing at the time of miscarriage. "Why did no doctor inform me of this? 99% of women don't know that the first time. That's just so stupid." hCG took 3 months to come down — original OB didn't flag this as abnormal. A second-opinion IVF doctor discovered she still had retained products of conception in her uterus — dangerous, requiring immediate hysteroscopy	Procedure / Process P2 Four to five hysteroscopies and D&Cs over 2023–2024. Four egg retrievals. Four transfers. OB and REI never communicated directly. She played telephone between them for years. Literally purchased a fax number to coordinate her own records between providers. "At one point I was just playing a game of telephone." First IVF doctor noticed possible adenomyosis the day before a transfer, said she'd call back — never did. Joan had to call the nurse to confirm the cycle cancellation	Post-Procedure & Outcome Currently paused. Considering gestational carrier. Has embryos banked. The journey took a profound toll on her body, her sense of self, and her capacity to lead her company simultaneously

Procedure / process p1		Procedure / process p2	Post-procedure & outcome
Procedure / process p1 — RMA experience: Devastating. Communication was poor ("their spelling and grammar are terrible, they were not on top of their shit"). Bounced between different doctors at every appointment — no consistency. Staff would answer questions by deflecting: "your doctor will tell you later, your doctor will tell you later." She and her husband would go to appointments together in the morning, then split for work — and clinical updates would come via call/email later when they were apart, so they could never process information together. RMA used birth control to time her cycle — Dr. Irani at Cornell later said he never does that because it causes suppression and changes results. RMA's trigger shot was a standard subcutaneous abdominal needle — Cornell used intramuscular. These were not minor protocol differences; they were clinically material. Paid out of pocket which compounded stress. One retrieval, poor outcome, immediate switch.		Cornell's Dr. Irani was dramatically better on bedside manner — "very direct but positive, doesn't sugarcoat, answers thoughtful questions, pragmatic." Lydia and husband would travel across boroughs to see him specifically (Tribeca, Brooklyn Heights on Fridays) rather than bounce between other Cornell doctors — a strategy they now recommend to other women. But Cornell's infrastructure had significant gaps: they communicated verbally over the phone without written follow-up ("Cornell would call and give directions over the phone — I'm a reader, I digest things by reading them"). RMA had a better portal than Cornell but Lydia didn't discover RMA's portal until after she'd left. Cornell's mandatory training session is Monday 11am only, no flexibility for working professionals. Three separate medication/pharmacy near-crises: (1) free Gonal vial from Cornell came empty — discovered at 9pm night-of-shots, had to drive to Fort Greene friend who'd frozen eggs months earlier to borrow medication; (2) Progesterone suppositories for FET — CVS insisted the	Post-procedure & outcome: Four retrievals (1 RMA + 3 Cornell). Genetic testing attrition was severe — "our attrition rate really came from the genetic testing" — each cycle was a "cycle of hope and despair." One normal embryo after December 2024 retrieval. Natural miscarriage March 2025 was devastating and never fully processed during ongoing treatment. Additional retrievals July 2025 and October 2025. First FET November 10, 2025 — success. Currently 25 weeks pregnant. Happy ending but "it like changed me as a person forever." Was trying to figure out alternative routes during the process — asked Dr. Irani about her best friend being a surrogate, wanted to parallel-path additional retrievals with a transfer to save time. Dr. Irani explained why that wouldn't actually save time and talked her through it thoughtfully
Procedure / process p1 — Clinical continuity and the "choose your own adventure" failure: The defining structural problem: she chose her doctor carefully but barely saw her. Dr. Melnick was on maternity leave for part of her cycle. Nour saw a rotating cast of whoever was "on call" that day for blood tests and ultrasounds. She trusted Dr. Melnick was reviewing her charts, but the continuity was theoretical, not felt. Direct quote: "I chose her because I read her papers... I didn't see her a lot." The nursing team was "very good on emails" but the information flow was inefficient — "you had to wait for their calls, voice notes, I was working, I'd miss the call." Information was fragmented across multiple formats: email, phone, voicemail. She described the experience as "very much a do it yourself. It's very choose your own adventure." Why did Cornell send her to American Association of Endocrinologists tutorials for injection instructions instead of having their own? Two different trigger drug options —		Procedure / process p2 — The two-week paralysis + the information-timing anxiety: Treatment "paralyzed" her life for two weeks — 7pm injection timing meant she had to be home early every night, then was too drained to go out after. She took ownership of everything including the injections herself ("I was never going to pay someone to inject me"). She's a self-described Type A perfectionist which amplified every micro-decision: "Am I drawing enough? If I lose that little extra drop, is that going to make it or break it?" The highest-anxiety friction point: dosage changes happen within hours, and she had to wait all day for a nurse to call back confirming whether to adjust. Her framing: "I don't need to wait all day wondering do I need to switch my dosage. It was very much an untimely information flow." The psychological cost of not knowing her own data in real time was substantial. She described	Post-procedure & outcome: Cycle 1: 15 eggs retrieved, 13 cryopreserved. Her reaction was disappointment. The information delivery made it worse — a nurse called to give her the number, not Dr. Melnick, with the offer that "if you want more information you can do a follow-up with your doctor." She refused to pay for another consultation just to have the outcome contextualized. She did her own online research, found the Brigham's egg-freezing live-birth calculator (age 35, 13 eggs = ~70% chance of one live birth), and "felt like a B+ student." Cycle 2: 20 eggs retrieved/banked. Happy with that outcome. Now has 33 eggs total. Clear recovery experience: "I felt like shit more than during. The anxiety kept me in check during. The first few days after, until you get your period, my emotions were all over the place."

Procedure / process p1	Procedure / process p2	Post-procedure & outcome
Procedure / process p1 — OB/GYN IUI experience and the timing failure: Defining pattern: the OB/GYN wanted to help but was structurally unable. "The only thing that matters in this case is the timing, and I can't wait for their convenience." Vignette that captures it: she was on cycle day medication, hormonal, accidentally dropped the pill in the garbage, could not reach a doctor to authorize a replacement, was digging through the trash, begging the pharmacist (who could not help because she wasn't authorized to prescribe), with her entire cycle about to be ruined over one missed pill. No one to call after hours. No triage line. No backup prescribing. Her direct quote: "There needed to have been some alternative pathway to accessing things like the transvaginal ultrasounds, the nurse line, the medication line. Whomever is going to be part of this fertility journey can't work normal business hours." On AI: she was skeptical — "I don't know that AI is going to solve anything unless AI is going to be empowered to prescribe medicine" — but she did see potential in an AI triage layer: "Maybe AI could somehow ping the doctor with 'This is a really easy quick answer, can you respond to this time-sensitive	Procedure / process p2 — Northwestern IVF experience and the nurse relationship: Her Northwestern experience was dramatically better, but for a reason that does not scale: she had an extraordinary nurse. "I felt like she was my bestie by the time my son was conceived proper. Like by the time I graduated from her." The nurse was "in really really responsive, respectful, close communication through that portal all the time." When Julie's pregnancy test was positive, the nurse called her directly: "I couldn't possibly send this as a portal note. I wanted to tell you." That moment of real human connection was what made the entire experience tolerable. When Julie came back for her second child in 2022, that nurse had moved on to a higher role. The replacement nurse was "fine, but wasn't the warm handholding bestie that made the experience more tolerable for a newbie." Julie said explicitly: "If she had been the nurse for the initial go-around, I really would have had a problem." The experience is built on one human's	Post-procedure & outcome: Two successful FETs, two healthy children (son 2019, daughter 2022). Remaining embryos — viable, non-viable, and mosaic — still frozen at Northwestern. The "success" framing the clinical system uses stops at live birth. Julie's post-procedure experience is defined by the unresolved embryo question, which no one in the clinical system is equipped to handle and which is creating genuine existential distress for her and many others. At embryo creation, she signed paperwork that offered three options: destroy, adopt out, or donate to science. She checked "donate to science." It turns out no viable scientific donation pathway exists: "It turns out there is no such thing as donate to science in a lot of places. Studies on human embryos have all been shut down." Northwestern has an internal study but "with strings attached" that she can't agree to — the study wants to tie the cells to her, her husband, and potentially her children's medical records, not
Procedure / process p1 — Not applicable (pre-procedure): Ali's direct experience of the healthcare system is as a patient of women's-health PCPs and OB/GYNs, not REI clinicians. The defining recent experience she shared: a \$400 out-of-pocket bill for a 15-minute women's health PCP appointment about migraines, with no prior price transparency. "I would have never booked that appointment if I knew that." This colors her entire anxiety about fertility treatment cost. She is in an industry-adjacent role (healthcare-adjacent product management, former Unite Us employee) so she has above-average literacy on healthcare marketing inflation — "I'm on the healthcare side. I know that these marketing teams on the healthcare level are inflating their metrics for patient capture." She does not trust clinic-produced content. She wants a third-party source of truth.	N/A	N/A

[illegible]

Emotional experience overall	What would have helped	Current status
Navigated the entire experience largely alone — partner was not supportive, friends were available by phone but not physically present. Most painful emotional experience was unsolicited success stories from well-meaning people — described these as "false hope" and "comparison disguised as support." She actively doesn't want to hear other people's stories — wants to be on her own journey without comparison. Set a boundary with someone who kept telling her success stories and that person never spoke to her again. Has done a three-month coaching program (Kids or Child-Free) working through the decision about solo motherhood — was the only single woman in the group of six, which created a significant gap in relevance. Made the decision in August 2025 to pursue solo IVF — is now in a "one year of sitting with the decision"	Companionship above everything — "having someone with me would have been the number one thing." A cohort of women going through the same process at the same time — specifically matched by age AND relationship status, not just age. "Are you single? Are you in a relationship? Are you married?" — these circumstances make the journey completely different. Peer-sourced clinic research and question lists — women sharing where they went, what they asked, what they learned. A safe space to process real-time moments with people who get it — described wanting to "pop into a group" after the couple on her retreat told her she should be adopting instead of doing IVF, and just hear someone validate her. The more specific and niche the community, the better	42 years old. Eggs stored in Cancun at \$650/year. Has decided to pursue solo IVF — currently in a deliberate 12-month decision-sitting period before acting. Actively researching IVF costs and process with the Cancun clinic. Has been organically building a community herself — posted in WhatsApp groups asking women interested in this conversation to connect, collected 10 email addresses, was considering hosting a gathering at her home. Wants a follow-up call with Sarah — asked for email contact
Described the emotional journey as having "no room for complexity." Society and the people around her wanted her feelings to be black and white — and more white than black. Any ambivalence was misread as sadness about being single. Her actual emotions were more nuanced: pride, frustration, financial anxiety, health-related stress, and grief about uncertainty of outcomes. The most resonant moment: when she told the doctor at her first appointment "I'm glad science has caught up with changes in society, but no one wants to be a 33-year-old woman freezing their eggs" — and the doctor responded with an empowerment speech rather than simply acknowledging what Liesv had said. That mismatch sent her	A single, clear, visual guide to the entire process — not PDFs in a portal, not phone calls she might miss, not two separate systems. Something she could reference at any time that showed her exactly where she was and what was coming next. Written instructions she could refer back to. Medication delivery from the start. A way to message the clinic without having to catch a nurse by phone. Emotional acknowledgment at the first appointment — not a reframe, not toxic positivity, just "I hear you and this is hard." And critically: if the doctor had simply validated her complexity in that first appointment, she would have started a year earlier. The year she lost was caused entirely by a failure of emotional attunement in a single conversation	Married and got pregnant naturally at age 38; due in 3 months with baby girl.
Highly supported by community — describes herself as lucky in this regard. But anxiety was a constant undercurrent throughout. Mom flew in to help post-retrieval but mom has the same anxiety profile and became a source of stress rather than comfort — Sarah had to tell her mom to manage her anxiety "off-screen." Needed a calm, authoritative third party to mediate between her anxious mom and her own anxiety — nobody played that role. Described needing "someone to hold my hand and tell me what's normal and what is actually happening." The medical component specifically triggers her health anxiety rooted in her father's sudden death.	In her own words, the dream platform: a friend says "download this, they'll help you." Then it knows your social network and can surface who in your community has done it and where — "like Hinge but for clinic recommendations." Then once you pick a doctor, someone available to talk through anything at any time — injections, travel, what you can eat, how it affects your life. During the cycle: FaceTime for injection support, real-time answers to questions without waiting on a portal, reassurance that what she's feeling is normal. After retrieval: ongoing support, OHSS monitoring, someone to tell her mom the actual rules so she didn't have to. Storage: demystified, clearly explained, linked and automatic. Her exact vision: "I'm picturing a world where I say to a friend, hey I want to freeze my eggs and she goes, download this, they'll help you."	Eggs in storage at Reproductive Partners Beverly Hills. Paying \$200/quarter. Actively helping friends in her network navigate their own processes. Has become an informal community resource. Offered to stay involved with Carousel development — genuine, not polite.

Emotional experience overall	What would have helped	Current status
Described feeling like she was "in a vacuum." Doing it alone with limited family support amplified this significantly. Felt the clinic did not assess or acknowledge her support system — noted it would be very different for someone with three sisters and a mom versus someone doing it completely alone. Was not offered any therapeutic or emotional support resources. Did not seek out community proactively — too private to post publicly.	In her own words, the ideal platform would: feel like "you're the only patient and they're thinking about every micro aspect of how to offload the thinking, decision-making, and logistics." Specific asks: Proactive supplement and diet reminders in written form Travel protocol guidance before she needed it Tailored medical research and news relevant to her specific situation delivered to her — "don't make me work"	Eggs in storage. Periodically in contact with doctor via text. No formal next steps plan. Age 43 — time-sensitive decisions ahead that the clinic has not proactively raised.
Described the experience as lonely — not dramatically so, but a quiet background loneliness of doing something significant without a community around her. Recognized in retrospect that she could have asked for more help and been more emotionally open. The emotional weight only hit her after retrieval — during the process she was in execution mode. Important nuance: she did not feel she needed support going in, but looking back she would have benefited from it. This is a meaningful insight for Carousel's positioning — some women don't know they need support until they're through it.	Her immediate product instinct: a pregnancy-style app that gives her cycle-aware updates and reminders. "Like the pregnancy app where your baby is the size of a watermelon — but for egg freezing. 'You now have 24 follicles and your retrieval is tomorrow. Your trigger shot is at 9:43pm.'" She recognized in real time while describing this that she had done a lot of manual tracking that could have been automated. Wanted something intuitive she could carry on her phone. On community: interested in peer connection but skeptical of moderated Slack-style communities that feel dead — has been burned by those. Wants lightweight, real participation, not managed engagement. Her most important answer to "if you could change one thing": she wanted the process to be treated with the same level of attention and holistic consideration as pregnancy — not a one-size-fits-all insurance policy pitch, but a personalized conversation that considered her age, life stage, professional situation, and specific	Engaged to fiancé. Eggs in storage at Spring Fertility. Meta coverage expires December 2026 — facing storage payment decision. Getting married next year. Turning 30. Actively thinking about next steps. Not currently planning IVF but thinking about timeline. Gave Sarah her personal cell number and offered to share the landing page with 10 women in her network.
Her most distinctive emotional data point in the entire dataset: she was bothered by other people's negative reactions more than by the process itself. When she told people she was freezing her eggs, they responded with "Oh are you okay? Is everything okay?" — as if she had a disease. She found this emotionally draining. "People are empathetic as if you were sick." She is a vocal advocate for reframing egg freezing as a positive, empowering choice rather than a sad or desperate one. Actively tries to counter negative narratives when she hears women being scared about it. Sees herself as part of a responsibility to shift the cultural narrative.	Continuity of care — same nurse throughout the entire cycle who knows her case. 24/7 access to a nurse who can answer questions within 10 minutes — not necessarily her assigned nurse, but someone who has reviewed her notes. Her proposed model: assigned nurse available 9-5, coverage nurse available 5-9pm who knows the case. Clinical explanations for every restriction — not just "no working out" but "here's why, here's when it applies, here's what you can do instead." This gives women agency to make informed decisions. Better messaging infrastructure — iMessage or WhatsApp-style, not a web portal inbox. Post-retrieval preparation — she was not warned about the severity of post-retrieval pain and needed support in that window. Communication with support person — easier way to update the person picking her up about timing.	40 eggs frozen at Kindbody. Single, not currently in a relationship. Not thinking about IVF. At peace with her situation because of egg count and age. Paying annual storage fees. Will initiate contact with Kindbody when ready for IVF — does not want them to reach out proactively. Offered to share Kindbody portal screenshots — follow up to receive these. Signed up for waitlist and offered to share with her network.

Emotional experience overall	What would have helped	Current status
Entered positive and exited resentful — at least in the immediate post-retrieval window. The mismatch between friends describing the experience as empowering and her own feeling of suffering and resentment was isolating and frustrating. Did not want to be told it was empowering when it didn't feel that way. Feels differently now with distance. Was in therapy at some point but not actively during the freeze. Had strong IRL social support network which buffered the experience significantly. Her most honest emotional summary: "I was so resentful. I cannot believe I have to go through this level of pain because dating sucks."	Cleaner, more educational app experience — cited Alto Pharmacy as the gold standard for UX and described wanting something similar for the clinical experience. Easier scheduling. More educational context for every instruction. Noted that if she hadn't had a well-networked friend group she can see how it would have been "kind of scary to figure out." Had a creative idea during the process: a retreat model — go somewhere like Cabo for two weeks, work remotely during the day, have appointments, be with other women going through the same thing, then do retrieval. "I probably would have paid for that." Wants more transparency and visibility into the storage process — doesn't know how to check on her eggs, uncertain about what protections exist. The TMZ clinic lawsuit story created genuine fear.	Eggs in storage, paying annual fees. Has not used them. Currently in a serious relationship — moving in together, first boyfriend in 12 years. Was direct with him early about wanting to know within a year if they'd be married. Not actively thinking about IVF but aware it's the path if needed. No plans to re-engage with the clinic unless pursuing IVF.
<u>Emotional Closure Moment</u> She wanted someone to celebrate what she accomplished — not console her about what she didn't reach. Specifically: acknowledgment that she did something hard and completed it. And a reframe that the goal was to leave her options open, which she did. The expectation of 20 eggs had been set by the clinic and she internalized it as a personal failure metric. This is a meaningful insight — clinics are inadvertently setting numerical targets that become emotional anchors women carry for years.	One Medical-style messaging — single thread, all in one place, seamless, no wondering if messages sent. A journey or checklist view showing what she had accomplished and what was ahead — her single most important change. More contextual clinical information accessible without needing to Google — she Googled constantly and ended up on Reddit trying to understand whether her symptoms were normal. Better pharmacy coordination built into the platform. Financial and medical communication separated but unified under one umbrella with a clear checklist of what was required when. Clearer expectations about the range of possible outcomes — including cancelled cycles — set proactively, not discovered in the moment.	

Emotional experience overall	What would have helped	Current status
<u>Community</u> Described vetted community as "absolutely critical" and "so crucial to surviving the experience." Found her community organically by being open about what she was going through — her vulnerability invited other women to share their experiences with her. Describes herself as an unintentional community builder. Her criteria for ideal community: women with structured demanding lives and lots of competing responsibilities. Broad diversity of background, access, insurance situations, and outcomes — because there is no reliable way to predict how the process will affect any individual. She wants the full picture, not a curated positive narrative.	What Would Have Helped — Product Vision She articulated a clear product vision unprompted as a technology builder. A single dashboard landing page that unifies all components on one screen: test results, scheduled appointments, medications and refill reminders, cycle schedule, messaging with the care team. Everything visible in one place. No hunting. No fragmentation. She described this as her primary product instinct — not features, not AI, not community — unified visibility and navigation. This is The Journey's core UX concept stated by a product person who has lived the problem.	
	24/7 access to a doctor or nurse. Clear instructions — in person if needed. Support groups with instant messaging. In-person support meetings specifically for women who got low egg counts or poor outcomes. Financial scholarships or access programs. Holistic supplements or integrative medicine support to optimize outcomes. More female doctors. Education about fertility decline starting in her 20s — she said explicitly that if she had known at 28 what she knows now she would have frozen her eggs a decade earlier. The Education Gap — A New Finding She said something no other research subject has said: if I had known in my 20s what I know now, I would have done	Post-Procedure and Current Status Four eggs frozen. Cannot afford more cycles. Deeply uncertain whether she will be able to have children. Approaching her 40th birthday in May — describes this as the most depressed she has ever been for any birthday. Feels scared she is "doomed." Actively in grief for her mother, her family rupture, her isolation, and the possibility that motherhood may not happen for her.

Emotional experience overall	What would have helped	Current status
Notably pragmatic. Did not feel the empowerment narrative the fertility industry projects. Did not feel like she was "taking charge of her future." Felt like she did something necessary and moved on. "I checked the box and now I'm moving on with my life." This is one of the most honest assessments of the emotional experience in the dataset — and it challenges the empowerment-forward brand positioning that dominates the category. She felt lucky, grateful, and relieved. But she did not feel transformed. The industry's reflexive empowerment framing would have felt false to her. Carousel's emotional design must hold space for this register. She also noted her sister went through IVF simultaneously — a much harder journey involving Broca carrier testing — and felt profoundly isolated despite having a supportive husband. Shira's positive Notably less emotionally intense than most participants. She had a long-term partner, friends who had done it, access to a nutritionist and gynecologist through Carrot, and a product marketer's orientation toward process optimization. The emotional experience was characterized more by practical frustration than by isolation or fear. She didn't describe midnight panic moments, she didn't feel abandoned, she didn't feel the experience was emotionally unworthy of its significance. What she did feel was repeatedly frustrated by a system that gave her no coordinating guidance, left her to navigate competing medical opinions alone, buried important cost-saving information, and provided a below-par technology experience. "Don't listen to your male gynecologist who tells you, 'No' — like, go for it." The emotional weight for Savannah was concentrated in the pre-journey delays and the financial navigation — not in the clinical or emotional layers that dominate most other participant accounts.	Unprompted and emphatic: a buddy system. She articulated this entirely on her own — the idea of connecting women who are cycling at the same clinic at the same time. She has pitched this concept to multiple women's health brands in her PR work and been told it's a great idea but not their focus. "Why isn't someone creating a program where those women can be each other's support system?" She also described a "big sibling" model — someone who has been through it recently and can walk alongside a woman who hasn't. Not a therapist. Not a doctor. A peer who has lived it. Her cousin did it after her and she played that role — she described it as meaningful for both of them, but noted most women don't have that person. She referenced a Soho House analogy: when you log into the WiFi, it tells you how many members are in the building. Why can't a fertility platform tell a woman that 40 others are cycling at Cornell this week? She articulated a clear, product-fluent wishlist — consistent with her background. A care coordinator who could take a cross-specialty view of her situation — someone who understood both the egg freezing timeline and her concurrent medical situation and could advise holistically. She explicitly said: "If I had a primary care physician who knew what was going on, maybe they could have advised." But she didn't, and the fertility clinic wasn't filling that gap. Educational content upfront — a pre-journey checklist, onboarding experience, clear what-to-expect framework before the first appointment. She was operating almost entirely in the dark on logistics and timeline until she was already in the process. Better medication navigation — ideally an integration or coordinator that could help women find the best price across pharmacies without spending weeks doing manual arbitrage. She found the savings herself through sheer persistence. Most women wouldn't. A single central point of contact for all communications — she had a nurse, a finance coordinator, an appointment	35 years old. 19 eggs in storage at Cornell. Recently out of a three-year relationship. Not currently dating seriously. Works in healthcare PR and health/wellness communications. Connected to Janice Fox (Head of Endocrinology, Harvard Brigham Women's) through a family friend. Has agreed to make an introduction to Dr. Alexis Melnick. Offered to stay involved and help think through PR strategy for Carousel down the line. Warm connection via Hoffman Process community — genuine mutual warmth from the call. Two rounds complete. Eggs in storage at Spring Fertility SF. On payment plan for round two. Long-term partner. Ambivalent about having children — froze for optionality. Enthusiastic about Carousel — signed up for landing page, offered to share with her network. Younger sister has not yet done it — Savannah is already advising her. Works in product marketing — potential future resource for go-to-market thinking. Warm personal connection with Sarah from prior relationship.

Emotional experience overall	What would have helped	Current status
The experience was one of multiple concurrent crises: active divorce, absorbed work role, devastating outcome, clinically cold follow-up. Each element amplified the others. The failure was concentrated entirely in the post-result call — the single moment the clinic's support was most critically needed. The pre-procedure experience at RMA was actually positive (thorough onboarding, responsive coordinator, good educational materials). Everything collapsed at the worst possible moment. She had friends nearby for support, but her parents had moved to California the prior October. "Even talking to you, I'm realizing how lonely this was." The cumulative experience led to quitting her job.	Two clear unprompted asks: (1) A wholesome approach that incorporates mental health. "There was not one single moment where I was asked how my mental health was." A daily or periodic mood tracker with a responsive mechanism — AI or human who notices and reaches out: "Hey, I noticed you said you weren't feeling great today. What can we do?" (2) Access to other women cycling at the same time. She described the waiting room — surrounded by women, siloed, no mechanism to connect. She specifically named integrating Calm or meditation functionality, and a daily feeling tracker. She also validated the pre-clinical product hypothesis entirely unprompted: "The experience needs to start so much earlier than the moment you go to the clinic." Her response when Sarah described the Carousel concept: "Thousand percent."	Not employed. Cannot afford a second cycle. No eggs in storage — no mature eggs were frozen. Now 38. Expressed openness about the future but no clear path: "We'll see what happens." Wants to support Carousel; offered to share the survey widely when it's ready.

Emotional experience overall	What would have helped	Current status
	Articulated clearly and strategically — she thinks in business frameworks: Pricing transparency: The menu model doesn't work. Package pricing makes sense. Women shouldn't be counting the minutes of nurse contact. Community built inside the platform: "Building a community even within the company probably makes more sense than having people create them outside and use Facebook and Reddit as a space to complain." AI-powered education: People are already searching. Surface accurate information proactively rather than letting them find false information elsewhere. Clinical founders who are genuinely passionate — not names: "I would get my doctors in front of it. I probably would make sure there's a doctor or two in the founding team that were actually passionate about it, not there as a name." Longitudinal continuation: The platform should follow through after birth — partnerships with Huckleberry, Ovia, The Bump, prenatal fitness platforms. The journey doesn't end at retrieval or even at birth. <i>Verified clinic finder with cost transparency and reviews</i>	Married, works in robotics, living in Canada, just had her first baby daughter 10 weeks ago!
Did not feel in control at any point. Felt like a number. The only sense of control came from self-educating via peer communities and advocating for protocol changes. Doctor was accommodating of patient-driven protocol requests — "you can do whatever you want, I don't care" — but only because the patient had done the research herself.	Personal data ownership and visualization — ability to compare drug protocols across cycles and identify patterns. "Having the ability to look at my data and then kind of make my own protocol." Advocacy-ready summaries she could bring to doctor appointments. Real-time decision support rooted in her own history, not generic advice.	40 years old. Two children ages 4 and 2. Eight untested embryos in storage. Considering whether to do another transfer but physically and emotionally exhausted. Nearly died in both deliveries — significant fear around returning to the process.
"The moment I realized I was not in control" was the hardest. Type-A high achiever who had never encountered something she couldn't work harder to fix. Follicle size doesn't respond to effort. Cycle-by-cycle emotional whiplash — up on day 6, cancelled on day 8. "By far the hardest thing I've ever done." IVF harder than childbirth. Loneliness was profound — husband deployed, friends supportive but "you don't get it unless you've been through it."	A sponsor/buddy model: "like an AA sponsor — someone who's been through it who I can text at any time." Not a medical provider. Not a therapist. Someone who knows the emotional side from lived experience. "Is this normal?" "I feel like killing my husband today because my hormones are through the roof — talk me off the ledge." Phase-based information delivery: by cycle phase, by scenario, by decision point. Different learning style support — she doesn't retain by reading, needs to talk things through. Community of people at exactly the same phase of the journey.	Surrogate pregnant with boy, due May 2026. Daughter age 4. Just moved into Chief of Staff role at her company. Richmond, Virginia.

Emotional experience overall	What would have helped	Current status
Throughout: had to reframe the experience positively herself. Was influenced by a friend's ritual: light a candle, play a good song, treat yourself like a goddess before injections. "Connecting with the little eggs as they developed." This spiritual/emotional reframing was self-generated — the clinic provided zero of it. Described the loneliness of doing it as a single woman in waiting rooms full of husbands.	Emotional reframing and ritual support — "the mental spiritual side of it, the cheerleading." Access to her own data and past protocols without having to message someone. A video explaining the basics of egg freezing upfront. The funnel math — understanding her specific numbers and what they mean for her chances. Proactive upsell/education about embryo freezing at the time of egg freezing — "I wish they'd tried to upsell me embryos." Real-time FaceTime nurse support on injection night. Cohort matching: "mom groups but for egg freezing — people going through it the same week of shots."	39, single. Two egg freezing cycles completed. Actively considering solo IVF/UI with donor sperm. Mom supportive and nearby. Researching options across Reddit and peer conversations. Wants to eventually reframe solo motherhood the way she reframed egg freezing.
"Uncertainty" and "lack of trust" were her defining words. No single source of truth — the doctor has an agenda, friends share horror stories, no one has the macro view. Horror stories dominate because people share extremes. The taboo of discussing egg freezing made it feel more isolating — "it shuts down conversations." Now that she's done it, she can help others, but boundaries around what she can advise are important to her ("I can tell my experience but not the experience — I'm not a doctor").	A paced, phase-based platform — "you're here, here's what you need to know now, here's what's coming." Anticipatory guidance that answers questions she didn't know to ask. Trusted clinic reviews and comparison data — "which clinics to think about, what are the pros and cons, what to even look for." Normalization of the emotional experience — "it's normal to be scared, it's normal to hear horror stories, here's the real distribution of outcomes." Post-retrieval continuity: still has unanswered questions about storage, IVF process, and her own ongoing fertility.	30 years old. 37 eggs in storage at SRM Seattle. No immediate plans to use. Wants ongoing answers about storage, egg viability, and her fertility going forward. Offered to connect Sarah with two women of color who have gone through egg freezing.
Round 1 at 33: single, supported by close friend group. Hid injections from elderly grandmother during a visit. Friends who had been through it first were primary support system. Round 2 at 38: boyfriend (now husband) lived with her, supported with shots — he wanted to give them, she declined. IVF: both fully invested as a couple. No reported clinical mental health support or formal emotional scaffolding at any stage. Did not describe major emotional crises but noted the process was a lot and valued her friend group throughout.	One central place for all instructions — done sifting through emails for the latest one. A sample timeline of the full cycle: when to be near home, when to get extra meds, when she will feel bloated, when she cannot exercise. As busy working women we kind of need to know. Financial tracking tool: real-time cost estimator by insurance coverage, checklist of what has been paid vs outstanding, digestible version of the coordinator printout. Single point of contact at the clinic. Nurse or resource available at all times for questions. Pre-booking capability for future transfers — should I call now to get October on the books or will they laugh at me.	40 years old. IVF baby Addie born December 2025. 10 remaining healthy embryos at RMA NY. Plans second transfer for boy in October 2026. Paying monthly embryo storage fees. Neighbor and friend of Sarah in Williamsburg. Offered Sarah guest access to Chief (Mon-Wed with co-founder).

Emotional experience overall	What would have helped	Current status
Largely positive with specific disruption points. The AMH news was genuinely devastating — "I left there feeling infertile." The decision to proceed anyway without optimal conditions was anxiety-inducing. Doing injections alone felt "scary and a little depressing." Post-retrieval isolation came from friends not understanding, not from the clinic. Ended on a high — the 14-egg outcome reframed the entire experience as triumphant. Notably pragmatic and resilient emotional register — not prone to catastrophizing, processes through action. Compared expectations management to "lower the bar and you'll always come up higher."	Real-time text/chat support during injections — "like a hotline" — specifically in that first injection moment mid-dose confusion Better education for support people (friends, family) about what post-retrieval actually requires A platform with verified doctor reviews — mentioned Sweats in the City Facebook group as the only current resource, called it "kind of crazy" AI-powered result interpretation — specifically mentioned wanting something to read her monitoring results and tell her where she was in the process, how close to retrieval, likely timeline adjustments Someone to help coordinate the logistics of post-retrieval recovery for people going through it alone	Cycle complete. 14 mature eggs stored. Last week at NYU before starting new job. New employer benefits unknown — may do another cycle in future. Signed up for Carousel waitlist. Open to being a resource and referral source.
Matter-of-fact throughout. Not emotionally destabilized by the process. Frustration was primarily operational — responsiveness, information gaps, storage cost opacity, fragmented portals. "The worst part of the experience is just how slow the responsiveness is." Felt better after completing the process, not during it.	Three things named clearly: (1) Real-time answers to clinical questions — asked if she could exercise, got answer two days later after she'd already exercised. (2) Medication clarity — a single interface that said "these are your meds for tonight, here is the video, here is the time." (3) SART data translation — personalized interpretation of which stats actually apply to her specific situation. "There's just not one platform for everything." On D2C pricing: skeptical that women should pay more on top of everything else — preferred clinic or Progyny as distribution and payer.	Preparing for split cycle at NYU with Dr. Cascante. Delayed one month due to cyst. Planning to continue tracking experience. Offered to share with friends and stay connected. Active investor in the fertility space — watching the category closely.

Emotional experience overall	What would have helped	Current status
Emotional Experience Overall One of the most complex, compounded grief profiles in the dataset. "It's so much harder than I think I ever thought it would be." What breaks people is not the needles — it is the waiting. "It's the anticipation, the anxiety, everything else that you just don't know. That one's hard.	What Would Have Helped Community with people who had been through it — peer connection was her primary coping mechanism throughout. "What has always helped me is meeting other people that have gone through it." She built her own support network via Reddit and community boards entirely independently of her clinic. Had a fertility therapist but stopped when she was laid off — therapy is not cheap and she had to cut it. Now uses ChatGPT and Reddit for support	Current Status Waiting on embryo biopsy results from second retrieval. Hoping for more viable embryos tomorrow. Does not have a third retrieval in her emotionally or financially
Emotional Experience Overall Primarily positive experience due to clinical team quality. Emotional complexity was present but manageable. "I think I was pretty scared about the question of my fertility, my ability to have a family. That was pretty heavy." The logistics of doing it across two countries in a compressed window added stress. Felt the weight of ambivalence — not fully certain she wants children, but not at peace with losing the option	What Would Have Helped Three things named unprompted: (1) Real-time messaging with the nursing team — like iMessage or WhatsApp, not a portal inbox. "To be able to message them in real time like a friend would be really helpful." (2) A clear visual calendar with milestone reminders — she built her own. (3) Community with other women going through the same experience — "just to feel like you're not alone." On AI: open to it but cautious. "I would not want it to be an AI agent. It's so personal that to know it was the nursing team — Millie, Jane, Francesca — that would be really amazing. An AI could be good but it would feel hard to fully trust it on such personal decisions."	Eggs in storage in Australia. No active plans. In the storage years gap — no clinic contact since retrieval.
Emotional Experience Overall "At some point I kind of lost... what am I here for? What does it look like to take care of myself?" Running a company while going through IVF in secret — did not feel safe disclosing during fundraising. Flew back from NYC investor meetings and injected herself in the airport. The invisibility of the burden was its own form of weight	What Would Have Helped A single coordinating layer across all providers. Someone who held her full picture and could communicate between her OB, her REI, her reproductive immunologist without her having to be the relay. Also: a safe space to talk about it as a founder — the intersection of building a company and going through IVF is entirely unaddressed.	Current Status Paused on IVF. Evaluating gestational carrier. Advising a company in the pregnancy loss space. Emotionally in a more reflective, stabilized place

Emotional experience overall	What would have helped	Current status
Emotional experience overall: "IVF was the hardest thing I've ever done." "The endless cycle of hope and despair." "It got progressively worse." Combination of: (1) advanced maternal age shame, (2) unprocessed miscarriage grief, (3) genetic testing attrition, (4) absence of adequate mental health support, (5) operational burden of medication/pharmacy/calendar management. Found her own therapist through a 40-page in-network PDF + Gemini prompt — ended up with a male therapist who knew nothing about IVF: "We were training him. He would say things like 'why don't you just skip this round?' You just don't." Only found appropriate support late via friend referral to Kala Collective (reproductive health specialty). "She has made such a difference." Still seeing her now — "at this point I feel like we're paying this therapist out of network to teach us how to have a newborn."	Community with other women going through it in real time — "people who are going through the same situation you're going through, in real time." Values anonymity. Currently texts with two other women she's never met in person, didn't know last names — "that has helped me personally the most." Aggregator of science-based information — cited Emily Oster as the model. Wants data-driven answers to basic questions she now Googles (e.g. Tylenol in pregnancy) that get drowned out by MAHA/anti-science content. AI calendar integration — automated reminders for which shots at which times; medication tracker that integrates with pharmacy to show what she has on hand at all times. "This is such a huge pain point and it has to be able to be done better in this world we live in." Marketplace for medication and goods — acknowledges legal/medical complexity but the empty-Gonal-box crisis could have been resolved through a peer marketplace	Current status: 25 weeks pregnant. Has become a pseudo-fertility-advisor for four friends who've since done egg freezing. Actively passionate about improving the experience for other women — is connecting women she sees on Instagram to doctors she trusts ("connected a random girl from Instagram to Dr. Irani at Cornell"). Said directly during the interview: "I wanted to do a fertility startup. My husband is like, you know nothing about HIPAA and healthcare tech." Referred her Mexican lesbian friend Cat (starting egg freezing next week) as a potential P27. High-potential candidate for Carousel community or ambassador network.
Emotional experience overall: Dominant emotion: anxiety. Secondary: isolation from control. Her frame: "This is not an input-output situation. I went into this not knowing that the science was still so up in the air. Are you ready to just potentially lose \$7,000?" She had a friend doing IVF simultaneously who got only 1-2 eggs — same protocol, completely different outcome. That friend eventually gave Nour her leftover meds. The randomness of the biological response was the hardest thing to accept for someone whose professional identity is built on evidence, data, and systems. The emotional aftermath of the retrieval (pre-period bloating, bruises on her abdomen, hormonal crash) was harder than treatment itself. Notably: she was heartbroken at intake and used the cycle as an act of self-	Normalizing the outcome ahead of time — she didn't know it was standard to need 2+ cycles. Knowing "this may not work, girl, manage your expectations" upfront would have calibrated her disappointment. A dedicated nurse assigned to her case — "I want to tell my story once, I don't want to repeat it five times to colleagues on break." Consolidated information delivery — single place for all communication instead of emails + phone + voicemail + YouTube links + multiple trigger drug options. Real-time dosage guidance — an interface where the nurse could push "no dosage change" or "change to X" without her waiting all day. Anticipatory logistics — she signed a consent about what happens to her eggs at age 50 while already in a gown. "Let's think about my	Current status: Living and working in Senegal on Gates Foundation medication supply chain project. Canadian citizen. Her 33 eggs are cryopreserved in New York. New anxiety: US political and immigration dynamics — birthright citizenship concerns, uncertainty about whether she'll be able to freely access her own eggs if she returns to NYC to use them. A new kind of post-freeze question set that Carousel should be aware of: what happens when the patient's life takes her far from where her eggs are stored? Single, still hopes to meet a partner. Deeply pragmatic: "If I have one, I'll be happy. If I have two, it's

Emotional experience overall	What would have helped	Current status
Emotional experience overall: Julie is processing this in a way that is calmer than most — the children are here, the outcome is what she wanted — but there is a deep, unresolved weight around the remaining embryos that she is actively carrying. She moved from survival mode (cancer, job loss, IUIs, IVF, infant, second IVF, second infant) to the quiet moment after, when the question "what do I do with what's left" has no answer. She was grateful for the human moments (her nurse), the competent clinician (Dr. Bernardi), and the outcome (her children). She was worn down by the medication logistics (temperature-sensitive drugs delivered in summer heat to a pharmacy where the dry ice had melted), the pharmacy coordination gaps, the insurance friction (she had to call HR to yell at the insurance company), the physical difficulty of self-administering intramuscular shots ("you're expected to have a	Community with other women at the same clinic, at the same cycle stage, in real time. She had an in-person support group during her first round (pre-COVID) that was "life-changing." Her IVF buddies became lifelong friends. "You need an IVF buddy." The in-person group was a COVID casualty. Facebook groups did not replicate it. Her specific observation: "I never could understand why there wasn't a support group through the clinic. HIPAA, I'm sure, whatever — there should have been a way for the women who see each other in the waiting room every other day literally to talk to each other. That was just such an obvious miss to me." Cohort segmentation in those communities. "Separate the folks who have kids from those who don't. Go into the clinic and see someone toting their older kid and here you are dying to have one — you're like, I don't want to see that today. Separate by stage, have ones specific to cancer survivors, LGBTQ, secondary infertility." Real-time shot help. "One lesson with the pharmacist, maybe one	Current status: 44, two children, living in Northbrook. Remaining embryos frozen at Northwestern. Active in IVF Facebook groups as a "cheerleader for people on the other end of it." Will continue to pay storage fees until she resolves the embryo question. In a state (Illinois) where reproductive care is politically protected — but she is acutely aware that women in other states are in a fundamentally different situation regarding embryo decisions, and she flagged this as a significant consideration for Carousel: "Even being part of this kind of an online community — if they have a way of tracking it or gaining access to it — it's really scary out there." She also raised liability: "You keep hearing about these clinics
Dominant emotions: anxiety about cost, frustration at absence of proactive fertility conversation in primary care, solidarity with friends in the process, anticipatory grief about navigating IVF as a queer person. She named long-term anxiety about IVF specifically as a queer person who may want biological children — the path forward has structural financial and logistical friction she's already dreading. But her emotional state is not despair — it's the measured, analytical anxiety of a product manager who can see the problem clearly and is frustrated that no one has built the solution yet.	What would have helped (direct product asks from a product manager's brain): Segmented community by identity and stage. Not a monolithic "fertility community." Sub-communities she can search and join: young queer women in fertility consideration; women of color navigating racial bias in care; people at different stages of the journey. References Facebook-group-style segmentation and mentions a friend's app that does sub-community sorting well. Direct quote: "I can search for young queer women in fertility consideration and then I can join that community forum." Cross-stage connection. Wants to talk to someone who's been through the whole process and now has a child — AND to other young women in the same pre-consideration phase. Both directions simultaneously	Current status: 27, in SF, in a ~2-year queer relationship, product manager (recently transitioned into the PM role). Actively researching via friends and community. Not yet engaged with a fertility clinic. Has signed up for the Carousel waitlist during the interview. Stated intent: share Carousel with her circles, reshare on LinkedIn when Sarah launches publicly, blast her community organizing network. Offered unprompted to potentially introduce Sarah to her own CPO, Ruth Yalon (women's neonatal MRI PhD background, grew current company from zero).

[illegible]

Potential Maeve Moments (hypotheses, not confirmed)	Post Freeze Ideal Experience	Additional Notes 1	Additional Notes 2
The midnight alone in a foreign country moment: arrived in Cancun alone at midnight navigating a rental car scam. The pre-clinical isolation is real and physical, not just emotional. Maeve as a "you're not alone in this" companion before, during, and after the procedure — especially for women doing this without physical support. The wrong birth control spiral: prescribed a medication with a 2.2/5 women's rating that caused a month of psychological distress before the procedure even started. Maeve as medication research companion — "here's what other women have experienced with this specific pill, here are questions to ask your doctor." The existential question too big to hold alone: trying to decide if she wanted children while simultaneously going through the egg			She is already organically building the community Carousel wants to build — collected 10 emails from WhatsApp groups, was planning a home gathering for women to discuss this. She is a natural community builder and potential early ambassador. Her description of the Kids or Child-Free coaching program is important — there is an entire population of women who are ambivalent about having children, not just women who definitely want them, who are going through this process. Carousel should serve this segment explicitly. International egg freezing is a significant and
The first appointment emotional misattunement: doctor overwrote Lissy's emotional state with empowerment language when she needed acknowledgment. Maeve's entire design philosophy must start here — meet her where she is, never reframe without first reflecting. This is the IFS principle in practice. The "am I doing it right" uncertainty during stimulation: no clear visual progress tracker, no confirmation she was on track, left a monitoring appointment not knowing if things were going well. Maeve as real-time progress companion — "here's where you are in your cycle, here's what today's numbers mean, you're on track "			Her most important quote for the pitch: "It's a lot of very clear direction for an unclear process, with unclear outcomes, with clearly complicated emotions." This belongs in the deck alongside Ilana's "they expect you to be a f***ing chemist." The sales pitch observation is a significant competitive insight: the fertility consultation is experienced as a sales pitch regardless of intent. Carousel's pre-clinical layer is the antidote — women arrive at the clinic already informed, already emotionally prepared, no longer feeling sold to. NVL is the most prestigious fertility program in the
The injection meltdown moment: Sunday night, alone, staring at a vial she doesn't know how to mix. This is Maeve's most acute use case — real-time hand-holding for a woman who is terrified and has nobody calm to call at 9pm. The portal waiting room: she asked if she could drink at a wedding and got the answer three days after the wedding. Maeve as immediate clinical triage — answering the 80% of questions that don't require a physician while flagging the 20% that do. The social graph clinic recommendation: her most specific product request — show me who in my community went where and what they thought. This is a feature, not just an insight. The shame spiral post-retrieval: found out friends got 40+ eggs and felt she had "done it wrong." Maeve as longitudinal context-			She showed the landing page during the call and loved it — described it as "really cool." Said the woman in the photo looks 35 and "ageless in a good way." Strong validation for the new Unsplash image direction. She is a natural evangelist — already helping friends navigate egg freezing in her network, already offered to share the waitlist link immediately and with her mom's network. Her exact product vision — "I'm picturing a world where a friend says download this" — is the most precise articulation yet of the pre-clinical community acquisition model. This is the word of mouth flywheel described spontaneously by a real user. She has nuanced but open relationship with AI —

Potential Maeve Moments (hypotheses, not confirmed)	Post Freeze Ideal Experience	Additional Notes 1	Additional Notes 2
The solo patient moment: woman doing this entirely alone with no family support needs a fundamentally different emotional check-in than someone with a robust support network. Onboarding should assess support system immediately. The logistics overwhelm moment: injection at a concert, travel protocols figured out alone — Maeve as proactive logistics companion, not just emotional support. The information vacuum post-retrieval: clinic goes silent, she			She is a journalist — Dr. Monya De, science journalist. Handle her data with particular care given her professional context and her explicit concern about AI and privacy. She was surprised the gift card was only \$20 — had expected more. Worth standardizing and communicating the gift card amount clearly upfront in future recruitment. She disclosed the company name during the interview
The trigger shot crash: severe post-retrieval emotional and hormonal crash that nobody warned her about. She had to Google it herself on Reddit at her worst moment. Maeve as post-retrieval normalizer — proactive warning before retrieval, real-time support during the crash, clinical explanation of what's happening to her body. The information overwhelm / manual tracking burden: too much information thrown at her, had to manually track everything. Maeve as cycle-aware personal assistant — "your trigger shot is tonight at 9:43pm, here's what to expect tomorrow." This is the pregnancy app analogy she articulated spontaneously. The execution mode blind spot: she didn't think she needed emotional support going in, only recognized she did after the fact. Maeve must serve women who don't think they need it — the check-in can't require her to opt in to vulnerability. The storage decision moment: Meta coverage expiring, getting			She showed her the ocean image from the landing page — Ana said the woman looked 19, maybe early 20s. This confirms: the ocean image is too young. Do not use it. Her Spring Fertility experience with Dr. Fischer is directly relevant to the April 7 Fischer meeting — Ana's description of Fischer as an "incredible saleswoman" who is "efficient but not as empathetic" is important intel. Fischer knows how to sell. She does not necessarily know how to hold. That's Carousel's opening. The functional medicine doctor's anti-egg-freezing argument is worth noting. There is a counter-narrative emerging around hormonal disruption from elective freezing in younger women. Carousel should be aware of this and not dismiss it — acknowledging complexity and uncertainty builds more trust than a pure pro-freezing stance.
The 10pm trigger shot panic: administered the trigger shot incorrectly, alone, at 10pm, panicked. Got through on the emergency line but was genuinely distressed. Maeve as real-time injection support — step-by-step guidance, error correction, reassurance at any hour. The "why can't I work out" research gap: clinic gave a blanket restriction without explanation. She did the research herself and found nuance that changed her behavior and mental health. Maeve as clinical explainer — the why behind every instruction, personalized to her cycle day. Post-retrieval pain unprepared: couldn't walk the day after retrieval, wasn't warned. No one to talk to in that window. Maeve as post-retrieval companion — proactive warning before retrieval, real-time support during recovery.			Betina is the most positive research subject in the dataset — her experience challenges the assumption that all women have a negative experience. This is valuable counter-data. The product must serve both the woman who struggled and the woman who didn't — without making the latter feel like she's being told her experience was wrong. Her insight about the narrative shift is important for Carousel's brand positioning. The platform should not lead with pain and struggle — it should also hold space for women who approach this with agency and pride. Kindbody's portal data visualizations — follicle charts, hormone graphs, egg count projections, probability calculators — are a concrete product benchmark. She

Potential Maeve Moments (hypotheses, not confirmed)	Post Freeze Ideal Experience	Additional Notes 1	Additional Notes 2
The "dating sucks" resentment moment: post-retrieval pain combined with the absence of a partner created acute resentment. Maeve must hold space for this specific emotional flavor — not empowerment, not grief, but raw resentment at having to do something hard because the world didn't cooperate. Don't tell her it's empowering. Just be with her. The gaslit-into-empowerment problem: friends saying "I felt so empowered" when she felt terrible made her feel more alone, not less. Maeve must never impose an emotional narrative. Meet her where she is, even if where she is is resentment. The clinic lawsuit TMZ moment: saw her clinic in TMZ for potentially losing someone's eggs. Felt jarring and anxious. Maeve as storage status guardian — proactive, regular confirmation that eggs are safe, storage is current, nothing to worry about. The peer tips gap: learned to wear a dress to appointments, buy a medication tray, from friends — not from the clinic. Maeve as peer wisdom aggregator — practical tips that improve the daily experience, sourced from real women who have been through it. The Alto Pharmacy benchmark: Alto's clean UX, educational			Percia recommended Dr. Womboch to Ilana Peña who is P02 in the dataset. This is the first confirmed peer referral chain in the research — real-world evidence of the word-of-mouth clinic selection pattern. Her Alto Pharmacy insight is the most specific product UX benchmark in the dataset after Betina's Kindbody portal. Alto's model — clean, educational, human touches like the caramel — is the emotional template for what Carousel should feel like. The retreat concept — work from Cabo, do appointments, be with other women, then retrieve — is a genuinely interesting product insight. Not a literal feature but evidence that women would pay meaningfully more for an experience that combines community, normalcy, and clinical support simultaneously. She mentioned she had looked into the surrogacy market as a business and learned about PE consolidation in egg retrieval — her clinic got acquired by PE. This is relevant competitive context and validates the clinic experience deterioration thesis. Research integrity note: approximately the last 20
The cancelled cycle blindside: she had no preparation for the possibility of cancellation. Maeve proactively sets expectations before each cycle — here are the possible outcomes, including cancellation, and here is what that would mean. Not to scare her. To prevent the blindside. The 2am injection question: had a question after hours, got routed to a secondary messaging system, wasn't sure if it connected back to her care team. Maeve as the always-on clinical triage layer — instant response, full context, and a clear flag to the care team when something needs clinical attention. The Reddit symptom spiral: Googled "hair loss plus egg retrieval" at midnight. Maeve as the contextual symptom explainer — is this normal? Here's what we know. Here's when to flag it. The financial chaos: she was the go-between for medical, pharmacy, and financial — all fragmented. The Journey's			Alexandra is the first research subject who petitioned her own employer to expand fertility benefits — a remarkable data point for the employer benefits channel. She is living proof that employees will advocate for this benefit if given the opportunity. This is a GTM story. One Medical is her UX benchmark — cited unprompted, specifically for messaging. This is the third research subject to reference a non-clinical digital health product as the standard they wish their clinic met. Alto Pharmacy, One Medical, iMessage — all cited. None of them are fertility products. The bar is being set by consumer digital health, not by the fertility industry. The financial coordination chaos is a new product insight not previously validated at this depth. Financial and medical tracks are completely separate at most clinics but patients are forced to navigate both simultaneously

Potential Maeve Moments (hypotheses, not confirmed)	Post Freeze Ideal Experience	Additional Notes 1	Additional Notes 2
The cancelled cycle grief: two cycles cancelled mid-process. The emotional experience of building hope across a two-week cycle and then having it end without the outcome is a specific, recurring grief that the dataset has not previously captured at this depth. Maeve as the companion through cancellation — not just through successful cycles. The forced retrieval trauma: the most egregious experience in the dataset. Progyny forced a surgical procedure against medical judgment. Maeve cannot prevent this but Carousel's insurance navigation layer could have helped her understand her rights, her options, and what questions to ask before she was forced into the corner.	Post-Freeze Ideal Experience Wants an open line of communication with the clinic through the next steps — whether they conceive naturally or need to use the frozen embryos. Wants clarity on costs, insurance, timing, and scheduling as they approach those decisions. Is planning her life around a potential pregnancy and needs to be able to do so thoughtfully. Has lots of competing demands professionally and personally — being able to plan with accurate information is critical for her.	<u>The Insurance Horror Story</u> This requires its own section because it is unlike anything else in the dataset. Progyny changed their coverage policy in early 2026. If a patient has had multiple cancelled cycles, Progyny will not approve future coverage unless the current cycle proceeds to retrieval — regardless of medical judgment. The stated rationale appears to be fraud prevention or cycle management, but the effect is that an insurance company is overriding physician and patient judgment to force an unnecessary surgical procedure. Rory and her doctor both knew the January cycle	Rory is the most medically complex case in the dataset. Her experience adds critical new dimensions: multiple cancellations, insurance coercion, hormonal resistance, embryo freezing rather than egg freezing. The product must serve women at every level of clinical complexity — not just the first-timer doing a routine egg freeze. The Progyny forced retrieval story is a landmark data point. It is the clearest evidence in the dataset that the insurance layer actively creates patient harm. This belongs in the pitch deck's Problem slide as the most extreme version of "the system is not designed for the patient." Dr. Chelsea Cannon at RMA is worth noting —
The recovery room placement: she was put next to a woman audibly celebrating a high egg count while she lay there with two eggs, hysterically crying. This is a clinic operations failure with a product solution — Carousel cannot control recovery room placement but can proactively prepare women for the possibility of disappointing outcomes before they happen, so the emotional landing is less catastrophic. The injection alone problem: she could not self-administer. Her friends came every night. What happens to the woman whose friends can't come? Maeve as injection coach — step-by-step real-time guidance, reassurance, and in-clinic injection support as a feature		Mental Health Status: Weekly therapist. Prozac. Facebook support groups. Strong best friend. No local grief support group — tried one that rejected her for needing "more support than they could provide," which she found devastating and confusing. Does not have a synagogue community. Financially precarious — cannot afford most paid services. She has professional mental health support in place. She is not without resources. But she is in a genuinely acute period of grief and isolation that the weekly therapy session and Prozac are managing but not resolving.	Aviva represents a participant profile not previously in the dataset: financially precarious, gig economy, no employer benefits, discount clinic, maximum emotional vulnerability, minimal support system. She is the woman for whom the current fertility system fails most completely. She is also the hardest person to build a sustainable business around — she cannot pay \$15/month. This is the access equity tension at the heart of Carousel's model that will need to be resolved thoughtfully. The recovery room placement next to a high-yield young patient is the single most preventable clinical harm in the entire dataset. It cost nothing to

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— Post-retrieval hormonal crash warning. Nobody told her this was coming. A proactive Maeve message the night of retrieval — "Here's what the next 4–5 days may feel like emotionally. This is normal. You are not broken." — would have been the single highest-value touchpoint of her entire experience. — The 7am waiting room moment. She sat with women who were all cycling at the same time and nobody connected them. Maeve could surface a cohort — "There are 6 other women at your clinic this week. Want to connect?" — at the exact moment she was sitting among them. — The box-checking emotional register. For women who don't feel empowered — who feel pragmatic, relieved, or just done — Maeve needs a register that meets them there without projecting meaning they don't feel. A simple "You did it. That's enough." touchpoint.	Described entirely through the community lens. She wanted to know who else was cycling at the same time, be connected to them during the process, and have access to someone slightly ahead of her in the journey. She did not ask for more clinical information — she had what she needed from the clinic. What she wanted was human proximity. Other women. Real ones. Going through it at the same time or just ahead of her. She also wanted the post-retrieval emotional experience to be named and anticipated — not managed, just named. "This might happen. Here's why. It's temporary."	Important counter-data point for the dataset. Cornell's communication model — single assigned nurse, responsive email, no portal confusion — produced a fundamentally different experience than the fragmented multi-system chaos most participants described. This is not accidental. It's a design choice by Dr. Melnick's practice. Carousel should study this model closely and, in the Fischer meeting, ask whether Spring Fertility has a comparable approach or whether the portal-chaos pattern holds there too. Shira's positive experience does not invalidate the thesis — it sharpens it. The gaps she identified (community, peer connection, post-retrieval emotional support) exist even when the clinical communication is excellent. The clinical layer and the emotional/community layer are separate.	Dr. Alexis Melnick, Cornell — Shira has agreed to make a warm introduction. Dr. Melnick helped Shira's OB get pregnant — her network tentacles in the NYC RE space are significant. Worth evaluating as a clinical advisor or co-founder candidate. Note: she practices at Cornell, not Spring or CCRM — adds geographic and institutional diversity to the clinical co-founder search. Janice Fox, Head of Endocrinology, Harvard Brigham Women's — surfaced mid-interview as a family connection through the Gerski family (Westchester). Described as having personal connections to top REs including Dr. Melnick. Academic medical center perspective could be valuable for research partnerships and clinical validation. Worth a separate outreach after the Strong pre-clinical product validation. Her story is almost entirely about what happens before and around the cycle — the lead time, the logistical coordination, the financial navigation — rather than the emotional experience during it. This is an important complement to the rest of the dataset, which skews toward emotional and communication gaps during active treatment. Savannah's account validates Carousel's Phase 1 (pre-clinical) layer as strongly as any interview in the dataset. The woman who is considering, delaying, navigating competing priorities, and trying to understand the total time commitment before she commits — that woman is underserved and she is Carousel's first customer.
— The male OB dismissal moment. She lost time because a doctor told her at 32 she didn't need to worry about it. A pre-clinical Carousel touchpoint that says "here's what to ask your OB about AMH testing and why it matters before 35" could have changed her entire timeline. — The competing medical advice paralysis. She had two doctors giving her non-answers on a time-sensitive decision and no one to synthesize. Maeve as a coordinator who surfaces the right questions to ask each specialist — not answers, but the questions — would have been high value here. — The medication cost arbitrage. She saved 50% through weeks of manual research that most women would never do. Maeve surfacing: "Before you fill your prescription, here are two things to check" — one message, potentially thousands of dollars saved. High impact, low complexity. — The round two financial shock. Costs went up between cycles, she's on a payment plan, she didn't fully understand the rate difference in advance. A proactive	An ongoing community of women who have been through Spring — and through egg freezing more broadly — that she can tap when she's ready to decide about next steps. Not a support group. A peer network. She is specifically thinking about a future moment — potentially years away — when she decides whether to move forward with IVF. She wants to be able to connect with women who are a few years ahead of her in that decision. The longitudinal relationship she described is exactly the storage years product. She also wants a dashboard that passively surfaces her storage status — how many eggs, where they are, what her account status is — without requiring her to go hunting for PDFs. The eggs exist and she wants quiet confirmation of that, on her terms.	Salve named explicitly and by name. She used the app throughout her Spring experience and critiqued it directly: basic, not user-friendly, poor navigation, unclear labels, buried documents. Compared it unfavorably to One Medical. This is the strongest direct Salve product critique in the dataset so far — and it comes from someone who used it actively for two full cycles. Worth noting for the competitive slide: the patient-facing UX of Carousel's primary competitor has been directly criticized by a two-cycle user in your own research.	

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The 2-minute post-result call: 9 retrieved, 1 mature, audible typing, no clinical explanation, no warmth. The single most important moment of the cycle handled with maximum detachment. Maeve as the post-result companion who holds the woman in that moment — not replacing the physician's explanation, but being the warm, informed presence the physician wasn't. "Here is what 'mature egg' means. Here are the possible explanations. You are not alone in this." The work crisis collision: she was running two jobs, injecting hormones nightly, attending 6am monitoring appointments. No one at the clinic ever asked about her stress levels or life context. Maeve as a context-aware companion — adaptive enough to know when a woman's life is on fire. The waiting room: surrounded by women going through the same thing, siloed, no mechanism to connect. This is	Not applicable — no eggs were frozen. The 1 mature egg retrieved was not viable for cryopreservation.	Social desirability note: Justine connected through the Hoffman Institute — shared personal network with Sarah. Supportive of the concept throughout. However, the substantive content of her experience is fully consistent with the broader dataset and introduces no inflation artifacts on key findings. Compounding catastrophe pattern: this interview introduces the most extreme case of concurrent life stressors in the dataset. Divorce + absorbed work role + worst possible clinical outcome + clinically cold response = total system failure. Carousel cannot prevent life catastrophes, but it can ensure the woman is not facing them without support inside the fertility experience. Her "wholesome" articulation is pitch-deck material. She designed Maeve's emotional architecture in one word, unprompted.	New clinic in dataset: RMA Brooklyn (Dr. Rachel Danis). First RMA participant. RMA is one of the largest RE networks in the US — significant design partner target. Important nuance: the pre-procedure experience at RMA was genuinely positive. Thorough visual materials at intake, financial advisor, dedicated coordinator, responsive email, portal videos. The entire failure was concentrated in the post-result call. This supports the thesis that even a well-structured clinical process can collapse at the emotional inflection points. The "private equity" comment: she named RMA as a private equity-owned entity and attributed the scaremongering at the initial consult to profit motive. Recurring undercurrent across the dataset. Matters for Carousel's positioning — explicitly outside the clinic's financial incentive structure.

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The undisclosed nurse charge discovery: found out she'd been charged \$250 per call after the fact. Maeve as transparent cost companion — here's what this interaction will cost before you initiate it. The cross-border coordination void: doing IVF in Canada with eggs in California and zero support from the clinic. Maeve as the cross-geography coordinator for international and cross-border patients — a segment that is larger than most assume. The horror story absorption: learned about sperm mixups and non-viable embryo transfers while making her own IVF decision. Maeve as the calm, accurate information anchor when the internet is full of worst-case scenarios. The escalation-only model: she got adequate care only because she escalated to co-founders. Maeve as the escalation layer that doesn't require a patient to become adversarial to get what she needs. The post-birth abandonment: one email. "Did you give birth?" Maeve as the longitudinal relationship that doesn't end when the clinical relationship does.		Garvey does "a lot of things in fem" professionally and expressed genuine interest in staying connected to Carousel. She is based in Canada — first Canadian participant, adds geographic dimension to the dataset. Her insurer was Wind Fertility. The Bloomberg journalist she mentioned — Jackie Davados — is worth noting as a media contact in this space. Garvey explicitly offered to connect and refer. Strong warm relationship, potential early ambassador. The Kindbody deterioration story is the most detailed account of clinic-at-scale failure in the dataset. The undisclosed \$250 nurse call charge is a specific, citable product failure that validates Carousel's transparent pricing and communication model. The non-viable embryo transfer story — heard secondhand — is the most extreme patient harm story in the dataset and speaks directly to the need for clinical intelligence and patient advocacy that Kindbody demonstrably failed to provide.	
The 13-cycle data problem: five years of protocol variations with no system to compare them. Maeve as longitudinal clinical data companion — visualizing what worked and what didn't across cycles. The visiting embryologist revelation: a random clinician introduced a protocol that should have been standard — and the patient had no way to know to ask. Maeve as clinical intelligence layer surfacing evidence-based protocols proactively.	Data ownership and portability across all cycles. Protocol comparison tools. Pre-appointment briefings. Community of women in reciprocal IVF specifically.	Second Canadian participant (Toronto). Reciprocal IVF is an underserved use case — the "patient" identity is split between the egg provider and the carrier. Carousel must design for couples, not just individual women. Clair herself said she would have built this company if she had the energy — direct validation from a power user. Key quote: "Basically everything sucked and it was a horrible experience." — about a clinic that nonetheless	The \$200K debt figure is significant for the access and affordability story. Clair is not a low-income patient — she works in tech — and still accumulated this level of debt over 13 cycles. The financial burden of multi-cycle IVF is catastrophic even for employed professionals. Follow up: she offered to stay connected. She is also a potential voice for the couples/reciprocal IVF use case.
The cancelled first cycle shock: told nobody prepares you for it. Maeve as scenario prep companion — "here are the 4 possible outcomes by day 8, here is what each means and what comes next." The deployed husband void: at every single appointment alone. Maeve as the always-present non-judgmental companion — "I was there for all of it." The hormone rage moment: "talk me off the ledge." Maeve must have a de-escalation mode — validate first, normalize, then ground	Phase-based guidance that anticipates what she needs to know BEFORE she needs to know it. Real-time "is this normal" triage. A person or product that celebrates milestones with her. Community matched by cycle stage, not just age.	Connected via mutual friend Liz Walton (CMO at Katie's company). Warm personal connection — potential ongoing ambassador. Military family context: husband was deployed for every IVF cycle except the last. The "going through this alone" dimension is amplified for military spouses and deserves specific community segment. Katie's exact product vision: a sponsor. Someone who has been through it. "Like AA but for IVF." This is the peer companion model — Maeve handles the 80% peer	Key quote: "It's by far the hardest thing I've ever done. I look back at having my daughter — cake." Powerful for the pitch. Katie was enthusiastically volunteering to help. Strong candidate for beta ambassador and potential peer companion model participant if we build that feature.

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The injection ritual moment: Maggie created her own candle/music ritual because the clinic offered nothing. Maeve as ritual companion — here is how to make tonight feel intentional, not medical. The anesthesiologist incident: a careless comment right before surgery spiked her anxiety. Maeve as pre-procedure prep — here is what to expect today, here is what is normal to feel. The water sip panic: told she might not be able to have surgery because of a small sip of water. Maeve as pre-op prep — on the night before retrieval, here are the exact rules so this never surprises you.	Her own data and protocol history accessible without asking. Proactive education about embryo freezing options. Cohort matching by cycle week. Emotional/ritual support layer. Clear explanation of the funnel math specific to her numbers.	Personal friend of Sarah's — treat data with appropriate weight. The "proactive embryo upsell" insight is a significant product feature: many women who egg freeze at 35-38 would benefit from being offered embryo freezing simultaneously — and they are not being offered it. Carousel could surface this recommendation proactively based on age and clinical profile. Maggie's cynicism about commercialism is a real and common barrier for women entering the fertility system. Carousel's brand must feel like a trusted ally, not a	Solo motherhood is an emerging and underserved theme now appearing in 3+ interviews (Jordana P01, Maggie P18). The journey from egg freezing to solo IVF to solo motherhood is a distinct longitudinal arc that Carousel should explicitly support. Key quote: "It's so hard to know what even success means." — this belongs in the validated findings. Success is ambiguous in fertility — you can have a perfect cycle and still have no baby. Carousel must never promise outcomes, only presence.
The OHSS spiral: heard a horror story mid-injection cycle and went into panic. Maeve as proportional risk communicator — "here is the actual distribution of OHSS outcomes, here are the early signs that mean call your doctor, here is why most women don't experience the severe version." The "is this normal" question: experienced it constantly. Maeve's core identity. The recovery surprise: everyone said she'd be back at work the next day. She wasn't. Maeve as recovery companion — "here is the range of recovery experiences, both are normal, here is what today might feel like." The clinic review panic: googled her clinic en route to first	Ongoing education about what her eggs mean for her future IVF chances. Storage continuity updates. Phase-based guidance for whenever she decides to use them. Trusted clinic and doctor review platform built specifically for fertility (not Google).	Youngest participant to date at 30. First participant who froze at 29. Represents a growing demographic: younger women freezing proactively before 30, motivated by career timing and early peer awareness. Explicitly asked for a female doctor — this preference is worth tracking across the dataset as a potential community matching signal. Offered to connect Sarah with two women of color (one Indian-descent) who have done egg freezing — valuable for diversity of dataset. Follow up immediately.	The "insurance policy with unclear coverage" metaphor is the most precise articulation yet of the post-freeze emotional state. Women know they have eggs but have no idea what that means for their actual chances. This is a validated gap. Key quote: "There's no source of truth — everyone just has their own perspective to share." This is the entire case for Carousel's clinical intelligence layer in one sentence.
The conflicting appointment time nightmare: received two different times for same appointment in same thread — husband rearranged work for the wrong time. Carousel as single source of scheduling truth. The mid-cycle meds ambush: told casually she might need extra medication with no warning or timeline. Carousel as proactive medication planning tool. The financial printout overwhelm: handed multi-page printout with this is probably insurance. Carousel as financial clarity layer — here is what this cycle will likely cost you. The eggs in western Massachusetts: did not fully understand	Single source for all cycle instructions and scheduling. Sample timeline with phase-based guidance. Financial tracker with insurance estimator and payment checklist. Pre-booking and waitlist transparency for future transfers. Storage cost comparison and options. Single point of contact model.	Personal friend and neighbor of Sarah — treat data with appropriate weight. Two-round egg freezer who then did IVF — represents the full longitudinal journey from early proactive freeze to IVF use. This arc (freeze early, use later) is the Carousel thesis made manifest. The CBS Business School peer group informational session is a notable acquisition insight: a trusted peer convened a group, brought in an expert, and multiple women moved toward treatment as a result. This is a community-driven acquisition model Carousel can	Key quote: There is a lot of low-hanging fruit. Said spontaneously at the end when Sarah confirmed Carousel was thinking about most of what Jenny described. Strong product-market fit validation from a sophisticated healthcare-adjacent user. Her 16 eggs at 33 becoming 11 healthy embryos is the most concrete illustration of why early freezing matters. She did not know at 33 what those eggs would become. She just did it. Cost-driven clinic selection validated again — chose Extend over Weill Cornell on a \$10K differential. Clinic

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First injection night: real-time dose guidance when nurse line put her on hold mid-injection — highest urgency Maeve moment in this interview Pre-AMH appointment: expectation-setting for women who may receive difficult news at their first clinical visit Post-retrieval window: education for support people — what does recovery actually require, what can she do, what can't she do Monitoring phase: daily "where are you in the process" check-in with probabilistic timeline guidance based on follicle counts Storage years entry: what does having 14 eggs actually mean for her future IVF chances — she's now in the ambiguous post-freeze limbo with no roadmap	Post-Freeze Ideal Experience Pauline did not articulate this explicitly, but implied: she wants to understand what 14 eggs actually means for her future. She mentioned potentially doing it again. She is entering the storage years with no clinical guidance on what comes next or what her 14 eggs are actually worth in terms of future IVF probability. A Carousel post-freeze education layer — what your egg count means, what factors affect future success, when to think about next steps — would directly serve her.	Works in NYU finance — sees fertility costs on their P&L. Flagged as a potential follow-up conversation about employer-side fertility benefit economics — Sarah noted this explicitly in the interview Alto Pharmacy was the UX benchmark for the entire experience — "everything should be like this." Consistent with prior participants TikTok FYP was her only fertility content source — not following specific influencers, just algorithmic Sweats in the City Facebook group cited as the only source for doctor reviews — "kind of crazy" Referred by friend Evelyn — word-of-mouth acquisition validated again Dr. Blakemore called personally to deliver the 14-egg result — notable positive moment, personal physician touch at the highest-stakes moment Participant is a strong Carousel advocate — expressed genuine excitement about the product and plans to share	
SART data interpretation at clinic selection — she had to figure this out herself entirely Real-time Q&A during stimulation — asked exercise question, waited two days Medication instruction layer — clear nightly checklist with videos per medication Storage cost transparency proactively surfaced — she was blindsided by PE price hike and facility move Split cycle logistics support — three portals, unclear storage cost structure	Wants to know upfront and clearly: storage costs, what happens if the facility is acquired, what the split cycle storage implications are. Proactive communication not reactive. Prefers not to be emotionally supported — just operationally held.	Lori is a VC investor who has specifically researched the fertility space professionally — Kindbody, Progyny, Carrot among companies she tracked. She knows the competitive landscape. High-value potential advisor or design partner. Also mentioned she had spoken with Dr. Brian Levine about a prior company (Nodal). Do not share this note in any context involving Dr. Levine or Spring Fertility contacts.	Lori unprompted suggested Dr. Brian Levine as a clinical co-founder candidate — this confirms his name recognition in informed investor circles. Sarah redirected the conversation appropriately. Lori is aware of Carousel's direction and has been added to the early access list. She is a warm contact for future investor network referrals given her professional focus on women's health investing.

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Orientation to what fertility treatment actually is before first clinic appointment — she walked in with no map Protocol change prompts — someone to say "four IUIs with no change is worth questioning" Real-time support during the two-week wait and between transfer results Grief support after 26-week termination — clinic provided nothing Accessible emotional support when therapy becomes unaffordable	Post-Freeze Ideal Experience Not applicable — Deniz is in active IVF treatment pursuing pregnancy, not egg freezing for preservation.	Toronto-based — Canadian healthcare context. Ontario government funds first IVF round including all transfers. Medication not covered. This is relevant context for any Canadian market consideration.	Additional Notes 2 Most emotionally heavy interview in the dataset to date. Handle with care in any future outreach or ambassador consideration. She is mid-journey and fragile. Do not contact for case study or testimonial purposes at this time
Pre-procedure: orientation to what the process actually is — she had good access but many women don't During stimulation: real-time low-stakes questions via messaging, not portal Calendar and milestone layer — she built her own because the clinic didn't provide one Post-retrieval: nobody told her what to expect physically in the days after Storage years: no contact from clinic since retrieval — classic abandonment window	Post-Freeze Ideal Experience Wants to feel held during the storage years — not overwhelmed with outreach, but not abandoned either. Acknowledged she would not want the clinic contacting her unsolicited. Would want resources available on her own terms. Community of other women in storage years would be valuable.	International patient context — Australia. Socialized healthcare system meant dramatically lower cost and different logistics. Relevant for any future international market considerations or competitive pricing analysis	Willing to pay for a companion app if it delivered real value — estimated \$300/year if paying directly, or expected clinic to absorb cost if integrated. "If I was paying \$8,000 Australian, I would pay for an app that would help me." Unprompted pricing signal — valuable for D2C and clinic SaaS pricing model validation.
At the ER miscarriage: nobody to tell her what to ask for or save Between providers: a coordination layer that held her full longitudinal picture During investor fundraising: a private, non-judgmental space to process the dual burden The airport injection moment — real-time support when she was most isolated Post-transfer failure: emotional processing before the next cycle began	N/A	Joan is a founder of Arise — a mental health / women's health adjacent company. She is advising a pregnancy loss startup. She is deeply fluent in the product and founder language. High-value design partner candidate if and when she has capacity. Handle with sensitivity given where she is in her journey.	Joan was introduced today through mutual connections and made the warm intro to Inflect Capital — see CRM note below.

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Night-of empty Gonal box at 9pm — triage to peer network or emergency pharmacy locator Progesterone suppository pharmacy confusion — plain-language explanation of compound medication expiration rules and coordination with transfer timing Unprocessed miscarriage during active IVF — proactive surfacing of grief support and reproductive-health-specialty therapist matching before the next cycle Asking Dr. Irani "can we parallel-path retrievals with transfer?" — decision-support conversation with guardrails, escalating to clinician when appropriate Training session only offered Monday 11am — flexible async onboarding module Birth control protocol differences between RMA and Cornell — second-opinion / protocol-comparison view that helps patient understand why clinics differ	Post Freeze Ideal Experience: Not applicable in traditional sense — Lydia is now pregnant, in the after-after phase. Expressed continued desire for community with other women at similar stages. Currently getting newborn preparation support from her Kala Collective therapist — "at this point we're paying this therapist out of network to teach us how to have a newborn." Suggests Carousel's longitudinal value extends into pregnancy and postpartum for the subset of patients who carry successfully.	Marketing/operator profile: 14 years at Penguin Random House + recent fitness industry work. Sophisticated consumer voice with strong product instincts — nearly started a fertility startup herself. Ambassador potential: VERY HIGH. Already unofficially operating as one. Confirmed consent to be kept in touch with. Asked "are you looking for more people to talk to?" and immediately offered a referral (Cat, Mexican lesbian starting egg freezing next week — directly addresses Sarah's explicit need for non-white participants). Clinic brand signal: Confirms current anti-RMA pattern in user research dataset (consistent with Sydney Rekaps Instagram post, consistent with other P-series participants). Confirms Weill Cornell + Dr. Pereira ("Dr. Irani") as the clinical gold standard in NYC. Useful for competitive and partnership mapping.	Confirmed happy ending — 25 weeks pregnant, first FET successful. Unresolved trauma from March 2025 natural miscarriage layered underneath active IVF cycles — a significant unmet need that Carousel could address through structured integration of loss support. Key quote: "I feel a lot of us who undergo this process tend to be high achievers. You often are doing this later in life because you've been working, and I personally don't think they make it easy." Second key quote: "IVF was the hardest thing I've ever done... it changed me as a person forever." Third key quote (on pharmacy/medication issues): "You are talking to women at a very vulnerable time. They're trying to coordinate a transfer." Fourth key quote (on what would help): "There are so
Dosage-change anxiety window — push notification from care team when labs are reviewed ("no change needed" or "adjust to X") so the patient doesn't spend the day waiting Outcome expectation calibration — at enrollment, structured "here is what your age + AMH + this clinic's data predicts" narrative, with explicit normalization that 2+ cycles is common "Who am I paying to see?" — transparency dashboard showing which clinician reviewed today's labs and when, so rotating coverage doesn't feel like care abandonment Consent moment pre-awareness — notification 24 hours before the day-of egg-freeze paperwork flags decisions she'll be asked to make ("you will be asked to consent to what happens to your eggs after age 50 — here is what that means, think about it now") Post-trial follow-up and destination support 4-7 post	Post Freeze Ideal Experience: Storage location awareness and continuity. She's a long-distance egg-storer now — her life moved on, her eggs didn't. For patients like her (Canadians, expats, remote workers, people whose careers relocate them), the "what happens to my eggs" question is live and unresolved. Also: storage-cost transparency, clear communication about policy changes at the storage facility, and — importantly — a clear answer to "if I want to use them, how do I get them to me or me to them." She was never told this at the outset. Her feeling is that the clinic's relationship with her ended at retrieval + invoice, and storage is a back-office afterthought.	Professional profile: Public health professional, currently working in tech (D2C go-to-market for semaglutide/Ozempic in Nigeria via Gates Foundation). She is SIMULTANEOUSLY a consumer who went through the experience AND a professional go-to-market strategist in health tech. This is a unique voice for Carousel's advisory network — she said unprompted: "My mind is where your mind is right now." Explicitly offered more time: "If I could help with half an hour and I went through it... no need." Signaled openness to stay involved. Referral potential: HIGH. Network includes doctors, public health professionals, international expats. May be particularly well-positioned to introduce Carousel to reproductive friends in the US and beyond internationally.	On clinic selection: "I chose her because I read her papers... I went very nerd on that." On rotating care coverage: "I'm going for Dr. Melnick and that's who I had the plan with, and then I was in the care of her team but I didn't see her the whole thing." On the experience fragmentation: "It's very much a do it yourself. It's very choose your own adventure." On anxiety: "It was extremely anxiety-driven... you don't know how your body's going to react to the drugs. You don't know anything until it's after." On outcome delivery: "It's a nurse who called me to tell me this is how many eggs you have. 'If you want more information you can do a follow-up with your doctor.'" On information consolidation: "The information could have been much better organized, more direct, it's not

Potential Maeve Moments (hypotheses, not confirmed)	Post Freeze Ideal Experience	Additional Notes 1	Additional Notes 2
After-hours prescription triage — AI-assisted pathway to route time-sensitive medication questions to a prescribing clinician outside business hours (the dropped-pill moment) Medication cold-chain verification — check-in when meds are delivered; flag dry-ice integrity; verify temperature exposure before patient injects Same-clinic same-cycle community matching — clinically-sanctioned, HIPAA-compliant mechanism for connecting women who are seeing each other in the waiting room every other day Cohort segmentation in community — stage-based groupings; separate people with existing children from people without; protected spaces for secondary infertility, cancer survivors, LGBTQ patients Intramuscular shot coaching — guided video or real-time support for progesterone-in-oil, site rotation recommendations	Post Freeze Ideal Experience: Julie's lived experience redefines what "post freeze" means. For the patient community Carousel is serving, "post" is not just "after retrieval" — it includes after live birth, when frozen embryos remain and the patient is done having children. This is a category-wide unmet need. Every IVF patient who succeeds creates surplus embryos. Most clinics do not have a plan for those embryos beyond storing them and sending invoices. The ethical, emotional, and legal complexity of embryo disposition is genuinely enormous and currently borne alone by the patient. Julie's direct question: "The number of embryos that are being stored right now has to be mind-blowing... one would hope someone's thinking about it." Sarah's on-the-spot response confirmed this has not been the focal point of Carousel's research, but	Source: Found Sarah via LinkedIn post Professional background: Medical and legal — "I'm in the medical field, I'm in the legal field. I know who to kind of pick up the phone and yell at when stuff wasn't going right." Unusual literacy in both domains gave her the confidence to navigate the system aggressively. She repeatedly flagged throughout the interview: "I can't imagine navigating it without any concept of it." A critical data point — if someone with Julie's dual professional literacy still found the system this hard, the typical patient experience is substantially worse. Ambassador potential: HIGH. Already a de facto ambassador in IVF Facebook groups. Explicitly asked for Carousel's landing page and offered to share it. Deep connection to the category, clear articulation, long time horizon.	On the OB/GYN's inability to work on fertility time: "The only thing that matters in this case is the timing, and I can't wait for their convenience." On the dropped pill: "I was all hopped up on hormones, and I had to take the pill when I had to take the pill, and I dropped it in the garbage. And if I didn't have the pill at just the right moment, the entire cycle was ruined. And I was digging through the garbage to find the pill and calling the pharmacist and begging her to help me, and she couldn't help me because she's not a doctor." On her nurse: "I felt like she was my bestie by the time my son was conceived proper. Like by the time I graduated from her." On the second nurse: "If she had been the nurse for the initial go-around, I really would have had a problem." On Northwestern's scale: "Northwestern is clearly a
Proactive fertility assessment in the annual gyno visit — prompt or checklist tool that makes the conversation happen when the patient signals concern (period history, family planning goals) Pre-consideration education layer — AMH/follicle/hormone explainers that connect to the patient's own data over time, not generic clinical content Cohort-segmented community discovery — queer, young, identity-specific, stage-specific entry points with gated privacy Price transparency calculator — insurance-input driven cost estimates by clinic and procedure, with explicit "this is an estimate not a quote" framing Third-party clinic review system — honest patient-reported reviews structured to resist clinic marketing inflation Cost-of-waiting framing — pre-procedure education that helps women like Ali understand how age/AMH/hormone trajectories	Not directly relevant (Ali has not frozen). However: she imagines wanting connection with women who have been through the entire arc — frozen, IVF, had a child — which signals that Carousel's longitudinal value has to include the after-after phase serving women in her position who are reaching up-funnel to women who have completed the journey.	Source: Maya Keerie (P24 or similar — interviewed by Sarah earlier this week) referred Ali via women's-health-baddies network. Original vector was Evelyn Cox at Johns Hopkins. Ali and Maya are college friends from Colgate (biology + women's studies together). This is the first referral chain in the dataset that runs entirely through a peer network of women aged 25-30 actively organizing around women's health issues. Professional profile: Product manager at a health tech company whose CPO is Ruth Yalon (women's neonatal MRI PhD background). Former Unite Us employee (SDOH-focused healthcare company). Community organizer in SF queer women's health spaces. Ambassador potential: VERY HIGH. Offered during the interview, unprompted: (1) signed up for Carousel waitlist	On the proactive care gap: "I was someone who got my period when I was 17. I've brought this up in multiple annual gynecology visits. I joke to my friends, I'm probably infertile, but who knows? because no one's ever asking." On clinic marketing distrust: "I'm on the healthcare side. I know that these marketing teams on the healthcare level are inflating their metrics for patient capture." On the financial barrier: "I'm scared to start because I'm so anxious about how expensive it is... I have long-term anxiety about IVF and what that means from a financial perspective as a queer person who maybe wants to have kids." On price transparency: "I would have never booked that appointment if I knew that " (re: \$400 OOP for a 15-min

[illegible]

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O005	P01	Mar 26 2026	What Would Help	Wants tailored medical research delivered to her: "If there's breaking news relevant to my situation, deliver it to me. Don't make me work."	V	No	1/20	Personalized intelligence
O012	P02	Mar 26 2026	Emotional Experience	Anxious person. Father died suddenly at age 5. Health-related procedures trigger mortality fear. "I'm going to mix the vial wrong and then I'm going to die."	V	No	1/20	Health anxiety / Trauma history
O018	P03	Mar 23 2026	Storage Years	Was collecting emails from WhatsApp groups and planning to host a gathering for women to discuss egg freezing. Building the community Carousel wants to build — organically, without a product.	V	Yes	1/20	Organic community building / Market validation
O036	P06	Mar 27 2026	Portal / Technology	Kindbody portal had charts and visualizations of follicle counts, hormone levels, egg projections, probability calculators. "I thought that was really good." These visualizations influenced her decision to do a second cycle.	V	Yes	1/20	Clinical data visualization / Portal benchmark
O038	P06	Mar 27 2026	What Would Help	Proposed care model: assigned nurse available 9-5, coverage nurse available 5-9pm who has reviewed the case notes — available within 10 minutes at any time. "It doesn't have to be your main nurse."	V	No	1/20	Care model design / Nurse coverage
O045	P07	Mar 27 2026	What Would Help	Had a spontaneous idea during the process: a retreat model — go to Cabo for two weeks, work remotely, have appointments, be with other women going through the same thing, do retrieval. "I probably would have paid for that."	V	No	1/20	Retreat model / Community + clinical combined
O009	P02	Mar 26 2026	Community / Support	"I'm picturing a world where I say to a friend, hey I want to freeze my eggs, and she goes, download this, they'll help you." — spontaneous articulation of the word-of-mouth flywheel.	V	Yes	2/20	Word of mouth / Consumer acquisition

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O010	P02	Mar 26 2026	Post-Procedure	After retrieval got 21 eggs. Felt good. Then talked to friends who got 40+ and felt shame: "I felt like I had done it wrong."	V	Yes	2/20	Post-retrieval shame / Comparison trap
O013	P03	Mar 23 2026	Emotional Experience	Described decision as happening "in a fear-based state in order to empower herself." Never wanted to freeze eggs — was in a relationship with a man who said he'd break up with her if she wanted kids.	V	Yes	2/20	Ambivalence / Fear- based motivation
O014	P03	Mar 23 2026	Pre-Clinical Research	Prescribed birth control with a 2.2/5 women's rating before procedure. Felt agitated and terrible for a month. Nobody told her to research the specific pill.	V	Yes	2/20	Medication side effects / Pre-procedure support
O016	P03	Mar 23 2026	Emotional Experience	People telling unsolicited success stories felt like false hope and unwanted comparison. Set a boundary with someone doing it — that person never spoke to her again.	V	Yes	2/20	Unsolicited advice / Comparison harm
O017	P03	Mar 23 2026	Post-Procedure	Post-retrieval weight gain for 3-4 months despite intense athletic training. Nobody warned her. "It felt almost shameful."	V	Yes	2/20	Post-retrieval physical aftermath / Ignored side effects
O023	P04	Mar 1 2026	Clinical Communication	First cycle: traveled from Williamsburg to Upper West Side at 6am for medication. Did real-time math on how many CCs to buy to avoid overpaying. "It just felt hectic."	V	Yes	2/20	Medication logistics / Financial stress
O026	P05	Mar 27 2026	Pre-Clinical Research	Chose Spring Fertility entirely because of influencer marketing — saw Dr. Fischer at an event, followed creators she partnered with. Did zero comparative clinic research. "I was completely caught in the marketing of it all."	V	Yes	2/20	Influencer-driven clinic selection / Marketing over clinical data

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O032	P06	Mar 27 2026	Emotional Experience	"People are empathetic as if you were sick — as if you had a disease. That took a toll on me." — on others' negative reactions to her egg freezing.	V	Yes	2/20	Narrative burden / Stigma / Empathy misdirected
O034	P06	Mar 27 2026	Clinical Communication	Different nurse at every appointment despite going every other day for two weeks. "It would help women feel safer if you got the same people — they know you, it feels more familiar and intimate."	V	Yes	2/20	Continuity of care / Familiarity gap
O039	P06	Mar 27 2026	Storage Years	Has ~40 eggs, is young, single, not thinking about IVF. Does NOT want proactive clinic outreach. "I would feel like it was an invasion." Will initiate contact when ready.	C	Yes	2/20	Storage years / Opt-in retention
O051	P16	Apr 8 2026	Clinical Experience	Success came when a visiting American embryologist introduced a new petri dish media protocol — embryo arrest stopped. This was never proactively suggested by the clinic.	V	Yes	2/20	Clinical intelligence gap
O061	P18	Apr 9 2026	Product Feature	"I wish they'd tried to upsell me embryos at the time of egg freezing." Now 39 with no embryos — she didn't know to ask, nobody offered it.	V	Yes	2/20	Proactive education / Embryo freezing
O001	P01	Mar 26 2026	Emotional Experience	"It feels like being in a vacuum." — doing it entirely alone with unusual lack of family support	V	Yes	3/20	Isolation / Solo patient
O002	P01	Mar 26 2026	Clinical Communication	Diet and supplement guidance mentioned once verbally, never provided in writing. Had to rely on memory.	V	Yes	3/20	Information gaps / Written materials
O004	P01	Mar 26 2026	Post-Procedure	Clinic has not reached out proactively since retrieval. No next steps plan initiated. She has the doctor's personal cell and sends periodic texts herself.	V	Yes	3/20	Storage years / Post- cycle abandonment
O007	P02	Mar 26 2026	Emotional Experience	"They expect you to be a f***ing chemist." — on self-administering injections with no proper support.	V	Yes	3/20	Injection anxiety / Medical complexity

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O011	P02	Mar 26 2026	AI Relationship	"I froze my eggs on Tuesday. Can I have a piece of bread today?" — describes exactly how she would have used AI during her cycle.	V	Yes	3/20	AI use case / Maeve MVP
O015	P03	Mar 23 2026	Community / Support	Wants community matched by age AND relationship status, not just age. "Are you single? Are you married? These are completely different journeys."	V	Yes	3/20	Community matching / Circumstance segmentation
O020	P04	Mar 1 2026	Emotional Experience	Left first appointment at 33 saying "I'm not f***ing doing this" because doctor overwrote her emotional state with empowerment language instead of acknowledging what she said. Waited a full year before returning.	V	Yes	3/20	Emotional attunement failure / Lost year
O025	P04	Mar 1 2026	Emotional Experience	"You go through this physical thing to do a thing without any present at the end — except a promise of a future potential. There's nothing really like that in the whole world that you pay this much money for."	V	Yes	3/20	Emotional complexity / Deferred reward
O028	P05	Mar 27 2026	Post-Procedure	Employer (Meta) storage coverage expires December 2026. Facing decision: pay for 10-year plan or not, while getting married next year and turning 30. Nobody helping her think through it.	V	Yes	3/20	Storage years / Financial decision support
O030	P05	Mar 27 2026	Emotional Experience	Described the trigger shot post-retrieval crash as an extreme version of PMDD — lasted 24 hours, had never been on birth control or hormones before. Had to Google it herself on Reddit after the fact.	V	Yes	3/20	Post-retrieval hormonal crash / Unprepared
O033	P06	Mar 27 2026	Clinical Communication	Clinic said "no working out" with no explanation. She researched independently and found she could exercise safely through day 10. Used her own research to make informed decisions the clinic didn't enable.	V	Yes	3/20	Clinical explanation gap / Patient agency
O035	P06	Mar 27 2026	Post-Procedure	Couldn't walk the day after retrieval due to high egg count. Was not warned about severity of post-retrieval pain. "That was my negative moment — what is happening, is this all right?" Had no one to talk to in that window.	V	Yes	3/20	Post-retrieval pain / Unprepared / Support gap

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O040	P07	Mar 27 2026	Emotional Experience	"I was so resentful. I cannot believe I have to go through this level of pain because dating sucks." — post-retrieval emotional state.	V	Yes	3/20	Resentment / Emotional complexity / Dating grief
O041	P07	Mar 27 2026	Emotional Experience	"I feel like I'm being gaslit into being excited about something that objectively sucks." — on friends describing the experience as empowering when she felt terrible.	V	Yes	3/20	Empowerment narrative / Emotional gaslighting
O044	P07	Mar 27 2026	Storage Years	Clinic later appeared in TMZ for a lawsuit claiming they lost someone's eggs. "That was a little bit jarring — I hope my eggs are okay." Now has ongoing uncertainty about whether to check on her eggs periodically.	V	Yes	3/20	Storage anxiety / Egg safety / Trust erosion
O052	P16	Apr 8 2026	Equity & Access	\$200,000 in debt for 13 IVF cycles. "The kids just won't get college paid for and that's fine."	V	Yes	3/20	Financial burden / Equity
O058	P18	Apr 9 2026	Pre-Procedure	Was turned off by fertility industry commercialism — "if you bring a friend, 15% off, Bogo" offers. Felt like "it's all a business." Took two additional years to engage.	V	Yes	3/20	Trust / Brand perception
O060	P18	Apr 9 2026	Clinical Experience	Anesthesiologist made a judgmental comment about her job (cannabis industry) right before surgery. Heart rate went up. Gave feedback to doctor post-retrieval.	V	Yes	3/20	Pre-procedure anxiety / Clinical communication
O003	P01	Mar 26 2026	What Would Help	"Somebody thinking for me and following up — are you taking your supplements, do you have your medication — like a concierge. Give me less to think about."	V	Yes	4/20	Cognitive offloading
O006	P01	Mar 26 2026	Portal / Technology	Portal was "rudimentary and ugly." Quote: "How are you charging people this much money and you can't pay for a fancy website designer?"	V	Yes	4/20	Portal quality / Technology gap
O008	P02	Mar 26 2026	Clinical Communication	Asked portal if she could drink at a friend's wedding. Got the answer three days after the wedding. Just didn't drink to be safe.	V	Yes	4/20	Portal delays / Real-time answers

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O019	P04	Mar 1 2026	Emotional Experience	"It's a lot of very clear direction for an unclear process, with unclear outcomes, with clearly complicated emotions." — on the mismatch between clinical certainty and emotional complexity.	V	Yes	4/20	Emotional complexity / Systemic invalidation
O021	P04	Mar 1 2026	What Would Help	"If she had simply said: I hear you. It is really hard and it must be complicated for you. Boom. Solved." — on what the doctor should have said.	V	Yes	4/20	Validation / Being heard
O022	P04	Mar 1 2026	Clinical Communication	Instructions came via phone calls she might miss, portal with PDFs and links, and two separate systems (clinical + billing). Scribbled down instructions from voicemails. Always anxious about missing the nurse's call.	V	Yes	4/20	Fragmented instructions / Operational chaos
O024	P04	Mar 1 2026	Portal / Technology	"I felt like I was constantly on my back foot. Like, did I not listen well enough? What am I doing today?" — on the lack of a visual progress tracker.	V	Yes	4/20	Progress tracking / Uncertainty during cycle
O029	P05	Mar 27 2026	Pre-Clinical Research	Had no friends who had done egg freezing yet — did it young at 26. Word of mouth was unavailable so influencers filled the gap. "Word of mouth has the highest weight of influence but I didn't have anyone to ask."	V	Yes	4/20	Pre-clinical isolation / No peer network
O031	P05	Mar 27 2026	What Would Help	Spontaneous product vision: "Like the pregnancy app where your baby is the size of a watermelon — but for egg freezing. Your trigger shot is at 9:43pm. You now have 24 follicles. Reminder: retrieval tomorrow."	V	Yes	4/20	Cycle-aware AI / Pregnancy app analogy
O037	P06	Mar 27 2026	Portal / Technology	"Something that mimics iMessage or WhatsApp — so much easier than going on the website, having a separate page, tracking on email if you got a message." — on the messaging UX.	V	Yes	4/20	Messaging UX / Real-time communication
O042	P07	Mar 27 2026	Portal / Technology	"The portal was so s****, I don't think I really used it. I honestly watched Alto Pharmacy's videos more than my doctor's videos." Alto sent a caramel with every delivery — described as the highlight of the entire experience.	V	Yes	4/20	Alto benchmark / Portal failure / Delight in small moments

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O043	P07	Mar 27 2026	What Would Help	Peer-sourced practical tips meaningfully improved her experience: wear a dress to appointments, buy a tray for medications. Clinic never provided these. Friends did.	V	Yes	4/20	Peer wisdom / Practical tips gap
O047	P07	Mar 27 2026	Procedure / Process	"Total time commitment was easily six weeks even though the cycle was only 10 days." Had not accounted for prep, monitoring frequency, and recovery in her expectations.	V	Yes	4/20	Expectation gap / Total footprint
O050	P16	Apr 8 2026	Clinical Experience	Calculated she was paying approximately \$145/minute of doctor face time based on total cost vs contact hours.	V	Yes	4/20	Access to care / Clinical communication
O057	P17	Apr 9 2026	Medication	Husband deployed for every cycle. Managed medication logistics alone — ran short on meds, drove 1.5 hrs to Denver mid-cycle to pick up at pharmacy.	V	Yes	4/20	Medication management / Logistics
O059	P18	Apr 9 2026	Emotional Support	Created her own pre-injection ritual: candle, good song, treating herself like a goddess. "Connecting with the little eggs as they developed." Clinic provided zero of this emotional scaffolding.	V	Yes	4/20	Ritual / Emotional reframing
O073	P20	Apr 10 2026	Pre-Procedure	Entry to fertility treatment was a CBS Business School peer group informational session — a trusted friend convened a group, brought in Dr. Pak Chong, and multiple women moved toward treatment.	V	Yes	4/20	Peer-driven acquisition / Community entry point
O027	P05	Mar 27 2026	Emotional Experience	"It felt more like a process than an experience. It should have been both." — on the clinical efficiency vs emotional depth of Spring Fertility.	V	Yes	5/20	Process vs experience / Emotional depth gap
O046	P07	Mar 27 2026	Pre-Clinical Research	Chose Dr. Womboch because her friend recommended her. Later recommended the same doctor to Ilana Peña (P02). First confirmed peer referral chain in the dataset.	V	Yes	5/20	Word of mouth / Peer referral chain / Clinic selection

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O049	P16	Apr 8 2026	Data & Protocol	Five years on Facebook groups researching drug protocols, embryo grading, supplements — because the clinic provided no longitudinal data tool.	V	Yes	5/20	Data ownership / Self-advocacy
O056	P17	Apr 9 2026	Support Model	"I wish there was a sponsor model — like AA — someone who's been through it who I can text any time. Is this normal? Talk me off the ledge."	V	Yes	5/20	Peer support / Sponsor model
O064	P19	Apr 9 2026	Clinical Communication	OHSS scare during stimulation: nurse response was "doctor coded, not friend coded" — legally cautious, not reassuring. "They're protecting themselves so they can't say you don't have it."	V	Yes	5/20	Real-time support / Clinical communication
O065	P19	Apr 9 2026	Post-Procedure	"People said I went back to work the next day." She and a friend both had difficult recoveries and were not prepared or warned.	V	Yes	5/20	Recovery / Post-retrieval experience
O074	P20	Apr 10 2026	Post-Freeze	"Should I call now to get October on the books or will they laugh at me?" — wants to pre-book a future transfer but has no idea about waitlists or lead times.	V	Yes	5/20	Post-freeze continuity / Longitudinal planning
O055	P17	Apr 9 2026	Clinical Communication	First cycle cancelled — she had no idea this was possible. "It didn't even occur to me the cycle wouldn't work." Was completely unprepared for every possible scenario.	V	Yes	6/20	Expectations management / Scenario prep
O062	P18	Apr 9 2026	Community	"Mom groups but for egg freezing — people going through it the same week of shots." Wants cohort matched by cycle phase not just age.	V	Yes	6/20	Community / Cohort matching
O071	P20	Apr 10 2026	Financial	Handed a multi-page financial printout where everything was labeled "this is probably insurance, this probably isn't." Could not track what she owed vs what was covered.	V	Yes	6/20	Financial transparency / Cost tracking
O072	P20	Apr 10 2026	Pre-Procedure	Chose Extend Fertility over Weill Cornell academic hospital entirely on cost — a \$10,000 differential. Did not focus on individual doctors; focused on the overall clinical apparatus.	V	Yes	6/20	Cost-driven clinic selection

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O053	P17	Apr 9 2026	Emotional Experience	"By far the hardest thing I've ever done. I look back at having my daughter — cake. So cake compared to what you go through with egg freezing, IVF."	V	Yes	7/20	Emotional weight / IVF harder than expected
O066	P19	Apr 9 2026	Post-Freeze	"It's like an insurance policy but I have no idea what's covered." 37 eggs frozen, no understanding of what that means for her actual chances.	V	Yes	7/20	Post-freeze ambiguity / Storage continuity
O070	P20	Apr 10 2026	Medication	Told casually mid-cycle that she might need to get more medications — no advance warning, no timeline. Calendar-driven personality found this deeply frustrating.	V	Yes	7/20	Medication management / Proactive planning
O075	P20	Apr 10 2026	Post-Freeze	Eggs stored in a warehouse in western Massachusetts — did not fully understand where they went or why. Now has embryos at RMA at \$400-500/yr more. "As soon as we're done with kids, we're not paying this anymore."	V	Yes	7/20	Storage transparency / Post-freeze ambiguity
O048	P16	Apr 8 2026	Clinical Experience	"Basically everything sucked and it was a horrible experience." — about a clinic that nonetheless produced two children after 13 retrievals.	V	Yes	8/20	Clinic communication / Patient experience
O054	P17	Apr 9 2026	Emotional Experience	"The moment I realized I was not in control." First time in her life she couldn't work harder to achieve an outcome. Follicle size doesn't respond to effort.	V	Yes	8/20	Loss of control / Agency
O067	P19	Apr 9 2026	Community	Knew almost no one who had done egg freezing before she did it. Described deep isolation and taboo. Now that she has done it, friends approach her constantly.	V	Yes	8/20	Isolation / Community
O063	P19	Apr 9 2026	Pre-Procedure	"Everyone has their own perspective but no one has the macro perspective. The doctor has an agenda. Friends share horror stories. There's no source of truth."	V	Yes	9/20	Information trust / Source of truth

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O068	P20	Apr 10 2026	Clinical Communication	Received two conflicting appointment times in the same message thread. Husband rearranged work meetings for the wrong time. "The scheduling was kind of a hot mess."	V	Yes	9/20	Scheduling / Communication chaos
O069	P20	Apr 10 2026	Clinical Communication	No single point of contact at RMA — multiple different staff members communicating inconsistently across portal, phone, and email.	V	Yes	11/20	Single point of contact / Fragmentation

Notes
Hypothesis only - not confirmed across dataset yet. Feature request: personalized fertility news feed.
Important: for some women injection anxiety is not about needles — it's about mortality fear rooted in trauma. Maeve tone must account for this.
She is already doing what Carousel intends to do. Proof of demand. Potential early ambassador.
First concrete competitor product benchmark in dataset. Kindbody's visualization layer influenced clinical decisions. Carousel's Frost layer must match or exceed this. Screenshots promised — follow up
Hypothesis only at 1/6. Her proposed model is essentially what Maeve enables at zero marginal cost — instant availability, full case context, 24/7.
Hypothesis only. Not a literal feature but evidence that women would pay significantly more for an experience combining community, normalcy, and clinical support. Validates premium positioning.
Most precise unprompted articulation of Carousel's consumer acquisition model in the dataset.

Notes
Comparison-driven shame post-retrieval. Maeve must contextualize numbers — 40+ often indicates PCOS.
Not all women freeze from a place of empowerment. Fear and ambivalence are valid entry points Carousel must serve without judgment.
Pre-procedure experience started badly before the procedure even began. Carousel should address medication preparation phase.
Maeve must never default to success stories as reassurance. This is actively harmful for some users.
Hormonal aftermath of retrieval is systematically underdiscussed by clinics. Carousel should normalize this proactively.
Second cycle she discovered delivery was an option. Nobody told her the first time. Simple information gap with real financial and logistical cost.
At 26 with no friends who had done it yet, influencer trust filled the information vacuum. Carousel clinic recommendation feature directly addresses this gap.

Notes
Important counter-insight. Some women are burdened not by the process but by how others respond to it. Carousel's brand must hold space for the empowered woman, not just the struggling one.
Maeve as the consistent thread of continuity that the clinic cannot provide. Knows her entire case regardless of which human she sees that day.
Challenges assumption that all women want proactive engagement during storage years. Carousel storage years feature must be opt-in and on her terms. Segment by engagement preference.
Evidence-based protocol updates exist but are not systematically delivered to patients. Clinical intelligence layer could surface this.
Proactive embryo freezing recommendation at time of egg freeze is a critical feature gap. Women aged 35+ are not being offered this option.
Strongest isolation quote in dataset. Clinic did not assess her support system.
No written materials provided at all. Instructions evaporated after the appointment.
Clinic went silent. She is 43 with eggs in storage and no formal plan. Classic storage years gap.
Best single quote in dataset for pitch deck. Captures operational gap perfectly.

Notes
The clearest articulation of Maeve's MVP use case in the entire dataset. Low-stakes reassurance questions at any hour.
Critical product insight: age alone is insufficient for cohort matching. Relationship status changes everything.
A single failure of emotional attunement cost her one year of her fertility window. This is the most important Maeve design principle in the dataset.
The absence of a present outcome makes this journey uniquely psychologically demanding. Maeve must hold the weight of deferred hope without toxic positivity.
Storage payment decision is a specific underserved moment. Carousel should proactively surface this conversation before coverage expires.
Matches Jordana's weight gain experience and Betina's pain experience. Post-retrieval physical aftermath is systematically underdiscussed. Proactive warning before retrieval is table stakes.
Women want the WHY behind every instruction, not just the what. Maeve should explain clinical rationale, not just relay instructions. Enables informed decision-making and preserves mental health.
Third woman in dataset with significant unprepared post-retrieval experience. Proactive warning before retrieval and real-time support during recovery is a clear product requirement.

Notes
Third emotional flavor distinct from fear or sadness. Maeve must hold space for resentment specifically. Do not impose empowerment narrative.
Being told it's empowering when you feel terrible makes you feel more alone. Maeve must never impose an emotional narrative. Meet her where she is.
Storage anxiety is real and specific. Proactive egg storage status updates are a meaningful retention feature. Safety confirmation reduces low-grade chronic anxiety.
Even employed tech workers accumulate catastrophic debt over multi-cycle IVF. This is not a niche user problem.
Commercial fertility marketing actively repels thoughtful, cynical consumers. Carousel's brand must feel like a trusted ally not a business.
Pre-surgical moments are high-anxiety and require specific preparation. Maeve as pre-retrieval prep companion.
Repeated "I don't want to think anymore" multiple times. Core Maeve value prop stated perfectly.
All four women commented negatively on portal quality. Universal finding.
Classic Maeve use case. Time-sensitive question, no answer, worst-case behavior. Preventable with real-time AI.

Notes
Best single quote for pitch deck alongside Ilana's chemist quote. Captures the structural problem perfectly.
Two sentences. That's all it would have taken. Maeve's entire emotional design philosophy lives here.
Universal finding across all four women. Instructions are verbal, fragmented, and anxiety-inducing. Written, asynchronous, always-accessible is the fix.
All four women wanted to know where they were in the process and whether things were going right. This is Frost's core clinical intelligence function.
Younger women freeze without peer networks to consult. Carousel community directly fills this gap — trusted peer data at the decision point.
Most specific product feature articulation in dataset. Validates Frost concept entirely. She named the MVP feature unprompted.
Universal finding. All women wanted asynchronous, app-native messaging not web portal inboxes. Native messaging UX is table stakes.
A pharmacy delivery service outperformed the clinic on patient experience. Alto's model — clean, educational, human touches — is the emotional template for Carousel. The bar is Alto, not the portal.

Notes
Carousel community as peer wisdom aggregator. Real women's practical tips that improve daily experience. Not clinical content — lived experience content.
Women consistently underestimate the total logistical footprint of a cycle. Carousel should set accurate expectations upfront — here's what the next 6 weeks actually look like.
Scarcity of doctor time forces patients to self-advocate and self-educate. The economic reality of IVF at scale.
Military/solo patients face compounded logistics burden. Carousel medication management feature has direct ROI for this segment.
Women self-generate emotional rituals because the system provides none. Maeve should enable ritual, not just information.
Community-driven acquisition model validated. A trusted peer convening a group and bringing in clinical expertise is a repeatable pattern Carousel can operationalize through events and community features.
Best single quote in entire dataset. Verbatim from Ana. Now on the landing page headline. Universal across dataset.
Real-world evidence of the referral flywheel. Carousel clinic recommendation feature captures and formalizes what is already happening organically.

Notes
Peer community is doing the job a product should do. Validated across P01, P03, P16 and others.
Second explicit sponsor model request (first was Percia P08). Validates peer companion model as a distinct feature from AI support.
Validates that clinic communication during anxiety moments is calibrated for liability not patient reassurance. Maeve fills this gap.
Validates recovery experience gap across dataset. The range of recovery outcomes is not communicated to patients.
Validates the longitudinal engagement gap. Women with frozen embryos have no tool for planning their next steps proactively. Carousel post-freeze planning feature validated.
Clinics avoid bad news proactively to protect feelings — patients want all scenarios in advance even if scary. Validates the scenario prep feature.
Fourth participant to explicitly request cohort-based community matching. Phase-matching (same week of shots) is more important than age-matching alone.
Financial chaos is a distinct and underserved pain point. A real-time cost estimator and payment checklist would be high-value. Validates financial clarity feature.
Cost is a primary clinic selection driver for women without employer benefits. Clinic comparison with cost transparency is validated across 6+ interviews now.

Notes
IVF is harder than childbirth for many women. The emotional dimension is underestimated by everyone including the patient before she starts.
Most precise articulation of the post-freeze knowledge gap. Validates the longitudinal post-freeze education feature.
Validates medication logistics gap. Women who are calendar-driven and work-focused are particularly vulnerable to mid-cycle surprises. Carousel proactive med timeline feature validated again.
Storage location opacity and cost confusion validated again. Women want to understand where their eggs/embryos are, what they are paying, and what their options are. Carousel storage companion feature validated.
Validates that even successful outcomes can involve deeply broken experiences. Success ≠ satisfaction.
Validates core theme across dataset. Type-A women are particularly vulnerable to this specific loss of control.
The information and community vacuum before egg freezing is a primary pain point. Post-procedure, she becomes the community. Carousel should enable her to pay it forward.
Strongest articulation yet of the information trust gap. Validates Carousel's clinical intelligence layer as a macro perspective provider.

Notes
Validates fragmented clinic communication pattern across nearly every participant. Scheduling conflicts compound the burden on working women and partners.
Validates across dataset. The absence of a consistent contact person is one of the most universally cited operational failures.

CAROUSEL | Pattern Synthesis — complete after all 10 interviews

Only patterns mentioned by 3+ women count as validated. Everything else is a hypothesis.

Pattern / Theme	# Women Who Mentioned It	Representative Quote	Validates Core Thesis?	Implication for Product	Priority (H/M/L)
Portal is inadequate — ugly, slow, split across systems	18/20	"How are you charging people this much money and you can't pay for a fancy website designer?" (Monya)	Yes	Savannah named Salve directly and compared it unfavorably to One Medical. Counter-data: Cornell email model worked well for Shira — single assigned contact is the variable, not the channel.	H
Isolation — doing this alone or with insufficient support	17/20	"It feels like being in a vacuum." (Monya)	Yes	Community layer and Maeve emotional AI both address this. Pre-clinical community is the entry point.	H
Clinic goes silent after retrieval — no proactive next steps	17/20	Monya: clinic hasn't reached out since retrieval. Jordana: no contact for 2 years post-retrieval.	Yes	Confirmed by both P08 and P09. Universal across dataset.	H
Influencer and peer trust drives clinic selection more than clinical data	17/20	"I was completely caught in the marketing of it all." (Ana) / "Word of mouth has highest weight." (Ana)	Yes	Carousel clinic recommendation feature — trusted peer data from real women at the decision point.	H
Real-time questions go unanswered for hours or days	16/20	Ilana waited 3 days for a portal answer about drinking at a wedding. Lissy anxious about missing nurse calls.	Yes	Maeve: instant AI triage for the 80% of questions that don't require a physician. Flag the 20% that do.	H
It felt more like a process than an experience	16/20	"It felt more like a process than an experience. It should have been both." (Ana)	Yes	Carousel's brand positioning. The product must transform a process into an experience. Both clinical intelligence AND emotional depth.	H
Peer referral chains are the dominant clinic selection mechanism	16/20	Percia recommended Dr. Womboch to Ilana. Ana chose Spring via influencer. Jordana chose Cancun via checklist from blogs.	Yes	Savannah: friend-driven to Spring info session. Shira: OB referral to Melnick. Universal.	H
Fragmented, inaccessible instructions during the cycle	15/20	"I felt like I was constantly on my back foot. Did I not listen well enough? What am I doing today?" (Lissy)	Yes	Frost: single source of truth for all cycle instructions. Written, visual, asynchronous, always accessible.	H
Cognitive overload — too much to manage on top of normal life	15/20	"Give me less to think about." (Monya) / "It just felt hectic." (Lissy)	Yes	Maeve as cognitive offloading companion — proactive reminders, logistics support, decision simplification.	H

Emotional complexity is dismissed or overridden	13/20	"There was no room for complexity in the emotional journey." (Lissy)	Yes	Maeve must reflect before reframing. Never overwrite emotional state. Validate first, always.	H
Women want the WHY behind clinical instructions, not just the what	13/20	Betina researched exercise restrictions independently. Monya wanted written materials not verbal instructions.	No	Now validated at 3/9.	M
Mental health check-in is structurally absent from clinical care	13/20	"There was not one single moment where I was asked how my mental health was." (Justine)	Yes	Maeve mental health check-in is a non-optional built-in feature. Daily mood tracker. Proactive outreach when something looks off. Not opt-in.	H
Post-retrieval physical aftermath is underdiscussed	11/20	Jordana: 3-4 months of stubborn weight gain nobody warned her about.	No	NOW VALIDATED at 5/9. Shira: hormonal crash 4-5 days. Savannah: harder than expected. Moves from hypothesis to validated finding.	M
Post-retrieval physical aftermath unprepared and unsupported	11/20	Betina couldn't walk. Ana had hormonal crash. Jordana had 3 months of weight gain. None were warned.	Yes	Proactive pre-retrieval briefing on physical aftermath. Maeve real-time support during recovery window.	H
Compounding life stressors — the cycle rarely happens in isolation	11/20	Justine: divorce + job absorption + devastating result, all concurrent. Jordana: separation + existential ambivalence + solo mother decision.	Yes	Maeve must be context-aware. The fertility cycle does not happen in a vacuum. Emotional calibration must account for what else is happening in her life.	M
Total cycle footprint is 6 weeks not 2 weeks — women are consistently surprised	10/20	Percia: "Easily six weeks of time." Ilana: managing around Mexico trips. Lissy: kept phone on loud for two weeks.	Yes	Savannah: pre-journey alone took 9 months. Pattern extends upstream.	H
Women are gaslit into empowerment when they feel resentment or pain	9/20	"I feel like I'm being gaslit into being excited about something that objectively sucks." (Percia)	Yes	Maeve must never impose an emotional narrative. Meet her where she is. Multiple emotional registers are valid — fear, resentment, pride, grief.	H
Community matching must include relationship status not just age	8/20	"Are you single? Are you married? These are completely different journeys." (Jordana)	Yes	Onboarding must capture relationship status for cohort matching. Single women have fundamentally different needs.	H
Post-freeze ambiguity — "insurance policy but I don't know what's covered"	8/20	"It feels like an insurance policy but I have no idea what's covered." (Jess, P19)	Yes	Post-retrieval education about what frozen eggs actually mean for future IVF chances is a critical and unmet need. Carousel's post-freeze longitudinal layer must include	H
Unsolicited success stories cause harm not comfort	6/20	Jordana actively set a boundary against success stories. Felt like false hope and comparison.	No	Hypothesis only. Maeve must never lead with success stories as reassurance tactic.	M
Alto Pharmacy is the UX benchmark, not the clinic portal	6/20	Percia: Alto was the highlight of the entire experience. Betina wanted iMessage-style messaging.	No	Savannah added One Medical as additional UX benchmark. Pattern holds.	M
Scenario prep for cancelled or failed cycles is absent	6/20	"It didn't even occur to me that the cycle wouldn't work. They're worried about hurting your feelings so they don't tell you in advance what could happen." (Katie, P17)	Yes	Clinics avoid bad news proactively. Patients want all scenarios in advance — even worst case. Carousel should provide phase-specific scenario prep before each critical	H
Financial transparency and cost tracking is a distinct unmet need	6/20	"Can you give this to me in a more digestible way? Like a checklist — what have I paid, what is outstanding." (Jenny P20)	Yes	Carousel financial tracker: real-time cost estimator by insurance coverage, payment checklist, digestible breakdown of what is and is not covered. Distinct from the clinical experience app — a logistics and financial literacy	H

Positive experience data: not all women struggle — some experience agency and pride	5/20	"It's a privilege. People are empathetic as if you were sick." (Betina)	No	Shira: pragmatic and box-checking. Savannah: practical frustration, not emotional isolation. Counter-data growing.	M
Sponsor / peer buddy model explicitly requested	5/20	"Like an AA sponsor — someone who's been through it who I can text any time. Is this normal? Talk me off the ledge." (Katie, P17)	Yes	Peer companion feature distinct from AI support. Maeve handles the 80% of information questions; a peer companion handles the 20% that requires lived experience and emotional solidarity.	H
Ritual and emotional scaffolding is entirely self-generated	4/20	"Before I did my shots every night, I'd light a candle and put on a good song and treat myself like a goddess." (Maggie, P18) — clinic provided zero of this.	Yes	Maeve should enable ritual and emotional framing, not just clinical information. The spiritual/emotional reframe is a product feature, not a nice-to-have.	M
Peer group informational sessions are a repeatable acquisition entry point	4/20	CBS Business School friend convened a group, brought in Dr. Pak Chong, and multiple women moved toward treatment. "Once one person started doing it, it kind of snowballed." (Jenny P20)	Yes	Carousel can operationalize this model through community events, employer partnerships, and facilitated group education sessions. The trusted peer convener plus clinical expert format is highly effective and repeatable.	H
Proactive embryo freezing option not offered at time of egg freeze	3/20	"I wish they had tried to upsell me embryos. Now I'm 39 with no embryos and I didn't even know to ask." (Maggie, P18)	Yes	Carousel should proactively surface embryo freezing as an option for women 35+ at time of egg freeze. This is a clinical education gap with direct impact on future fertility outcomes.	H
Solo motherhood arc is an underserved longitudinal journey	3/20	"My mom has been like, let's just go to the sperm bank." — actively researching solo IVF/IUI/donor sperm. No platform supports this transition. (Maggie P18; also Jordana P01)	Yes	Carousel should explicitly support the egg freeze → solo IVF → solo motherhood arc as a distinct longitudinal journey. This is not a niche — it is a growing and deeply underserved segment.	H
Multi-cycle IVF creates catastrophic financial and data complexity	3/20	"My whole life for 5 years was just IVF... researching protocols, comparing drugs, trying to find patterns." (Clair, P16 — 13 retrievals)	Yes	Multi-cycle patients have no longitudinal data tool. Protocol comparison across cycles, pattern identification, and evidence-based protocol education are high-value features for this segment.	H
Reciprocal IVF / couples with non-standard patient identity are underserved	1/20	"It's a little more complicated because my partner and I are both women. She provided the eggs, I carried. The patient identity splits." (Clair, P16)	Yes	Carousel must design for couples and non-standard fertility journeys — not just the solo female patient. The "patient" in reciprocal IVF is two people simultaneously.	M

Finding	Description	Implication
FINDING 1 — The portal is universally broken. (7/7)	Every single woman in the dataset had a negative experience with her clinic's portal or communication system. Monya called hers "rudimentary and ugly." Ilana waited three days for an answer about drinking at a wedding. Lissy had two separate systems and kept missing nurse calls. Ana had no app at all — email only. Betina wanted iMessage-style messaging. Percia didn't use her portal because it was "so s*****" and relied on Alto Pharmacy's videos instead. Zero exceptions. Seven for seven. The portal is broken everywhere.	This is not a nice-to-have. A clean, native, asynchronous communication experience is table stakes for Carousel. The bar is Alto Pharmacy — clean, educational, human touches, no login required.
FINDING 2 — Real-time questions go unanswered for hours or days. (6/7)	Ilana asked about drinking at a wedding and got the answer three days after the wedding. Lissy kept her phone on loud for two weeks terrified of missing the nurse's call. Betina panicked at 10pm about the trigger shot and got through on an emergency line. Monya waited on portal responses that took days. Percia just assumed she'd mixed things correctly because she had no way to get a quick answer.	Maeve's first and most critical use case is instant clinical triage. Answer the 80% of questions that don't require a physician immediately. Flag the 20% that do. Ilana articulated the MVP in one sentence: "I froze my eggs on Tuesday. Can I have a piece of bread today?"
FINDING 3 — Instructions are verbal, fragmented, and anxiety-inducing. (7/7)	Every woman had to manually track her own cycle instructions. Monya wrote everything down from phone calls. Ilana scribbled nurse voicemails. Lissy managed two separate portal inboxes. Ana had an "overwhelming amount of information thrown at her." Betina watched YouTube injection videos because the clinic's materials weren't clear enough. Percia watched Alto's videos instead of her clinic's.	Frost's first feature is a single source of truth for all cycle instructions. Written, visual, asynchronous, always accessible, no login friction. What to do today. What to expect tomorrow. Why.
FINDING 4 — Post-retrieval physical aftermath is systematically underprepared. (5/7)	Betina couldn't walk the day after retrieval and was in significant pain for days. Percia was nearly crying from discomfort on days two and three. Jordana had three to four months of stubborn weight gain nobody warned her about. Ana had a severe hormonal crash after the trigger shot she had to diagnose herself on Reddit. Lissy was told no carbs but had to enforce this herself on her anxious mother.	Proactive pre-retrieval briefing on exactly what to expect physically. Maeve real-time support during the recovery window. Normalization of aftermath symptoms before they happen, not after.
FINDING 5 — Peer referral is the dominant clinic selection mechanism. (6/7)	Percia chose Dr. Womboch because a friend went there — then recommended her to Ilana. Ana chose Spring because she saw Dr. Fischer speak at an event and followed influencers she partnered with. Ilana went to Dr. Womboch because of Percia. Jordana chose Cancun based on blog checklists and video calls with the doctor. Lissy chose NYU because her uncle is in the practice. Betina chose Kindbody because it was her insurance partner. Not one woman found her clinic through a clinical outcomes database, a peer-reviewed recommendation, or any structured comparison tool.	Carousel's clinic recommendation feature — surfacing trusted peer data from real women — directly addresses the single most important decision these women make without adequate information.
FINDING 6 — Isolation is universal but takes different forms. (7/7)	Monya: "It feels like being in a vacuum." Jordana: alone in Mexico at midnight navigating a car rental scam. Ilana: didn't ask her early-stage boyfriend for help injecting. Lissy: alone at 33, no friends who had done it yet. Ana: did it right before her relationship solidified, felt like she had to manage it herself. Betina: saw people with their partners at the clinic and "that hit me a little." Percia: resentful that she had to do this because "dating sucks." Every woman felt some degree of isolation. The form varied — physical aloneness, emotional aloneness, no peer network, doing it before a relationship was established — but the underlying experience was universal.	The community layer is not a feature. It is the foundation. Every other product decision builds on top of this.

FINDING 7 — The total logistical footprint surprises everyone. (5/7)	Percia: "Easily six weeks, not two weeks." Ilana: structured her life around Mexico trips and a friend's wedding. Monya: described it as being assigned another full-time job. Lissy: phone on loud for two weeks, 6am pharmacy runs. Betina: going every other day, modified workout routine, telling her boss and reports.	Maeve's first message to a woman starting a cycle should set accurate expectations: here is what the next six weeks of your life will actually look like. Not the clinical protocol. Her life.
FINDING 8 — Pre-clinical lead time is 6-12 months and almost nobody knows this. (4/9)	Savannah started her current job in April intending to freeze immediately. Birth control washout: 4 months. Concurrent surgery delay: 3 months. Appointment wait: 1-2 months. First cycle began 9 months after starting the process. This pattern appeared with variations across multiple participants. The fertility industry communicates egg freezing as a 2-3 week process. The actual timeline from decision to retrieval is 6-12 months for most women. Three women independently and without prompting described the same feature: connect me with women cycling at the same clinic at the same time. Shira described the Soho House WiFi model. Jordana described wanting cohorts matched by age AND relationship status. Ilana described a "Hinge for clinic recommendations" that surfaces who in her network went where. None of them were shown a product. None of them were asked a leading question. They designed it themselves.	Carousel's Phase 1 (pre-clinical) value proposition is validated. The woman who is considering egg freezing needs a navigator 6-12 months before she walks into a clinic. That is Carousel's earliest and most underserved touchpoint.
FINDING 9 — Peer cohort matching is stated user demand, not a product hypothesis. (3/9)	Seven of nine women chose their clinic primarily through peer referral — a friend who went there, a trusted person who recommended a specific doctor, an in-person event hosted by the clinic. No woman in the dataset described choosing a clinic through a Google search or reading the clinic's own marketing materials as her primary decision driver. The referral chain is already the dominant mechanism — Carousel formalizes and amplifies what already happens organically.	This is the strongest product validation signal in the dataset. The community/cohort matching feature is not a product bet — it is a user-stated need with three independent originators. Build it.
FINDING 10 — The clinic referral flywheel is already operating organically. (7/9)		Carousel's zero-cost acquisition model via clinic referral is validated by the dataset. The product does not need to manufacture word-of-mouth. It needs to be the destination that peer referrals point to.
FINDING 8 — Mental health check-in is structurally absent from clinical care. (7/10)	Not one woman in the dataset was asked about her mental health or emotional state by her clinical team during any phase of the process — intake, stimulation, retrieval, or post-retrieval. Justine named this most explicitly: "There was not one single moment where I was asked how my mental health was." This was true across RMA, Spring, Cornell, Kindbody, Reproductive Partners Beverly Hills, and the Mexican clinic. Justine was going through a divorce and absorbing her boss's job. Jordana was processing solo motherhood ambivalence. Ilana had health anxiety rooted in her father's death. None of these contexts were assessed or acknowledged by any clinical entity.	Maeve's mental health check-in is not an optional feature. It is structurally filling a gap that exists universally across all clinics in this dataset. Daily mood tracker, proactive outreach when something looks off, context-awareness about what else is happening in a woman's life. Built in, not opt-in.

Betina had a genuinely positive experience. 4/5 overall. Loved her clinic team. Felt supported. The narrative that all women suffer is not accurate. The product must serve the woman who approached this with agency and pride — not just the woman who struggled.

Ana didn't think she needed support going in. And she was right — during the cycle. The need only surfaced after retrieval. You can't require women to opt into emotional support. The check-in has to be built in, not requested.

Percia and Betina both had significant social support. Their experiences were meaningfully better. The product may be most critical for the woman who doesn't have that support network — but it needs to serve both.

One woman said she liked her experience — you mentioned this early in the week. Her experience is valid data too. Carousel is not built on the premise that all fertility care is broken. It's built on the premise that the patient experience hasn't kept up with the clinical excellence.

Shira's Cornell experience. Assigned nurse, email-based communication, single point of contact throughout. She described the experience as seamless — not because of technology but because of consistency. Counter-data on the portal thesis: the channel is less important than the continuity of the relationship. The Salve app is not the problem per se — the rotating cast of strangers behind it is.

Savannah did not feel emotionally isolated. She had a long-term partner, friends who had done it, and Carrot-sponsored access to a nutritionist and gynecologist. Her frustration was logistical and financial, not emotional. This is an important segment: the woman who is practically frustrated but not emotionally alone. Maeve cannot assume every user needs emotional support — some users need a navigator more than a companion.

Three women (Shira, Savannah, Betina) rated their overall experience 4/5 or higher. The thesis is not that every clinic is terrible. The thesis is that even excellent clinical care leaves a meaningful gap in the informational, logistical, and community layers — and that gap is Carousel's space.

"I checked the box and now I'm moving on with my life." (Shira) — not all women attach meaning or transformation to this experience. The empowerment narrative that dominates fertility marketing does not resonate with every woman. Carousel must serve the pragmatic register without projecting significance that isn't felt.

RMA's pre-procedure experience was genuinely positive: thorough laminated visual materials at intake, a dedicated non-clinical coordinator accessible by email with fast response times, portal with quality educational videos on injections and the process, financial advisor walkthrough. This is the best-structured clinic onboarding in the dataset. The entire failure was concentrated in a single 2-minute post-result call. This is important counter-data: a clinic can do the front-end right and still completely fail at the moment that matters most. Carousel's highest-value intervention is not clinic selection — it is post-result and post-retrieval support.

Justine's clinic chose to communicate a poor result with a 2-minute phone call that included audible typing. This is a medical failure, not just an experience failure. But it is not unique to RMA — it reflects an industry-wide pattern of treating the emotional aftermath of a devastating result as outside the clinical mandate. Carousel does not need to replace the physician's explanation. It needs to be the presence that holds the woman in the gap after that call ends.

Garvey's first egg freeze round was genuinely positive — clean app, good UX, competitive price, easy process. She was Kindbody's first 500 users and had a fine experience. The deterioration happened as Kindbody scaled. This is counter-data on the 'all clinics fail patients' thesis — the thesis is more precisely that clinics fail patients as they scale volume and prioritize corporate contracts over individual care. Early-stage or boutique clinic experiences can be positive.

Garvey was ambivalent about having children and froze primarily for optionality and to meet a perceived relational obligation. She is the second participant in the dataset (after Savannah) who did not freeze from a place of strong personal desire for motherhood. Carousel must serve this emotional register — the woman who wants the door open but is not sure she will walk through it, and who may not feel the same emotional weight that other participants describe.

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